

COURT ONLINE COVER PAGE

IN THE LABOUR COURT OF SOUTH
AFRICA HELD AT CAPE TOWN

CASE NO: **2026-083299**

In the matter between:

Wilbur Matthee William Matthee

Plaintiff / Applicant

and

**AFROCENTRIC HEALTH (RF) (PTY)
LTD, MEDSCHEME HOLDINGS (PTY)
LTD**

Defendant / Respondent

Referrals or Statement of claim

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ELECTRONICALLY SIGNED BY:

**Registrar of The Labour Court,
Cape Town**

**IN THE LABOUR COURT OF SOUTH AFRICA
CAPE TOWN**

MATTHEE
v
**AFROCENTRIC HEALTH (RF) (PTY) LTD
& MEDSCHEME HOLDINGS (PTY) LTD**

INDEX AND ANNEXURE BUNDLE

Statement of Claim — 9 April 2026

Rule 6, Rules for the Conduct of Proceedings before the Labour Court



Applicant	Wilbur William Matthee (Self-Represented)
Email	info@ethichawks.co.za
CCMA Cases	WECT23564-25 WECT24486-25
Concurrent Proceedings	Case No. 2026-077546
Date Filed	9 April 2026
Total Annexures	28 documents

INTRODUCTORY NOTE TO THE REGISTRAR AND RESPONDENTS

1. This Index and Annexure Bundle is filed together with the Statement of Claim by the Applicant, Wilbur William Matthee, in his self-represented capacity, pursuant to Rule 6 of the Rules for the Conduct of Proceedings before the Labour Court.
2. This bundle comprises 28 annexures, designated Annexures “A” through “Z”, “P1 RISK”, and “PDA BILL”. Each annexure is preceded by a divider tab identifying its reference designation, full description, date, and the Part of the Statement of Claim to which it relates.
3. The purpose of the divider tab system is to assist the Registrar, the presiding Judge, and the Respondents’ legal representatives in locating documents efficiently during proceedings. Any cross-reference in the Statement of Claim to an Annexure (e.g., Annexure “N”) may be resolved by locating the corresponding tab in this bundle.
4. All annexures are documents within the Applicant’s personal knowledge, obtained during his employment with the Second Respondent or from regulatory and judicial processes arising from that employment.
5. Where a document is redacted, the redaction is limited to the personal information of third parties (e.g., GEMS member identity numbers) consistent with the Protection of Personal Information Act 4 of 2013. No redaction has been applied to any information material to the issues in dispute.
6. Annexure “Z” (EthicHawks Master Chronological Register) constitutes a comprehensive forensic chronology of 148 material events from 11 January 2024 to 2 April 2026. It is commended to the Court as a navigational aid to the factual record.
7. The Applicant confirms that each document in this bundle is a true and accurate copy of the original, save where indicated to the contrary on the relevant divider tab.



MASTER INDEX OF ANNEXURES

Tab	Description	Relevance (Part)	Date
"A"	Interview invitations for 'Specialist: Care Manager' role	<i>Part C — Employment Relationship</i>	January 2024
"B"	Assessment instructions — Specialist: Care Manager	<i>Part C — Employment Relationship</i>	29 January 2024
"C"	Fingerprint criminal check booking confirmation	<i>Part C — Employment Relationship</i>	21 February 2024
"D"	Employment contract — 'Care Coordinator' (Grade B4M1)	<i>Part C — Employment Relationship</i>	9 April 2024
"E"	First Aider roster and EHS directives drafted by Applicant	<i>Part C — Employment Relationship</i>	25 July 2025
"F"	Sick note — Dr Jacques Malan (Akeso Stepping Stones Clinic)	<i>Part D — Mental Health / Disability</i>	1 April 2025
"G"	Mental health disclosure to Mapule Mashele (formal written notice of disability)	<i>Part D — Mental Health / Disability</i>	16 May 2025
"H"	Jones: "we do not apply staggering of money owed"	<i>Part D — Mental Health / Disability</i>	16 April 2025
"I"	Payslip — July 2025 (R14,811.77 deduction; 40.47% of gross salary)	<i>Part E — Unlawful Salary Deduction</i>	July 2025
"J"	AOD Policy confirming 25% cap + "what is illegal?" exchange	<i>Part E — Unlawful Salary Deduction</i>	24 July 2025
"K"	"Wilma" Microsoft Teams message (screenshot, team-wide channel)	<i>Part F — Gender Identity Discrimination</i>	23 September 2025
"L"	Formal discrimination complaint lodged with HR	<i>Part F — Gender Identity Discrimination</i>	25 September 2025
"M"	Mashele investigation closure report ("typo" finding)	<i>Part F — Gender Identity Discrimination</i>	1 October 2025
"N"	Gexus/DMS root cause analysis and protected disclosure	<i>Part G — Protected Disclosures</i>	23 October 2025
"O"	Transcript — 31 October 2025 meeting (retaliatory admission by Jones)	<i>Part I — Retaliatory Intent</i>	31 October 2025
"P"	Formal grievance against Jones	<i>Part J — Institutional Silence</i>	2 November 2025
"Q"	Executive escalation bundle to leadership (32-page evidence bundle)	<i>Part J — Institutional Silence</i>	4 December 2025
"R"	Employer's opposing affidavit — CCMA (Xolile Vincent Manyathi)	<i>Part K — Bad-Faith Affidavit</i>	10 December 2025
"S"	Resignation letter	<i>Part L — Constructive Dismissal</i>	11 December 2025
"T"	CPUT Postgraduate Diploma in Occupational Health Nursing — acceptance letter	<i>Part M — Career Destruction</i>	2026



"U"	uFiling screenshot — 'No Information' for June 2025	<i>Part N — Post-Resignation Victimisation</i>	2026
"V"	Certificate of Service (disputed) + Naming Convention request	<i>Part N — Post-Resignation Victimisation</i>	January 2026
"W"	CMS referral — 'Administrative Misconduct and Ethical Failure'	<i>Part O — Regulatory Confirmation</i>	2026
"X"	SAHRC acknowledgment and referrals	<i>Part O — Regulatory Confirmation</i>	2026
"Y"	CCMA Certificates of Non-Resolution (WECT23564-25 and WECT24486-25)	<i>Part A — Jurisdiction</i>	9 January 2026
"Z"	EthicHawks Master Chronological Register (148 entries, 11 Jan 2024 – 2 Apr 2026)	<i>Part O — Regulatory Confirmation</i>	3 April 2026
"Z1"	The Initial Disclosure — Duplicate LACE Files and Early Suppression (6 Mar 2025)	<i>Part G — Protected Disclosures</i>	6 March 2025
"P1 RISK"	NQF Mandate / SAQA Statutory Obligation to Report (ELO mapping; qualification transcript)	<i>Part G — Protected Disclosures</i>	8 April 2026
"PDA BILL"	Protected Disclosures Amendment Bill, 2026 (Government Gazette, 8 April 2026)	<i>Preamble</i>	8 April 2026



IN THE LABOUR COURT OF SOUTH AFRICA
CAPE TOWN

CASE NO: _____

In the matter between:

WILBUR WILLIAM MATTHEE

Applicant

and

AFROCENTRIC HEALTH (RF) (PTY) LTD

First Respondent

MEDSCHEME HOLDINGS (PTY) LTD

Second Respondent



STATEMENT OF CLAIM

Final — 9 April 2026

Lodged pursuant to Rule 6 of the Rules for the Conduct of Proceedings before the Labour Court

Applicant (Self-Represented)	Wilbur William Matthee info@ethichawks.co.za 069 635 1435 / 063 378 8782
Address for Service	15 Bellevue Street, Voorbrug, Cape Town, 7001 (email service accepted)
CCMA References	WECT23564-25 and WECT24486-25 (Certificates of Non-Resolution: 9 January 2026)
Concurrent Proceedings	Case No. 2026-077546 (BCEA s77A — unlawful salary deduction)

NOTE TO THE REGISTRAR REGARDING PART H (COPYRIGHT / CPD PROGRAMME)

The Applicant draws the Registrar's attention to Part H of this Statement of Claim. The facts concerning the CPD programme — developed by the Applicant in April 2025, rejected by management, and subsequently implemented without attribution in October 2025 — are pleaded solely as evidence of the Respondents' bad faith, retaliatory intent, and the intolerability of the employment relationship. The Applicant does not seek any damages or corrective relief for copyright infringement or intellectual property misappropriation in these proceedings. The Applicant expressly reserves the right to pursue those claims in separate proceedings. Part H is included for evidentiary purposes only, consistent with Rule 6 of the Labour Court Rules.

PREAMBLE — THE PROTECTED DISCLOSURES AMENDMENT BILL, 2026

The Applicant draws the attention of this Honourable Court to the Protected Disclosures Amendment Bill, 2026, published by the Minister of Justice and Constitutional Development on 8 April 2026 [Government Gazette No. [●] of 8 April 2026, GN [●] — Gazette number to be confirmed and inserted upon receipt of official copy; a copy is attached as Annexure “PDA BILL”]. While the Bill is not yet enacted, it represents Parliament’s most recent and authoritative statement on the interpretation and purpose of the Protected Disclosures Act 26 of 2000 (“the PDA”).

The Bill expressly:

- Expands the definition of “occupational detriment” to include threatening a frivolous or vexatious complaint in response to a protected disclosure — precisely what the Applicant’s manager did on 31 October 2025, stating: “You made a case of harassment against me, so I can make a case of harassment against you”;
- Creates a Whistleblower Protection Fund for destitute whistleblowers — the Applicant is presently homeless, without running water, and has had personal possessions seized by creditors;
- Extends protection to former employees — the Applicant resigned on 12 December 2025, yet the retaliation continues; and
- Imposes criminal penalties for retaliation against whistleblowers.



This Court is respectfully requested to interpret the existing PDA purposively and in light of the Bill as a clear indicator of parliamentary intent to strengthen, not weaken, whistleblower protection. Any narrow interpretation that would exclude the Applicant’s documented protected disclosures would contradict the very mischief the Amendment Bill seeks to remedy.

PRELIMINARY NOTE TO THE REGISTRAR AND RESPONDENTS

The Applicant is a self-represented litigant. This Statement of Claim is filed pursuant to Rule 6 of the Rules for the Conduct of Proceedings before the Labour Court following the issuing of Certificates of Non-Resolution by the CCMA (Case Nos. WECT23564-25 and WECT24486-25) on 9 January 2026. The Applicant respectfully requests that the Registrar and the Court apply the principle of equality of arms and afford the Applicant reasonable procedural latitude consistent with his self-represented status.

PART A: JURISDICTION AND REFERRAL BASIS

1. This Court has jurisdiction to adjudicate this dispute in terms of section 157(1) of the Labour Relations Act 66 of 1995 (“the LRA”), which confers exclusive jurisdiction on the Labour Court in respect of any matter that the LRA requires to be determined by the Court.
2. The Applicant referred two disputes to the Commission for Conciliation, Mediation and Arbitration (“CCMA”):
 - (a) Case WECT23564-25: Unfair Labour Practice — unlawful salary deduction, discrimination, and victimisation; and
 - (b) Case WECT24486-25: Automatically Unfair Constructive Dismissal — retaliation for protected disclosures.

3. Conciliation was held on 9 January 2026. Certificates of Non-Resolution were issued on the same date (Annexure “Y”). The CCMA further confirmed, by letter from the Provincial Senior Commissioner dated 28 January 2026, that it lacks jurisdiction to arbitrate automatically unfair dismissal disputes and protected disclosure retaliation disputes, and directed the Applicant to the Labour Court.
4. This Statement of Claim is filed within the 90-day period prescribed by section 191(11)(a)(i) of the LRA, calculated from 9 January 2026. The filing deadline is on or before 9 April 2026. This Claim is accordingly timeously filed.
5. Additionally, in respect of the claim for automatically unfair dismissal under section 187(1)(h) of the LRA (protected disclosure retaliation), no period of prescription applies in terms of section 77(3) of the Basic Conditions of Employment Act 75 of 1997 (“BCEA”), read with section 16 of the Prescription Act 68 of 1969, as the cause of action arose from a continuing wrong that concluded upon the Applicant’s resignation on 12 December 2025.
6. A further claim under section 77A of the BCEA for the unlawful salary deduction has been filed separately with this Court as Case No. 2026-077546 (Matthee v AfroCentric Health (RF) (Pty) Ltd). The Applicant requests that the Court consider consolidating the two matters in the interest of justice and procedural efficiency, alternatively notes the BCEA claim as a concurrent proceeding.



PART B: THE PARTIES

1. The Applicant is Wilbur William Matthee, an adult male, a Registered Nurse (SANC No. 15831886), holding a Postgraduate Diploma in Health Services Management (SAQA ID 118316, NQF Level 8), awarded by the Cape Peninsula University of Technology with a 90% Distinction in Professional Development. He is a self-represented litigant residing in Cape Town, Western Cape. As a direct and proximate result of the Respondents’ conduct, the Applicant is currently rendered destitute and unable to pay his SANC annual registration fee, and accordingly unable to practise as a registered nurse.
2. The First Respondent, AfroCentric Health (RF) (Pty) Ltd (Registration No. 1996/009540/07), is a private company incorporated and registered in South Africa, with its principal place of business at 37 Conrad Street, Florida North, Roodepoort, Gauteng. It is the holding company of the Second Respondent.
3. The Second Respondent, Medscheme Holdings (Pty) Ltd (Registration No. 1970/015014/07), is a wholly-owned subsidiary of the First Respondent and was the Applicant’s direct employer at all material times. It is an accredited administrator of medical schemes registered under the Medical Schemes Act 131 of 1998, including the Government Employees Medical Scheme (“GEMS”).
4. The directors and officers of the First Respondent (including Bruno Fernandes, Dr Nkateko Munisi, Alice le Roux, Gerald Van Wyk, and LSM Mpumlwana) and certain employees of the Second Respondent (Mary-Ann Jones, Mapule Mashele, Xolile Vincent Manyathi, and Lisa Mari) are not cited as formal respondents in this Statement of Claim. Their identities, roles, and conduct are fully set out herein. The Applicant reserves the right to apply for joinder of any or all of these persons in the event that the Court orders a personal liability enquiry pursuant to section 162 of the Companies Act 71 of 2008, section 60 of the Employment Equity Act 55 of 1998 (“EEA”), or section 4 of the PDA, or in any separate proceedings as the evidence may warrant.

PART C: THE EMPLOYMENT RELATIONSHIP AND THE BAIT-AND-SWITCH

1. With effect from 1 May 2024, the Applicant entered into a contract of employment with the Second Respondent in Cape Town, Western Cape.
2. In January 2024, the Applicant was recruited by the Second Respondent for the position of “Specialist: Care Manager”. He received three separate interview invitations (Annexure “A”), underwent fingerprint criminal verification on 21 February 2024 (Annexure “C”), and completed Specialist-level assessment instructions on 29 January 2024 (Annexure “B”). All recruitment documentation described the position as a Specialist role.
3. However, the contract presented to the Applicant for signature on 9 April 2024 downgraded his position to “Care Coordinator” (Grade B4M1) at a cost-to-company of R490,032 per annum — without any prior notification of the change in designation (Annexure “D”).
4. This deliberate act of title suppression constituted a “bait-and-switch”: the Applicant was recruited and utilised as an NQF Level 8 Specialist but was officially designated as a junior “Care Coordinator” to:
 - (a) avoid paying a remuneration package commensurate with a Specialist;
 - (b) suppress his management status in employment records, certificates of service, and UIF declarations; and
 - (c) facilitate potential B-BBEE fronting, in respect of which the B-BBEE Commission has allocated Case No. 5/02/2026.
5. In support of the assertion that the Applicant performed Specialist-level functions, it is noted that during his employment he was appointed as Acting Chairperson of the Employee Health and Safety (EHS) Committee and as Lead First Aider for the 2nd, 3rd and 4th floors of the Second Respondent’s premises. On 25 July 2025, the Applicant drafted and implemented a First Aider roster covering all three floors and issued directives to team leaders regarding occupational health and safety compliance (Annexure “E”). These responsibilities constitute operational management control and confirm that his true role was that of a Health Services Management Specialist (NQF Level 8).
6. The Senior HR Business Partner, Cheryl-Dawn Modern, confirmed in writing on 19 January 2026 that the “naming convention changes [were] still in progress” — an admission that the Applicant’s designation had not been updated to reflect his true role, and that the Certificate of Service issued to him is therefore inaccurate.



PART D: MENTAL HEALTH DISCLOSURE AND FAILURE TO ACCOMMODATE

1. From 18 March 2025, the Applicant was admitted as an inpatient at Akeso Stepping Stones Clinic for severe depression and post-traumatic stress disorder (“PTSD”). A sick note issued by his treating psychiatrist, Dr Jacques Malan, is attached as Annexure “F”.
2. On 16 May 2025, the Applicant provided Dr Malan’s psychiatric letter to Mapule Mashele (Senior Manager, Second Respondent), placing the Second Respondent on formal written notice of his disability within the meaning of section 1 of the EEA and his need for reasonable accommodation (Annexure “G”).
3. Notwithstanding this formal notification, Mary-Ann Jones (Team Leader, Second Respondent) failed to provide reasonable accommodation. Specifically:

- (a) On 16 April 2025, Jones stated in writing: “we do not apply staggering of money owed” — a position the Second Respondent later deployed to justify the unlawful deduction (Annexure “H”);
 - (b) On 7 May 2025, the Applicant requested staggering of unpaid leave deductions; Jones declined; and
 - (c) On 20–21 May 2025, the Applicant suffered a medical crisis and was required to leave the workplace to obtain medication; Jones instructed him to log unpaid leave, thereby applying a punitive response to a medical emergency.
4. The Respondents’ failure to provide reasonable accommodation to an employee with a documented disability, of which they had formal written notice, constitutes:
 - (a) unfair discrimination on the ground of disability in terms of section 6(1) of the EEA, read with section 7(b)(iii) of the Code of Good Practice on the Employment of Persons with Disabilities; and
 - (b) a failure to comply with the duty to take all reasonable steps to prevent unfair discrimination under section 60 of the EEA.



PART E: THE UNLAWFUL SALARY DEDUCTION — BCEA SECTION 34(2)

1. In July 2025, while the Applicant was between psychiatric admissions and while the Respondents were in possession of Dr Malan’s letter confirming his medical vulnerability and need for accommodation, the Second Respondent deducted the sum of R14,811.77 (representing 40.47% of his gross monthly salary) in a single pay cycle, leaving him with a net pay of R15,679.29. The payslip is attached as Annexure “I”.
2. This deduction was made:
 - (a) without any prior written agreement or written consent as required by section 34(1) of the BCEA;
 - (b) in excess of the 25% cap imposed by section 34(2) of the BCEA;
 - (c) without any prior consultation with the Applicant, despite his express earlier request for staggering; and
 - (d) in breach of the Second Respondent’s own HCBP Policy, which also imposes a 25% cap and requires prior consultation.
3. On 24 July 2025, Jones shared the AOD Policy confirming that “monthly deductions must not be more than 25% of the employees CTC as per legislation” (Annexure “J”). When the Applicant pointed out that the deduction was therefore unlawful, Jones replied: “Hi Wilbur, what is illegal?” — feigning ignorance of the very statutory cap she had just confirmed. This exchange constitutes direct evidence that the Second Respondent was aware of the legal prohibition and proceeded regardless.
4. The deduction contravenes section 34(2) of the BCEA. The Applicant is accordingly entitled to recover R14,811.77 together with interest at the prescribed rate from 23 July 2025 to date of payment in terms of section 77A of the BCEA. The Applicant notes that a separate civil claim for this amount has been filed in this Court as Case No. 2026-077546.

PART F: GENDER IDENTITY DISCRIMINATION — THE “WILMA” INCIDENT

1. On 23 September 2025, immediately after the Applicant proposed a clinical filtering strategy to improve the operational management of member adherence cases, Jones posted a message in a team-wide Microsoft Teams channel in which she:
 - (a) referred to the Applicant as “Wilma” — a feminised version of his name, knowing the Applicant is gay; and
 - (b) in the same message, prohibited the Applicant’s filtering strategy with the words “NO FILTERING” highlighted in red.
2. The temporal and contextual nexus between the Applicant’s clinical proposal and Jones’s response is direct: the feminisation of his name and the simultaneous rejection of his strategy in a team-wide communication constitutes harassment on the grounds of gender identity and sexual orientation within the meaning of section 6(3) of the EEA, read with clause 3.6 of the Code of Good Practice on the Handling of Sexual Harassment Cases in the Workplace.
3. On 25 September 2025, the Applicant lodged a formal complaint of sexual orientation discrimination and harassment with Human Resources, requesting: a neutral investigator; that he not be required to meet with the accused manager; that Jones be suspended pending investigation; and that he be removed from Jones’s reporting line (Annexure “L”).
4. The HR response was inadequate:
 - (a) The Applicant was told he would not be required to meet Jones, but he was never removed from her reporting line — leaving him directly exposed to further retaliation;
 - (b) On 29 September 2025 — the same day the formal complaint was filed — Jones loaded a double punitive unpaid leave entry against the Applicant. Mapule Mashele reversed the entry on 1 October 2025, confirming that it was discretionary and retaliatory; and
 - (c) On 1 October 2025, Mapule Mashele closed the investigation, concluding that Jones’s use of “Wilma” was a “typo” — a manifestly inadequate finding that ignored the Applicant’s May 2025 disclosure of Jones’s hostile conduct, the immediate context, and the team-wide visibility of the message. The closure report is attached as Annexure “M”.
5. The Respondents’ failure to take reasonable steps to prevent and address the harassment constitutes a contravention of section 60 of the EEA. The discriminatory conduct and the whitewash investigation constitute unfair discrimination in terms of section 6(1) of the EEA on the grounds of sexual orientation and/or gender identity.



PART G0: THE INITIAL DISCLOSURE — DUPLICATE LACE FILES AND EARLY SUPPRESSION (6 MARCH 2025)

1. On or about 6 March 2025, as reflected in Annexure “Z” (EthicHawks Master Chronological Register), the Applicant identified and raised concerns regarding the existence of duplicate LACE files within the Respondents’ operational system.
2. The Applicant expressly questioned the operational inefficiency created by these duplicate files, noting that time spent processing duplicated or redundant data entries could instead be utilised for direct patient engagement, including contacting GEMS members requiring clinical intervention.

3. The concern raised by the Applicant was not administrative in nature alone, but directly implicated clinical governance, resource allocation, and patient care outcomes. The duplication of LACE files created a foreseeable risk that clinically indicated patient contact could be delayed or deprioritised.
4. The Applicant's raising of the duplicate LACE file issue constituted the first instance in which he questioned systemic operational processes with direct implications for patient care and clinical efficiency.
5. The response to the Applicant's concern was not one of engagement or investigation. Instead, the Applicant experienced intimidation and suppression when raising these questions, establishing an early pattern of hostility toward his attempts to address systemic issues.
6. This incident is significant in that it predates the formal protected disclosure of 23 October 2025 (Annexure "N") and demonstrates that the Respondents' adverse attitude toward the Applicant did not arise spontaneously in October 2025, but was part of a developing pattern of resistance to, and suppression of, the Applicant's efforts to raise operational and clinical concerns.
7. The 6 March 2025 incident accordingly constitutes either:
 - (a) an initial protected disclosure within the meaning of section 1 of the Protected Disclosures Act; or
 - (b) at minimum, a precursor disclosure forming part of a continuous course of conduct culminating in the formal protected disclosure of 23 October 2025.
8. The intimidation experienced by the Applicant in response to this early disclosure forms part of the cumulative pattern of occupational detriment contemplated in section 3 of the Protected Disclosures Act and is relevant to establishing the causal nexus between the Applicant's protected conduct and the subsequent retaliation, including the constructive dismissal.
9. The Applicant further recorded the events of 6 March 2025 in a contemporaneous handwritten note (attached hereto as Annexure "Z1"). This note was made shortly after the interaction and reflects the Applicant's immediate account of the confrontation, including the Respondent's aggressive tone, defensive posture, and statements perceived by the Applicant as intimidating in response to his query regarding duplicate LACE files.
10. While the note constitutes the Applicant's own record, it is consistent with the broader documentary evidence and timeline set out in Annexure "Z" and serves to corroborate that the Applicant's concerns regarding operational inefficiencies were met with hostility rather than engagement.



G1. Continuity and Escalation of Protected Conduct

1. The events of 6 March 2025 (Part G0) mark the inception of the Applicant's engagement with systemic operational deficiencies affecting patient care.
2. From this point onward, the Applicant continued to raise concerns regarding clinical governance, data integrity, and patient contact inefficiencies within the Respondents' systems.

3. These concerns culminated in the formal protected disclosure dated 23 October 2025 (Annexure “N”), which must be understood not as an isolated act, but as part of a continuous course of protected conduct.
4. The Respondents’ conduct toward the Applicant similarly reflects a pattern of escalation—from initial hostility and intimidation (March 2025), to formalised retaliation and adverse treatment following the protected disclosure.
5. Accordingly, the Applicant will contend that the occupational detriment suffered was not coincidental, but causally linked to a sustained pattern of protected disclosures as contemplated in the Protected Disclosures Act.

PART G: PROTECTED DISCLOSURES — PATIENT SAFETY (GEXUS/DMS INTEGRATION GAP) AND STATUTORY OBLIGATION TO REPORT

G2. The Statutory Obligation to Report (SAQA / NQF / Nursing Act)

The Applicant holds a Postgraduate Diploma in Health Services Management (SAQA ID 118316, NQF Level 8) registered on the National Qualifications Framework under the NQF Act 67 of 2008. Its Exit Level Outcomes (ELOs) are not merely academic achievements — they are statutory professional obligations imposed by Parliament through the South African Nursing Council (SANC) and the South African Qualifications Authority (SAQA). Specifically:

- ELO 2 requires the holder to: “identify and apply legislative and regulatory requirements pertaining to healthcare management ... and advocate for safe and quality care”;
- ELO 5 requires the holder to: “apply evidence-based solutions grounded in management processes”; and
- ELO 6 requires the holder to: “draw a strategic plan for managing risks and adverse events in a healthcare service and develop a quality assurance programme on a risk identified in a unit”.

When the Applicant identified the Gexus-DMS integration failure — a Priority 1 clinical governance risk generating clinically incorrect non-adherence letters to elderly GEMS beneficiaries — and delivered a formal Priority 1 Material Risk Report with 38 attachments to the board on 9 December 2025, he was fulfilling a statutory obligation imposed by his NQF-registered qualification, the NQF Act, and the Nursing Act. The full statutory argument is set out in Annexure “P1 RISK” attached hereto.

The Respondents cannot lawfully characterise a statutorily mandated patient safety report as “insubordination” or “misconduct”. To do so would bring the NQF Act, SAQA, SANC, and Parliament into disrepute.

G3. The Protected Disclosure of 23 October 2025

On 23 October 2025, the Applicant made a protected disclosure to Jones, copied to Lisa Mari (Responsible Pharmacist, Second Respondent), identifying the systemic structural integration gap between the Gexus and DMS platforms that was generating false non-adherence letters to GEMS members, including a 75-year-old dependent (Annexure “N”).

The disclosure constitutes a protected disclosure within the meaning of section 1 of the PDA in that it related to:

- (a) a failure to comply with a legal obligation, namely the Responsible Pharmacist’s obligation under section 49 of the Pharmacy Act 53 of 1974 to ensure the integrity of pharmaceutical care communications; and



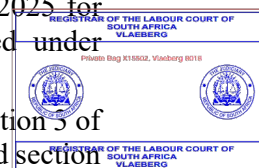
- (b) conduct likely to endanger the health or safety of any individual, namely GEMS members receiving clinically incorrect adherence assessments.

The Applicant's root cause analysis confirmed that the integration gap was structural — not an isolated error — affecting any member transitioning between benefit categories. He stated: "Given that the integration gap still exists, I will not contact the member at this time." Jones responded: "The below is noted. I am not in agreement with your feedback though." Lisa Mari, as Responsible Pharmacist, did not intervene to correct the system error.

The Applicant was subsequently:

- (a) labelled as "insubordinate" for declining to contact a vulnerable patient on clinically incorrect grounds;
- (b) excluded from the resolution process for the integration error he had identified; and
- (c) compelled to issue a forced apology to Mary-Ann Jones on 23 October 2025 for "poor communication and failure to comply" — an apology extracted under constructive duress for a legitimate patient safety disclosure.

The Applicant's disclosure, and the adverse treatment that followed it, engage both section 5 of the PDA (prohibition on occupational detriment for making a protected disclosure) and section 187(1)(h) of the LRA (automatically unfair dismissal where the reason for dismissal is the making of a protected disclosure).



PART H: MISAPPROPRIATION OF INTELLECTUAL PROPERTY — CPD PROGRAMME

Pleaded only as evidence of bad faith and retaliation. Substantive intellectual property claim expressly reserved. No relief in respect of the CPD programme is sought in this Statement of Claim save as specified in paragraphs 2 and 3 below.

1. Factual Timeline of the CPD Programme Development and Misappropriation

- 1.1 In April 2025, the Applicant developed a comprehensive Continuing Professional Development ("CPD") programme for nursing staff within the Second Respondent's clinical operations. The programme was designed as a direct application of his NQF Level 8 Professional Development competency and was presented to management as a bespoke professional development offering.
- 1.2 The programme was developed independently by the Applicant in his personal capacity, drawing on his professional qualifications and clinical governance experience. No request for its development had been made by management, and no institutional resources were applied to its creation.
- 1.3 The Second Respondent's management rejected the programme in April 2025. No feedback, refinement request, or acknowledgment of the Applicant's authorship was provided.
- 1.4 In October 2025 — approximately six months after the Applicant's submission and without any notice to, consultation with, or attribution of the Applicant — management silently implemented the CPD programme across the department. The Applicant was not informed. He received no credit, no acknowledgment, no compensation, and no opportunity to participate in or refine the programme he had authored.

2. Evidentiary Value of the Misappropriation

The Applicant pleads the facts set out in paragraph 1 above not as a standalone claim for copyright infringement in these proceedings, but as evidence of the following:

- (a) Bad faith: The decision to reject the programme in April 2025 and silently implement it in October 2025 — after the Applicant had made protected disclosures and was being subjected to retaliation — demonstrates that the rejection was not based on quality or merit. The programme was always fit for purpose. The rejection was a tactical act of professional suppression; the implementation was an act of appropriation designed to erase the Applicant's authorship.
- (b) Retaliatory intent: The timing of the implementation (October 2025) coincides directly with the period in which the Applicant's protected disclosures (Part G) and formal discrimination complaint (Part F) were being whitewashed. The decision to appropriate the Applicant's intellectual work while simultaneously labelling him "insubordinate" forms part of a single, coherent pattern of retaliation.
- (c) Institutional bad faith supporting aggravated costs: The misappropriation is pleaded in support of the Applicant's prayer for costs on an attorney-and-client scale (prayer R.10). A respondent that rejects an employee's professional work and then secretly implements the same work without credit, while litigating against that employee in bad faith, warrants a punitive costs order.



3. Causal Link to the Respondents' Pattern of Conduct

- 3.1 The misappropriation of the CPD programme is not an isolated act. It forms part of the Respondents' broader pattern of:
 - Suppressing the Applicant's professional contributions (the Priority 1 Material Risk Report);
 - Appropriating his expertise without recognition (the CPD programme);
 - Attacking his professional character under oath (the opposing affidavit of Xolile Vincent Manyathi, addressed in Part K); and
 - Systematically erasing his professional identity (suppressed job title, false Certificate of Service, denial of management status).
- 3.2 This pattern is relevant to the Court's assessment of the intolerability of the employment relationship for purposes of section 186(1)(e) of the LRA, and to the claim for automatically unfair dismissal under section 187(1)(h) of the LRA.

4. Reservation of Rights to Pursue Separate Proceedings

The Applicant expressly reserves his rights to institute separate proceedings for:

- Copyright infringement under section 23 of the Copyright Act 98 of 1978;
- Infringement of moral rights under section 20 of the Copyright Act 98 of 1978;
- Unjustified enrichment under the common law; and
- Any other intellectual property or contractual claim arising from the development, submission, rejection, and subsequent unauthorised implementation of the CPD programme.

PART I: THE RECORDED ADMISSION OF RETALIATORY INTENT

1. The retaliatory conduct evidenced in this section must be read in light of the earlier events set out in Parts G0 and G1. The hostility directed toward the Applicant did not arise only after the formal protected disclosure of October 2025, but forms part of a

- continuous pattern beginning in March 2025 when the Applicant first raised concerns regarding systemic inefficiencies affecting patient care.
2. On 31 October 2025, in a meeting that the Applicant lawfully recorded, Jones stated: “You made a case of harassment against me, so I can make a case of harassment against you. You misspelled my name in an email — go look.” Jones further warned the Applicant to “think of the ripple effect” of his actions as a person who had made formal complaints and protected disclosures. A transcript of the recording is attached as Annexure “O”.
 2. This statement constitutes direct contemporaneous evidence of a causal nexus between the Applicant’s protected activities (formal discrimination complaint; patient safety disclosure) and Jones’s retaliatory intent. It satisfies the causal test for automatically unfair dismissal under section 187(1)(h) of the LRA and for occupational detriment under section 3 of the PDA.
 3. Jones’s warning about the “ripple effect” of the Applicant’s whistleblowing activities constitutes a veiled threat within the meaning of “any act or omission that prejudices the employee” as contemplated in the definition of “occupational detriment” in section 1 of the PDA.



PART J: INSTITUTIONAL SILENCE AND EXHAUSTION OF INTERNAL REMEDIES

1. On 2 November 2025, the Applicant filed a formal grievance against Jones, detailing malicious intent, gaslighting, weaponisation of past issues, orchestrated provocation, and a sustained campaign for his removal. He demanded an investigation, removal from Jones’s team, and a written guarantee against retaliation (Annexure “P”). The Respondents met this grievance with over 30 consecutive days of institutional silence — no acknowledgment, no investigation, no response.
2. On 4 December 2025, the Applicant escalated the matter to executive leadership, including Gerald Van Wyk (CEO, First Respondent), with a 32-page evidence bundle, requesting paid recuperation leave, a financial settlement, and a written guarantee of non-retaliation. This disclosure was de-escalated back to the very department I was complaining about effectively shutting the door on the Applicant (Annexure “Q”).
3. The Chief Risk Officer (Monwabisi Kula) responded on 9 December 2025, acknowledging receipt but stating that interim feedback would only be provided in January 2026 due to the “festive break”. This response, given the humanitarian urgency of the Applicant’s situation (two months in rental arrears, facing eviction, without food or medication funds), constituted a deliberate delay tactic rather than a good-faith engagement.
4. On 9 December 2025, the Applicant escalated to the Board of Directors with 38 attachments documenting discrimination, retaliation, King IV governance violations, and the Priority 1 Material Risk Report (Gexus-DMS integration failure).
5. On 17 December 2025, the Chief Risk Officer sent a further communication instructing the Applicant to “stop sending these e-mails to our directors”, characterising continued escalation as indicative of “impatience and unreasonableness”. The Applicant declined, asserting his right to escalate material risks to oversight committees. This gatekeeping response constitutes evidence of the Board’s deliberate shielding from governance accountability.

6. The Applicant had exhausted all reasonable internal remedies prior to his resignation. The 30+ days of institutional silence following the November 2025 formal grievance, the “festive break” delay, and the Chief Risk Officer’s gatekeeping communications collectively demonstrate the futility of further internal engagement — a relevant consideration under section 186(1)(e) of the LRA for purposes of establishing constructive dismissal.

PART K: THE EMPLOYER’S BAD-FAITH OPPOSING AFFIDAVIT

1. On 10 December 2025, the Respondents, through their legal representative Xolile Vincent Manyathi, filed an opposing affidavit in the CCMA (Case WECT23564-25) (Annexure “R”). The affidavit contained the following material misrepresentations and personal attacks made under oath:
 - (a) The deponent claimed personal knowledge of the Applicant, notwithstanding that the Applicant confirms he has never met, engaged with, or been observed by this individual — a false oath administered before a Commissioner of Oaths;
 - (b) The Applicant’s diagnosed depression and PTSD were dismissed as a “cynical excuse” and an “afterword”;
 - (c) The Applicant was labelled as having an “untruthful character”; and
 - (d) His clinical and professional conduct was described as “sloppiness” and a “fishing expedition”, with a demand that his application be dismissed “with cost”.
2. This affidavit was filed the day after the Applicant’s board-level escalation. The characterisation of a SANC-registered NQF Level 8 nurse’s documented psychiatric condition as a “cynical excuse” in a sworn document constitutes:
 - (a) discrimination on the ground of disability under section 6(1) of the EEA;
 - (b) a violation of the right to dignity under section 10 of the Constitution of the Republic of South Africa, 1996; and
 - (c) evidence of the bad faith that pervaded the entire employment relationship and that renders the employer’s conduct unlawful for purposes of the costs prayer.
3. The filing of this affidavit, combined with the cumulative conduct described above, eliminated any objective possibility of restoring the employment relationship and constituted the final act rendering continued employment intolerable within the meaning of section 186(1)(e) of the LRA.



PART L: RESIGNATION — AUTOMATICALLY UNFAIR CONSTRUCTIVE DISMISSAL

1. On 11 December 2025, the Applicant gave formal notice of resignation, effective 12 December 2025, citing the employer’s sworn affidavit as the final intolerable act. The resignation letter is attached as Annexure “S”.
2. The employment situation had become objectively intolerable for the purposes of section 186(1)(e) of the LRA due to the cumulative effect of the following:
 - (a) The unlawful salary deduction of 40.47%, made without consultation and in breach of the statutory cap, while the employer was in possession of the Applicant’s psychiatric assessment confirming medical vulnerability;

- (b) Discrimination and harassment on the grounds of gender identity and sexual orientation (the “Wilma” incident), followed by a whitewash investigation;
 - (c) Retaliation and insubordination labelling following a bona fide protected disclosure of a patient safety risk;
 - (d) The recorded admission of retaliatory intent by Jones on 31 October 2025;
 - (e) Over 30 days of institutional silence on formal grievances, demonstrating the futility of internal remedies;
 - (f) The executive “festive break” delay and the Chief Risk Officer’s gatekeeping of Board-level escalation;
 - (g) The sworn, defamatory affidavit attacking the Applicant’s documented psychiatric condition and professional character; and
 - (h) The Board’s failure to respond substantively to the Priority 1 Material Risk Report.
3. The Applicant’s resignation constitutes a constructive dismissal within the meaning of section 186(1)(e) of the LRA. The dominant reason for the dismissal — pervading all of the conduct described above — is the Respondents’ retaliation against the Applicant for making protected disclosures as defined in the PDA. The dismissal is accordingly automatically unfair in terms of section 187(1)(h) of the LRA.
4. Alternatively, if the Court finds that the dismissal does not fall within section 187(1)(h), the Applicant contends that the dismissal was automatically unfair under section 187(1)(f) of the LRA in that the reason for the dismissal relates to the Applicant’s disability (diagnosed psychiatric condition) — an enumerated ground under section 6 of the EEA.
5. Further alternatively, if the Court finds that the dismissal is not automatically unfair, the Applicant submits that it was substantively and procedurally unfair within the meaning of section 185 of the LRA, for all the reasons set out above.
6. The Applicant submits that the cumulative effect of: early-stage intimidation following the March 2025 disclosure (Part G0), continued hostility and resistance to his concerns (Part G1), and subsequent retaliatory conduct (Part I), rendered the employment relationship objectively intolerable, leaving the Applicant with no reasonable alternative but to resign.



PART M: CAREER DESTRUCTION — INABILITY TO PRACTISE NURSING

1. As a direct and proximate result of the unlawful 40.47% salary deduction in July 2025, the Applicant was rendered financially destitute. He could not afford to pay his annual registration fee to the South African Nursing Council (“SANC”) and consequently his nursing registration was not renewed.
2. As a result, the Applicant is currently barred from practising as a registered nurse — constituting a deprivation of his professional livelihood and career destruction.
3. The Applicant had been accepted into the Postgraduate Diploma in Occupational Health Nursing at the Cape Peninsula University of Technology (“CPUT”) for the 2026 academic year. Due to the Respondents’ financial sabotage, he could not pay the registration fee and lost that professional development opportunity. The acceptance letter is attached as Annexure “T”.
4. The loss of nursing registration and the lost CPUT opportunity constitute recoverable consequential damages caused by the unlawful deduction and retaliatory conduct. They are pleaded as damages under section 4 of the PDA (occupational detriment) and under the general damages jurisdiction of this Court.

PART N: POST-RESIGNATION VICTIMISATION

1. After the Applicant's resignation, the Respondents continued to cause him harm through the following acts:
 - (a) UIF Non-Declaration: UIF contributions were deducted from the Applicant's June 2025 salary but the Respondents failed to declare his employment for that month. The uFiling portal continues to reflect 'No Information' for June 2025 (Annexure "U"). This constitutes a contravention of section 5 of the Unemployment Insurance Contributions Act 4 of 2002, as well as potential fraud under section 68 of the Unemployment Insurance Act 63 of 2001;
 - (b) Misrepresentation of Certificate of Service: The Certificate of Service designates the Applicant as 'Care Coordinator' rather than 'Care Manager' or 'Health Services Management Nurse Specialist'. The Respondents have refused to correct this despite the Applicant's formal demand and have withheld the recording of the internal 'Naming Convention' meeting (Annexure "V"). This constitutes a contravention of section 42 of the BCEA; and
 - (c) Withholding of Exit Documents: The Respondents delayed providing the UI-19 form and retirement fund claim forms, causing additional financial prejudice to an already destitute Applicant.



PART O: REGULATORY AND THIRD-PARTY CONFIRMATION OF MATERIAL FACTS

1. The following regulatory and institutional acknowledgments independently corroborate the material facts set out in this Statement of Claim:
 - (a) Council for Medical Schemes (CMS): Formally identified 'Administrative Misconduct and Ethical Failure' in relation to the Gexus/DMS integration gap and referred the matter to HPCSA and SANC (Annexure "W");
 - (b) South African Human Rights Commission (SAHRC): Acknowledged the 'seriousness of the allegations' regarding victimisation and financial prejudice, and made referrals to the CCMA, Labour Court, and Department of Employment and Labour (Annexure "X");
 - (c) JSE Issuer Regulation Division: Acknowledged receipt of the Applicant's 325-page evidentiary bundle and has engaged the First Respondent for a response regarding potential SENS disclosure failures;
 - (d) B-BBEE Commission: Allocated Case No. 5/02/2026 for investigation of the B-BBEE fronting allegation;
 - (e) Department of Employment and Labour: Allocated an integrated inspection for UIF non-compliance (DEL IES Ref, 16 February 2026); and
 - (f) SAPS: Criminal case CAS 705/3/2026 registered at SAPS Delft on 19 March 2026 and transferred to SAPS Woodstock on 25 March 2026 for investigation of statutory fraud under the Unemployment Insurance Contributions Act and the BCEA.
2. A full Master Chronological Register containing 148 entries from 11 January 2024 to 2 April 2026, summarising all material events, institutional responses, and patterns of deflection, is attached as Annexure "Z".

PART P: SUMMARY OF CAUSES OF ACTION

The Applicant's causes of action are as follows:

- (a) Primary claim: Automatically unfair constructive dismissal under sections 186(1)(e) and 187(1)(h) of the LRA — the dominant reason for the dismissal was retaliation for protected disclosures as defined in the PDA;
- (b) Alternative claim A: Automatically unfair dismissal under sections 186(1)(e) and 187(1)(f) of the LRA — the dominant reason relates to the Applicant's disability;
- (c) Alternative claim B: Constructive and substantively/procedurally unfair dismissal under sections 185 and 186(1)(e) of the LRA;
- (d) Unfair discrimination and harassment under sections 6(1), 6(3) and 60 of the EEA — on the grounds of sexual orientation, gender identity and disability;
- (e) Occupational detriment for making protected disclosures under sections 3 and 4 of the PDA;
- (f) Contravention of section 34(2) of the BCEA — unlawful salary deduction (also pleaded in Case No. 2026-077546);
- (g) Contravention of section 42 of the BCEA — failure to provide a true and accurate Certificate of Service; and
- (h) Contravention of section 5(1) of the Unemployment Insurance Contributions Act 4 of 2002 — failure to declare contributions for June 2025.



PART Q: QUANTIFICATION OF LOSS

Head of Damage / Claim	Statutory Basis	Amount (ZAR)
Automatically unfair dismissal compensation (24 months × R42,796.13)	LRA s194(3)	R1,027,107.12
Unlawful salary deduction — principal amount	BCEA s34(2) / s77A	R14,811.77
Interest on unlawful deduction (prescribed rate from 23 Jul 2025)	Prescribed Rate Act	To be calculated
EEA — solatium for injury to dignity (sexual orientation / disability discrimination)	EEA s50(2)	R350,000.00
EEA — compensation for failure to reasonably accommodate	EEA s50(2)	To be assessed
PDA — compensation for occupational detriment (financial loss, SANC registration, CPUT)	PDA s4	To be assessed
Consequential damages: lost CPUT programme (registration fees and costs)	PDA s4 / general damages	To be quantified
Career destruction: loss of nursing registration and future earning capacity	PDA s4 / general damages	To be assessed
TOTAL (known components)		R1,391,918.89 plus assessed amounts

Items marked 'to be assessed' or 'to be quantified' will be supplemented by expert evidence and/or further particulars as the matter develops. The Applicant reserves the right to amend the quantum of the claim upon receipt of further documentation from the Respondents.

PART R: RELIEF SOUGHT

A. Declaratory Relief

- R.1.** A declaration that the Applicant's dismissal constitutes an automatically unfair constructive dismissal within the meaning of sections 186(1)(e) and 187(1)(h) of the LRA, on the grounds that the dominant reason for the dismissal was retaliation by the Respondents for the Applicant making protected disclosures as defined in the PDA.
- R.2.** Alternatively to R.1, a declaration that the dismissal was automatically unfair within the meaning of sections 186(1)(e) and 187(1)(f) of the LRA on the grounds that a dominant reason for the dismissal relates to the Applicant's disability.
- R.3.** Further alternatively, a declaration that the dismissal was unfair within the meaning of section 185 of the LRA.



B. Compensation and Damages

- R.4.** An order that the First and/or Second Respondents pay the Applicant compensation equal to 24 months' remuneration as provided for in section 194(3) of the LRA for an automatically unfair dismissal, calculated at his monthly cost-to-company of R42,796.13 per month, amounting to R1,027,107.12, alternatively the maximum permissible compensation amount.
- R.5.** An order that the First and/or Second Respondents reimburse the Applicant the unlawfully deducted amount of R14,811.77 together with interest at the prescribed rate calculated from 23 July 2025 to the date of final payment, alternatively that this relief be granted in Case No. 2026-077546.
- R.6.** An order that the First and/or Second Respondents pay the Applicant compensation in terms of section 50(2) of the EEA for unfair discrimination on the grounds of sexual orientation, gender identity and disability, including solatium for injury to dignity in the amount of R350,000, alternatively such amount as this Court deems just and equitable.
- R.7.** An order that the First and/or Second Respondents pay the Applicant compensation for occupational detriment under section 4 of the PDA, including compensation for all financial loss, career destruction, loss of nursing registration, and the lost opportunity to pursue the CPUT Postgraduate Diploma in Occupational Health Nursing, in amounts to be assessed or proved.

C. Corrective Orders

- R.8.** An order directing the First and/or Second Respondents to issue a corrected Certificate of Service reflecting the Applicant's true designation as Health Services Management Nurse Specialist / Case Manager (NQF Level 8) in compliance with section 42 of the BCEA, within 10 business days of the grant of this order.
- R.9.** An order directing the First and/or Second Respondents to remit the outstanding UIF contributions for June 2025 to the Unemployment Insurance Fund and to provide the Applicant with a corrected UI-19 form within 10 business days, in compliance with the Unemployment Insurance Contributions Act 4 of 2002 and section 37 of the Unemployment Insurance Act 63 of 2001.

D. Costs

R.10. Costs of suit on an attorney-and-client scale, in terms of section 162 of the LRA and the inherent jurisdiction of this Court, having regard to:

- (a) the Respondents' bad faith conduct, including the filing of a defamatory opposing affidavit attacking the Applicant's documented psychiatric condition under oath;
- (b) the systematic institutional cover-up, whitewash investigations, and 30+ days of deliberate institutional silence;
- (c) the post-resignation victimisation through misrepresentation of the Certificate of Service and UIF non-declaration; and
- (d) the Chief Risk Officer's gatekeeping communications designed to prevent Board-level accountability.

E. Further and/or Alternative Relief

R.11. Further and/or alternative relief as this Honourable Court deems just and equitable in the circumstances.



RESERVATION OF RIGHTS REGARDING PERSONAL LIABILITY

The Applicant expressly reserves the right to apply for joinder of any director, officer, or employee of the Respondents (including Bruno Fernandes, Dr Nkateko Munisi, Alice le Roux, Gerald Van Wyk, LSM Mpumlwana, Mary-Ann Jones, Mapule Mashele, Xolile Vincent Manyathi, and Lisa Mari) in the event that the Court orders a personal liability enquiry under section 162 of the Companies Act 71 of 2008, section 60 of the EEA, or section 4 of the PDA, or in any separate proceedings as the evidence may warrant. No criminal or perjury proceedings are sought in this Statement of Claim.

PART S: VERIFICATION AND DECLARATION

I, WILBUR WILLIAM MATTHEE, the Applicant herein, declare that the facts set out in this Statement of Claim are within my personal knowledge, save where indicated to the contrary, and are to the best of my knowledge true and correct.

Signature	
Full Name	WILBUR WILLIAM MATTHEE
Identity Number	890903 5151 087
Capacity	Applicant (Self-Represented)
Email	info@ethichawks.co.za
Telephone	069 635 1435 / 063 378 8782
Date	9 April 2026, Cape Town

SCHEDULE OF ANNEXURES

Tab	Description	Date
"A"	Interview invitations for 'Specialist: Care Manager' role	January 2024
"B"	Assessment instructions — Specialist: Care Manager	29 January 2024
"C"	Fingerprint criminal check booking confirmation	21 February 2024
"D"	Employment contract — 'Care Coordinator' (Grade B4M1)	9 April 2024
"E"	First Aider roster and EHS directives drafted by Applicant	25 July 2025
"F"	Sick note — Dr Jacques Malan (Akeso Stepping Stones Clinic)	1 April 2025
"G"	Mental health disclosure to Mapule Mashele (formal written notice of disability)	16 May 2025
"H"	Jones: "we do not apply staggering of money owed"	16 April 2025
"I"	Payslip — July 2025 (R14,811.77 deduction; 40.47% of gross salary)	July 2025
"J"	AOD Policy confirming 25% cap + "what is illegal?" exchange	24 July 2025
"K"	"Wilma" Microsoft Teams message (screenshot, team-wide channel)	23 September 2025
"L"	Formal discrimination complaint lodged with HR	25 September 2025
"M"	Mashele investigation closure report ("typo" finding)	1 October 2025
"N"	Gexus/DMS root cause analysis and protected disclosure	23 October 2025
"O"	Transcript — 31 October 2025 meeting (retaliatory admission by Jones)	31 October 2025
"P"	Formal grievance against Jones	2 November 2025
"Q"	Executive escalation bundle to leadership (32-page evidence bundle)	4 December 2025
"R"	Employer's opposing affidavit — CCMA (Xolile Vincent Manyathi)	10 December 2025
"S"	Resignation letter	11 December 2025
"T"	CPUT Postgraduate Diploma acceptance letter	2026
"U"	uFiling screenshot — 'No Information' for June 2025	2026
"V"	Certificate of Service (disputed) + Naming Convention request	January 2026
"W"	CMS referral — 'Administrative Misconduct and Ethical Failure'	2026
"X"	SAHRC acknowledgment and referrals	2026
"Y"	CCMA Certificates of Non-Resolution (WECT23564-25 and WECT24486-25)	9 January 2026
"Z"	EthicHawks Master Chronological Register (148 entries, 11 Jan 2024 – 2 Apr 2026)	3 April 2026
"P1 RISK"	NQF Mandate / SAQA Statutory Obligation to Report (ELO mapping; qualification transcript)	8 April 2026
"PDA BILL"	Protected Disclosures Amendment Bill, 2026 (Government Gazette, 8 April 2026)	8 April 2026



END OF STATEMENT OF CLAIM



ANNEXURE

“A”

Interview invitations for ‘Specialist: Care Manager’ role

Date: January 2024

Relevance: *Part C — Employment Relationship*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



Interview: Specialist Care Manager (Wilbur William Matthee)

2 messages

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com> Thu, 11 Jan 2024 at 15:47
To: wilburmat@gmail.com <wilburmat@gmail.com>, Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Gretchen Veldtman <Gretchen.Veldtman@afrocentrichealth.com>

Good day Wilbur

We look forward to meeting with you for a face-to-face interview for the **Specialist: Care Manager Role** at Medscheme. J



- o **Date:** 15 January 2024
- o **Time:** 12:00
- o **Venue:** [Searle street, The Boulevard](#), 4th Floor, Cape Town.

You are to please enter **4th Floor**, you will be meeting with **Nthabiseng Nkatane**.

If you have any questions, please do not hesitate to contact me. Moreover, kindly please accept or decline this invitation.

Kind Regards

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Wilbur William Matthee <wilburmat@gmail.com>
To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>
Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Gretchen Veldtman <Gretchen.Veldtman@afrocentrichealth.com>

Thu, 11 Jan 2024 at 17:10



Interview: Specialist Care Manager (Wilbur William Matthee)

2 messages

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com> Fri, 12 Jan 2024 at 08:06
To: wilburmat@gmail.com <wilburmat@gmail.com>, Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Gretchen Veldtman <Gretchen.Veldtman@afrocentrichealth.com>

Good day Wilbur

We look forward to meeting with you for a face-to-face interview for the **Specialist: Care Manager Role** at Medscheme. J



- o **Date:** 15 January 2024
- o **Time:** 12:00
- o **Venue:** [Searle street, The Boulevard](#), 4th Floor, Cape Town.

You are to please enter **4th Floor**, you will be meeting with **Nthabiseng Nkatane**.

If you have any questions, please do not hesitate to contact me. Moreover, kindly please accept or decline this invitation.

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To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>
Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Gretchen Veldtman <Gretchen.Veldtman@afrocentrichealth.com>

Fri, 12 Jan 2024 at 09:44



Interview: Specialist Care Manager (Wilbur William Matthee)

2 messages

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com> Mon, 15 Jan 2024 at 12:49
To: wilburmat@gmail.com <wilburmat@gmail.com>, Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Gretchen Veldtman <Gretchen.Veldtman@afrocentrichealth.com>
Cc: Susan Mostert <susanm@medscheme.co.za>

Good day Wilbur

We look forward to meeting with you for a face-to-face interview for the **Specialist: Care Manager Role at Medscheme**. J



- o **Date:** 18 January 2024
- o **Time:** 11:00
- o **Venue:** Searle street, The Boulevard, 4th Floor, Cape Town.

You are to please enter **4th Floor**, you will be meeting with **Nthabiseng Nkatane**.

If you have any questions, please do not hesitate to contact me. Moreover, kindly please accept or decline this invitation.

Kind Regards

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976 KB

Wilbur William Matthee <wilburmat@gmail.com>
To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>
Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Gretchen Veldtman <Gretchen.Veldtman@afrocentrichealth.com>, Susan Mostert <susanm@medscheme.co.za>

Mon, 15 Jan 2024 at 12:55

I will be attending the interview.
[Quoted text hidden]

ANNEXURE

“B”

Assessment instructions — Specialist: Care Manager

Date: 29 January 2024

Relevance: *Part C — Employment Relationship*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



13/4/2026-11:58:23 AM

Vacancy - Specialist Care Manager

6 messages

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com> Mon, 29 Jan 2024 at 08:36

Good day

Thank you for showing interest in the abovementioned position. As a part of our hiring process, we send out assessments related to the position to test your skills.

You will find an attachment to this mail covering all the instructions related to the assessment. You will also find the link to the assessment in the attachment. In order to complete the assessment successfully, you will need to go through the instructions carefully.



The due date of the assessment is **30 January 2024** @ 12am/Midday.

Don't forget to send pivotal assessment document.

Wishing you all the best!



Neo Sekgoka
Human Capital Intern

- +27 (011) 671 4819
- neo.sekgoka@afrocentrichealth.com

- 37 Conrad St, Florida North, Roodepoort | PO Box 1101, Florida Glen, 1709
- www.afrocentric.za.com

13/4/2026-11:58:23 AM



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6 attachments

 **image001.png**
1 KB

 **image002.png**
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 **image003.png**
1 KB

 **image004.png**
1 KB

 **image005.png**
29 KB

 **PT Assessment Instructions_ Medscheme (002) (Case manager etc.).docx**
15 KB

13/4/2026-11:58:23 AM

Wilbur William Matthee <wilburmat@gmail.com>
To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

Mon, 29 Jan 2024 at 09:33

Thank you, will do.
[Quoted text hidden]

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>
To: Wilbur William Matthee <wilburmat@gmail.com>

Mon, 29 Jan 2024 at 11:06

May you kindly send me everything I have requested for before the end of business day

[Quoted text hidden]
[Quoted text hidden]



Wilbur William Matthee <wilburmat@gmail.com>
To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

Mon, 29 Jan 2024 at 12:02

Thank you, will do.
[Quoted text hidden]

Wilbur William Matthee <wilburmat@gmail.com>
To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

Mon, 29 Jan 2024 at 14:14

Hello Neo

I hope you are well. I have just completed the online assessment as requested. I will submit the other required documents now.

My MIT fingerprint test was done in December 2023, i have the report and will send that to you on request. I have to update my CV with regards to the statement I made "criminal record" clear, in the report it states that i have a pending trial for pending blood results for the level of alcohol in my system. I do not want this statement to determine the success of my application so I have provided you with the updated CV.

thank you so much for your assistance with this.

Regards,
Sr. W. Matthee
[Quoted text hidden]

 **WillCvJan update.pdf**
752 KB

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>
To: Wilbur William Matthee <wilburmat@gmail.com>

Mon, 29 Jan 2024 at 14:20

May you kindly fill in the consent form as we do our own fingerprint checks. We do not allow any fingerprint checks that were done by the candidate themselves unless otherwise specific.

[Quoted text hidden]
[Quoted text hidden]

ANNEXURE

“C”

Fingerprint criminal check booking confirmation

Date: 21 February 2024

Relevance: *Part C — Employment Relationship*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



13/4/2026-11:58:23 AM

Vacancy - Specialist Care Manager

4 messages

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>
To: Wilbur William Matthee <wilburmat@gmail.com>

Mon, 29 Jan 2024 at 14:39

Good day

May you kindly fill this in

Warm regards



Neo Sekgoka

Human Capital Intern

- +27 (011) 671 4819
- neo.sekgoka@afrocentrichealth.com
- [37 Conrad St, Florida North, Roodepoort](#) | PO Box 1101, Florida Glen, 1709
- www.afrocentric.za.com

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6 attachments

 **image001.png**
1 KB

 **image002.png**
1 KB

 **image003.png**
1 KB

 **image004.png**
1 KB

 **image005.png**
29 KB

 **MIE Processing Notification.pdf**
171 KB

13/4/2026-11:58:23 AM

Wilbur William Matthee <wilburmat@gmail.com>
To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>

Mon, 29 Jan 2024 at 14:54

<https://app.luminpdf.com/viewer/65b79b5d4b7608a70677500b>

I have completed the form and left out the blanks for your perusal.

Thank you

[Quoted text hidden]

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>
To: Wilbur William Matthee <wilburmat@gmail.com>

Mon, 29 Jan 2024 at 15:25



I cannot access the form

[Quoted text hidden]

[Quoted text hidden]

Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>
To: Wilbur William Matthee <wilburmat@gmail.com>

Mon, 29 Jan 2024 at 15:58

May you kindly send me the form without any link

From: Wilbur William Matthee <wilburmat@gmail.com>
Sent: Monday, January 29, 2024 2:55 PM
To: Neo Sekgoka <Neo.Sekgoka@afrocentrichealth.com>
Subject: Re: Vacancy - Specialist Care Manager

<https://app.luminpdf.com/viewer/65b79b5d4b7608a70677500b>

[Quoted text hidden]

[Quoted text hidden]

[Quoted text hidden]



COMPANY DETAILS "Company"

To be completed by Company Agent

Company Name: _____ Email: _____
 Agent Name: _____ Mobile No: _____

CANDIDATE PERSONAL INFORMATION

To be completed by the Candidate

Surname: **MATTHEE**
 Full Names: **WILBUR WILLIAM** Maiden Name: _____
 Mobile Number: **069 490 9914** Date of Birth: **03/09/1989**
 ID Number / Identifier:

8	9	0	3	0	9	5	1	5	1	0	8	7
---	---	---	---	---	---	---	---	---	---	---	---	---

Description of Identifier: **SOUTH AFRICA ID NUMBER**

E.g. South African ID Number, Zimbabwean Passport Number, etc.

Physical Address: **15 BELLEVUE STREET, VOORBRUG, DELFT, 7100**Previous Convictions: Yes ☐ No ☒

If Yes, Please Provide the Conviction Details:

Date Convicted: _____
 Offence: _____
 Sentence: _____

BACKGROUND SCREENING CHECKS

To be completed by the Company Agent

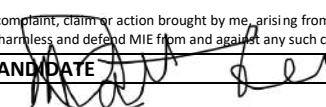
☐ Credit Check	☐ Sanctions
☐ Qualification	☐ Identity Verifications
☐ Employment References	☐ Insurance Regulations
☐ Criminal Checks	☐ Drivers license & Vehicles
☐ Fraud Check	☐ Social Media Screening Checks

DEFINITIONS

- ✓ "Candidate" means the person on whom the Company will process background screening checks for lawful purposes including but not limited to employment/ continuation of employment;
- ✓ "Company" refers to MIE Client;
- ✓ "Consumer Credit Information" shall have the meaning ascribed to it in section 70 of the NCA;
- ✓ "FAIS Act" shall mean the Financial Advisory and Intermediary Services Act of 2002;
- ✓ "FSCA" refers to the Financial Sector Conduct Authority;
- ✓ "NCA" shall mean the National Credit Act, No. 34 of 2005, as amended from time to time, including any regulations made under the Act;
- ✓ "Personal Information" shall have the meaning ascribed to it in Chapter 1 of POPI and includes, but is not limited to a name, address, email address, telephone or fax number, fingerprints, criminal history and education or other personal credentials provided, or which is collected from the candidate or other third parties, before and/or during the background screening process and/or thereafter;
- ✓ "POPI" shall mean the Protection of Personal Information Act, No. 4 of 2013, as amended from time to time, including any regulations made under the Act;
- ✓ "Privacy and Data Protection Conditions" refers to the 8 (eight) statutory prescribed conditions for the lawful Processing of Personal Information;
- ✓ "Responsible Parties" have meaning to the Company and MIE together, and "Responsible Party" any one of them;
- ✓ "Verification Information Suppliers" shall mean third parties acting on behalf of MIE, including, but not limited to, criminal record bureaus, credit bureaus, governmental bodies, and any educational, training, and fraud prevention organisations;

ACKNOWLEDGMENT OF THE USE OF PERSONAL INFORMATION

- I acknowledge
- ✓ that the Company's duly authorized verification agent, Managed Integrity Evaluation (Pty) Ltd ("MIE"), will need to process my Personal Information to conduct background screening checks as indicated above which are required by the Company.
 - ✓ that verification requests form part of the background screening process and that:
 - ✓ requests for credit information from Credit Bureaus will only be conducted under the regulations defined as per the NCA and for the below prescribed purposes only
 - ✓ For employment in a position of trust and honesty and entails the handling of cash or finances;
 - ✓ Fraud prevention or detection.
 - ✓ data obtained from the FSCA serve only for the purpose to determine the fitness and propriety as envisaged in the FAIS Act.
 - ✓ that any Personal Information supplied to the Company is provided voluntarily and is accurate and current as the Company may not be able to comply with its obligations if the correct Personal Information is not supplied to the Company, I further agree to correct and update such information when necessary;
 - ✓ that privacy is important to the Responsible Parties and the Responsible Parties will use reasonable efforts in order to ensure that any Personal Information in their possession or processed on their behalf is kept confidential, stored in a secure manner, and processed in terms of South African law and or applicable Data Protection Legislation, for the purposes I have authorised.
 - ✓ that all information, including Personal Information, supplied to the Company is accurate and current and agree to correct and update such information when necessary.
 - ✓ may be shared by the Company with MIE and may be further shared by MIE with the Verification Information Suppliers for verification or other legitimate purposes;
 - ✓ may be shared by the Verification Information Suppliers with MIE and be further stored and shared by MIE with the Company and Fraud databases or services when the information provided for verification is deemed fraudulent by the Verification Information Suppliers for purposes of continued or future employment or for other legitimate purposes as per the NCA or other National or Provincial Legislation;
 - ✓ may be stored by MIE, for legislated retention periods and where such periods are not legislated then my personal information may be stored by MIE for as long as the information may be needed for verification purposes, or any other period as I may agree with the MIE. and
 - ✓ Personal Information may be transferred cross-border to countries, for verification or storage purposes. In any cross-border transfer of personal information MIE will comply with the security safeguards as provided for in the POPIA including but not limited to ensuring that the information is secured when transported to or from the recipient.
 - ✓ I take note that if the Responsible Party has utilised the Personal Information contrary to the Privacy and Data Protection Conditions, I may first resolve any concerns with that Responsible Party. If I am not satisfied with such process, I have the right to lodge a complaint with the Information Regulator.
 - ✓ A copy of Personal Information kept by the Responsible Parties will be furnished to me upon request in terms of the provisions of POPI or the NCA and I understand that I may dispute any information in the record provided.
 - ✓ "I" agree that MIE, to the extent permitted by law, will not be liable for any complaint, claim or action brought by me, arising from any action or omission by MIE, to the extent that such action or omission resulted from MIE complying with the terms of this agreement and I shall indemnify, hold harmless and defend MIE from and against any such claims or actions brought against MIE."

SIGNATURE	CANDIDATE 	COMPANY AGENT
DATE	29/01/2024	

All signatories to this document agree that MIE will NOT be held liable for the content, factual correctness or accuracy of any Supplier Data supplied to MIE for the Company by MIE's suppliers. The Company and the Candidate hereby indemnifies and holds MIE harmless against any loss arising from neglect or damage in procuring, communicating, or failing to communicate information to the Company.

ANNEXURE

“D”

Employment contract — ‘Care Coordinator’ (Grade B4M1)

Date: 9 April 2024

Relevance: *Part C — Employment Relationship*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



09 April 2024

W Matthee
C/O AFROCENTRIC HEALTH (RF) (Pty) Ltd

Dear Wilbur,

INTERNAL APPOINTMENT: Care Coordinator

We are pleased to offer you the position Care Coordinator (Reference IRC78739) within AfroCentric Health (RF) (Pty) Ltd with effect from 01 May 2024. This position is situated at Cape Town.

The Key elements of your duties and responsibilities will be as discussed with you and may be revised from time to time thereafter.

CONDITIONS OF EMPLOYMENT

1. PROBATION

You will be required to successfully complete a probationary period of **six (6)** continuous months in this new position. In terms of the Group Talent Acquisition Policy, you will not be permitted to apply for any vacant position internally during this probation period.

2. REMUNERATION

We are pleased to advise you that your total package on a cost to company basis will be **R490,032.00 per annum**, payable by bank transfer into your Bank account.

The above cost to company figure includes all your contributions towards Medical Aid, Pension Fund, Group Life Assurance and Disability Assurance. If you are required to use your cell phone for business purposes, this will be deemed to be part of your cost to company package.

The Grade for your position based on our Grading Model is that of **B4M1**.

3. RETIREMENT FUNDING INCOME

www.afrocentric.za.com



AfroCentric Investment Corporation Limited Reg #1988/000570/06
T +27 11 671 2000
 PO Box 1101, Florida Glen 1708 | 37 Conrad Street | Florida North | Roodepoort 1709



DIRECTORS:

Dr ATM Mokgokong (Chairman), MJ Madungandaba (Deputy Chairman), GN Van Wyk (CEO), JW Boonzaaier (CFO), M Chauke, MK Dippenaar, JB Fernandes, PB Hanratty, AM le Roux, KN Mkhize, Dr ND Munisi

Group Company Secretary: LSM Mpumlwana

A percentage between **78** and **90** (78 / 80 / 85 / 90) of your total cost to company figure, as selected by yourself, will constitute your "**Retirement Funding Income**". All contributions, both employer and employee, to the Pension / Provident Fund, Group Risk Benefits such as Life and Disability Schemes will be based on this figure.

4. LEAVE ENTITLEMENT

Your annual leave entitlement will be **seventeen (17)** working days per annum.

5. HOURS OF WORK

You will be required to work **forty-five (45) hours per week**. Your working hours are flexible and will be scheduled by the Business Unit and may include a Saturday which is deemed part of your normal working week. You will be entitled to a thirty (30) minute lunch break as scheduled. Any work performed on a Saturday will be paid at normal rates. Your working hours will be determined by the requirements of the Business Unit to which you are assigned to from time to time and may be amended to suit the operational requirements of the Business Unit.

It is a condition of employment that, as and when the need arises, you will be required to work overtime. Payment, or time off in lieu of overtime worked, hours more than forty-five (45) working hours per week, will be made in accordance with the Human Capital Payroll and Benefits Management Policy.

6. OTHER CONDITIONS OF EMPLOYMENT

All other conditions of employment not specifically mentioned in this letter remain unchanged.

7. PACKAGE RESTRUCTURING

Please feel free to have a package structuring calculation/discussion with your Line Manager or the HC Business Partner responsible for your Business Unit before accepting this offer of employment. Package restructuring takes place during June/July.

Candidate Initial _____ GM Initial BG

8. OFFER ACCEPTANCE

Kindly sign and return the copy of this letter to your HC Business Partner. Also note that this offer is valid for three (3) days from receipt.

Please initial every page and sign in full on the last page of this document before returning to your HC Business Partner.

We would like to take this opportunity to wish you well in your new position.

Yours sincerely,

B Johnson

Barbara Johnson
General Manager: GEMS Operations.5
AfroCentric Health (RF) (Pty) Ltd

18 April 2024

Date Signed



Candidate Initial _____ GM Initial *BJ*

I ACCEPT OFFER extended to me for the position of Care Coordinator within AfroCentric Health (RF) (Pty) Ltd at Roodepoort. I will take up the position on 01 May 2024.

I hereby acknowledge that I have read the contents of this document and fully understand and agree to abide by the provisions thereof. I also acknowledge that the contents of this document are confidential, and I will not disclose the contents to any party within the working environment.

I acknowledge that I have not been induced to complete this contract by any representations made to me by any person/s and hereby confirm that I have entered into this contract of my own free will.

By accepting this offer of appointment, I undertake to keep abreast with updates pertaining to all company policies and procedures regularly and adhere to them.



W Matthee

DATE SIGNED

Candidate Initial _____ GM Initial _____

BG

ANNEXURE

“E”

First Aider roster and EHS directives drafted by Applicant

Date: 25 July 2025

Relevance: *Part C — Employment Relationship*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



Updated First Aid Daily Roster & Backup Plan

1 message

Wilbur William Matthee <wilburmat@gmail.com>
To: Wilbur Matthee <wilbur.matthee@medscheme.co.za>

Mon, 05 May 2025 at 10:24

Dear First Aid Team,

I hope this message finds you well.

Please be informed that we have implemented a new daily roster for all First Aiders to ensure fair distribution of duties across all floors. This change comes in response to recent observations that only certain First Aiders are being contacted during emergencies. The new roster aims to ensure equal participation and support throughout the building.



Each day will have a designated First Aider, as well as a backup person, in case the primary is not available.

- Important:
- If you are scheduled for leave this month, please indicate your leave dates on the roster.
 - Kindly share the updated roster with your team leaders so they are aware of your allocated days.
 - If you are the backup, please make sure the scheduled First Aider is on duty. If not, be prepared to step in.

Your cooperation is greatly appreciated in making sure that all areas are covered and that we respond swiftly in the event of an emergency.

Thank you for your commitment and continued support.

Best regards,

First Aider Allocation Roster: July 2025 (Weekdays Only)



Second Floor

First Aiders: Nigel Abrams, Adeline Appolles, Wilbur Matthee, Ntokozo Nxumalo (Ntokozo Nxumalo is WFH indefinitely)

Date (July 2025)	Primary First Aider	Backup First Aider	Notes
Wednesday, 2	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Thursday, 3	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Friday, 4	Adeline Appolles	Wilbur Matthee	Ntokozo Nxumalo WFH
Monday, 7	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Tuesday, 8	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Wednesday, 9	Adeline Appolles	Wilbur Matthee	Ntokozo Nxumalo WFH
Thursday, 10	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Friday, 11	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Monday, 14	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Tuesday, 15	Adeline Appolles	Wilbur Matthee	Ntokozo Nxumalo WFH
Wednesday, 16	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Thursday, 17	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Friday, 18	Adeline Appolles	Wilbur Matthee	Ntokozo Nxumalo WFH
Monday, 21	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Tuesday, 22	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Wednesday, 23	Adeline Appolles	Wilbur Matthee	Ntokozo Nxumalo WFH
Thursday, 24	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Friday, 25	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Monday, 28	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH
Tuesday, 29	Adeline Appolles	Wilbur Matthee	Ntokozo Nxumalo WFH
Wednesday, 30	Nigel Abrams	Adeline Appolles	Ntokozo Nxumalo WFH
Thursday, 31	Wilbur Matthee	Nigel Abrams	Ntokozo Nxumalo WFH

Third Floor

First Aiders: Tarryn Newman, Lucian Geswindt, Robin Lang, Zakee Ebrahim (Tarryn

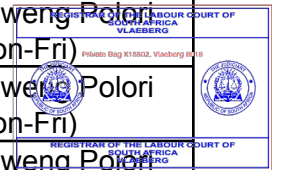
Newman on annual leave from July 7th to 11th)

Date (July 2025)	Primary First Aider	Backup First Aider	Notes
Wednesday, 2	Tarryn Newman	Lucian Geswindt	
Thursday, 3	Lucian Geswindt	Robin Lang	
Friday, 4	Robin Lang	Zakee Ebrahim	
Monday, 7	Lucian Geswindt	Robin Lang	Tarryn Newman on AL (Mon-Fri)
Tuesday, 8	Robin Lang	Zakee Ebrahim	Tarryn Newman on AL (Mon-Fri)
Wednesday, 9	Zakee Ebrahim	Lucian Geswindt	Tarryn Newman on AL (Mon-Fri)
Thursday, 10	Lucian Geswindt	Robin Lang	Tarryn Newman on AL (Mon-Fri)
Friday, 11	Robin Lang	Zakee Ebrahim	Tarryn Newman on AL (Mon-Fri)
Monday, 14	Tarryn Newman	Lucian Geswindt	Tarryn Newman returns from AL
Tuesday, 15	Lucian Geswindt	Robin Lang	
Wednesday, 16	Robin Lang	Zakee Ebrahim	
Thursday, 17	Zakee Ebrahim	Tarryn Newman	
Friday, 18	Tarryn Newman	Lucian Geswindt	
Monday, 21	Zakee Ebrahim	Tarryn Newman	
Tuesday, 22	Tarryn Newman	Lucian Geswindt	
Wednesday, 23	Lucian Geswindt	Robin Lang	
Thursday, 24	Robin Lang	Zakee Ebrahim	
Friday, 25	Zakee Ebrahim	Tarryn Newman	
Monday, 28	Robin Lang	Zakee Ebrahim	
Tuesday, 29	Zakee Ebrahim	Tarryn Newman	
Wednesday, 30	Tarryn Newman	Lucian Geswindt	
Thursday, 31	Lucian Geswindt	Robin Lang	

**Fourth Floor****First Aiders: Maureen Mudau, Precious Nkonde, Sandz Dyantyi-Nzunga, Audrey Rinquest, Goduletoweng Polori (Goduletoweng Polori WFH from July 7th to 21st)**

Date (July 2025)	Primary First Aider	Backup First Aider	Notes
Wednesday, 2	Maureen Mudau	Precious Nkonde	
Thursday, 3	Precious Nkonde	Sandz Dyantyi-Nzunga	
Friday, 4	Sandz Dyantyi-Nzunga	Audrey Rinquest	
Monday, 7	Maureen Mudau	Precious Nkonde	Goduletoweng Polori WFH (Mon-Fri)
Tuesday, 8	Precious Nkonde	Sandz Dyantyi-Nzunga	Goduletoweng Polori WFH (Mon-Fri)
Wednesday, 9	Sandz Dyantyi-Nzunga	Audrey Rinquest	Goduletoweng Polori WFH (Mon-Fri)

Date (July 2025)	Primary First Aider	Backup First Aider	Notes
Thursday, 10	Audrey Rinquest	Maureen Mudau	Goduletoweng Polori WFH (Mon-Fri)
Friday, 11	Maureen Mudau	Precious Nkonde	Goduletoweng Polori WFH (Mon-Fri)
Monday, 14	Audrey Rinquest	Maureen Mudau	Goduletoweng Polori WFH (Mon-Fri)
Tuesday, 15	Maureen Mudau	Precious Nkonde	Goduletoweng Polori WFH (Mon-Fri)
Wednesday, 16	Precious Nkonde	Sandz Dyantyi-Nzunga	Goduletoweng Polori WFH (Mon-Fri)
Thursday, 17	Sandz Dyantyi-Nzunga	Audrey Rinquest	Goduletoweng Polori WFH (Mon-Fri)
Friday, 18	Audrey Rinquest	Maureen Mudau	Goduletoweng Polori WFH (Mon-Fri)
Monday, 21	Sandz Dyantyi-Nzunga	Audrey Rinquest	Goduletoweng Polori WFH (Last day of WFH for Polori)
Tuesday, 22	Goduletoweng Polori	Maureen Mudau	Goduletoweng Polori returns to office
Wednesday, 23	Maureen Mudau	Precious Nkonde	
Thursday, 24	Precious Nkonde	Sandz Dyantyi-Nzunga	
Friday, 25	Sandz Dyantyi-Nzunga	Audrey Rinquest	
Monday, 28	Maureen Mudau	Precious Nkonde	
Tuesday, 29	Precious Nkonde	Sandz Dyantyi-Nzunga	
Wednesday, 30	Sandz Dyantyi-Nzunga	Audrey Rinquest	
Thursday, 31	Audrey Rinquest	Goduletoweng Polori	



ANNEXURE

“F”

Sick note — Dr Jacques Malan (Akeso Stepping Stones Clinic)

Date: 1 April 2025

Relevance: *Part D — Mental Health / Disability*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

Dr Jacques Malan

SPECIALIST - PSYCHIATRIST
MBChB(UCT) FCPSYCH(SA)
PR. NO. 022 000 0449113



Akeso Kenilworth Clinic
32 Kenilworth Road
Kenilworth 7708
Tel: 021 763 4502

drmalanpsychiatrist@gmail.com

The employer

15 May 2025

Re: Mr Wilbur Matthee
Date of Birth: 03/09/1989

Mr Matthee was admitted to Akeso Stepping Stones Clinic on 18 March 2025 due to significant mental health difficulties. He required inpatient treatment for severe depression and PTSD. Due to the severity of his illness his initial 3 week treatment period was extended to 6 weeks in order to provide optimal treatment and restore his occupational function to premorbid levels.



However due to major personal and social problems (theft from his home and loss of his pets) and a lack of any remaining sick leave days Mr Matthee was forced to leave treatment early to deal with matters at home. Now that this has settled Mr Matthee should return to hospital to complete his treatment properly. He would ideally be re-admitted to Stepping Stones to finish the remainder of his therapy and complete his recovery program including comprehensive discharge planning. Currently he is at risk and struggles to cope with ordinary daily tasks and occupational requirements and he may relapse further in his mental health if not afforded the time to finish his treatment. He is expected to need 2 further weeks to complete the treatment program and should be admitted as soon as possible.

If you have any queries regarding this request please contact the office.

Kind regards

Dr Jacques Malan

ANNEXURE

“G”

Mental health disclosure to Mapule Mashele (formal written notice of disability)

Date: 16 May 2025

Relevance: *Part D — Mental Health / Disability*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



13/4/2026-11:58:23 AM

Urgent intervention required

1 message

Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>

Thu, 04 Dec 2025 at 12:30

To: Gerald Van Wyk <Gerald.VanWyk@afrocentrichealth.com>, Lindiwe Miyambu <Lindiwe.Miyambu@afrocentrichealth.com>, Monwabisi Kula <Monwabisi.Kula@afrocentrichealth.com>, Andrew Schwulst <Andrew.Schwulst@medscheme.co.za>, Aklaaq Mahmood <aklaaqm@afrocentrichealth.com>
Cc: wilburmat@gmail.com <wilburmat@gmail.com>

[Quoted text hidden]

Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Case Manager: GEMS Active Disease Risk Management

- +27 12 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock,7925
- www.medscheme.com



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mailto:Meds_disclaimer@medscheme.co.za .

----- Forwarded message -----

From: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>

To: Mapule Mashele <Mapule.Mashele@medscheme.co.za>

Page 53 of 243

Cc:
Bcc:
Date: Fri, 16 May 2025 12:37:35 +0000
Subject: Emailing Wilbur Matthee - letter to employer.pdf

13/4/2026-11:58:23 AM

Dear Mapule,

Please find letter from my treating Dr attached as requested for further information feel free to speak to my Dr directly via details provided or speak to me if you require additional information.

Best regards



Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Care manager: GEMS Active Disease Risk Management

- +27 21 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock,7925
- www.medscheme.com



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ANNEXURE

“H”

Jones: “we do not apply staggering of money owed”

Date: 16 April 2025

Relevance: *Part D — Mental Health / Disability*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>

13/4/2026-11:58:23 AM

Wed, 16 Apr 2025 12:59:04 +0200
Subject: Meeting Feedback 16 April 2025

ning Wilbur

st you are well.

se find a summary of our meeting yesterday.

Welcome back-I wish you all the best with your new path. Thanks for what you shared and the changes you are making to your life.
5 days unpaid leave-I have reached out to HR via email regarding this matter but just want to mention that we do not apply staggering of money owed. Email has been sent to HR and feedback will be provided as soon as received.
Sick note or letter from doctor required to assist with accommodations.
Accommodations made upon return to work:



1. Allocation to LACE cases only for 2 weeks/until end of the month.
2. Working hours will be from 08h00-17h00
3. 7 working days QA grace period-email sent to QA with request.
4. 3 hours may be worked in in advance per month for doctor's visit. Department only allow 2 hours per month but part of the accommodation made now is due to follow ups with doctor. Process will be reviewed as time goes on (3 months-6 months).

1/3



Comm... Highlig... Draw Text Fill & S... Edit PDF More t...

ANNEXURE

“I”

Payslip — July 2025 (R14,811.77 deduction; 40.47% of gross salary)

Date: July 2025

Relevance: *Part E — Unlawful Salary Deduction*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

PO Box 1101, Florida Glen, 7708
 37 Conrad Street, Florida North,
 Roodepoort, 7709
 t +27 11 671 2000
 www.medscheme.com

Employee Identity Number	Matthee, Wilbur William(Wilbur),Mr. 8909035151087	Employer Name	Medscheme Holdings (Pty) Ltd
Employee Number	7074649	Grade	B4M1
Location	The Boulevard	Position	Care Coordinator.25
Hire Date	01-May-2024	Original Hire Date	01-May-2024
Assignment Number	7074649	Payroll	Medscheme Holdings (Permanent)
Employee Address	Street Number: 15 Street or Name of Farm: Bellevue Street Suburb or District: Voorbrug, Postal Code: 7100	Employer Address	The Boulevard 4th Floor Searle Street Cape Town 7925



Payroll Information		
Period End Date	Tax Period	Payment Frequency
31-Jul-2025	5	Calendar Month

Tax Information		
Tax Status	Tax Reference No	Pay Date
Normal	0694546243	23-JUL-2025

Summary of Payment		
Total Earnings	Total Deductions	Total Net Pay
21,777.04	6,097.75	15,679.29

Earnings	
Description	Amount
Normal Earnings	36,588.81
Unpaid Leave Recovery	(13,824.32)
Unpaid Leave	(987.45)

Deductions	
Description	Amount
Sanlam Funeral Cover	52.00
Bonitas EE	1,471.33
Sanlam Pension Fund EE	2,503.57
UIF Employee	177.12
PAYE	1,743.73
Access Tags Loss	150.00

Fringe Benefits and Additional Info	
Description	Amount
Medical Aid Bonitas ER	2,942.67
PHI Disability ER	469.00
Sanlam Pension Fund ER	2,536.95
UIF Employer Contribution	177.12
Group Life	258.70
Medical Aid Fringe Benefit	2,942.67
Medical Aid Tax Credit	728.00
Monthly Salary Info - CTC	42,796.13
PHI FBT	469.00
Retirement Funding Income %	78.00

YTD Accumulations & Balances	
Description	Amount
Medical Aid Tax Credit YTD	3,640.00
Medical Aid ER YTD	14,713.35
Medical Aid EE YTD	7,356.65
PAYE YTD	23,491.73
Normal Earnings YTD	181,133.44
Total Taxable Income YTD	161,080.84
UIF Employee Contribution YTD	885.60

Net Pay Distribution			
Payment Method	Bank Branch Code	Account Number	Amount
ACB	679000	10961611474	15,679.29

Message(s)
No Message(s) Found

ANNEXURE

“J”

AOD Policy confirming 25% cap + “what is illegal?” exchange

Date: 24 July 2025

Relevance: *Part E — Unlawful Salary Deduction*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

Got it!

Thursday, 24 July

Mary-Ann Jones 07/24 10:35

MJ

**Acknowledge of Debt
(AOD)****Payroll:**

- Payroll to provide a recon on benefits paid during **waiting period**.
- In cases of a recovery, HCBP must have a consultation meeting with the employee to make arrangements to recover the owed amount.
- Declined cases – payroll to recover unpaid leave and benefits paid
- AOD monthly deductions must not be more than 25% of the employees CTC as per legislation.

07/24 10:54

This doesn't make sense to me right now so they are allowed to deduct more than 50% of my salary at a time?



Mary-Ann Jones 07/24 11:19

MJ

This is for disability

You can go back to the policy and read the caption and the extract-that will give more context

07/24 11:20

but according to BCEA its illegal.

Edited

But im just glad its settled

Mary-Ann Jones 07/24 12:46

MJ

Hi Wilbur, what is illegal?

Comm... Highlig... Draw Text Fill & S... Edit PDF More t...

ANNEXURE

“K”

“Wilma” Microsoft Teams message (screenshot, team-wide channel)

Date: 23 September 2025

Relevance: *Part F — Gender Identity Discrimination*

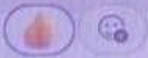


Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

Afternoon Belinda, Charles, Wilbur and Shuryda. We received a better report which we will filter and assign to you each for the members to be contacted. Same process apply. Just using a better list

Mary-Ann Jones 9/23 5:18 PM

Afternoon Wilma, Shuryda, Belinda and Charles. Please find updated lists added to your sheets. **NO NEED TO FILTER THESE LISTS. THIS REPORT COMES FROM HIU AND ARE THE MEMBERS WHO HAVE NOT HAD THEIR TESTS DONE YET.** PLEASE JUST CALL EACH MEMBER AND UPDATE YOUR SHEET AS WE WOULD NEED THE TOTALS FOR WEEKLY REPORTING. **NO FILTERING PLEASE**



Status of Wilbur Matthee: Social Style: Expressive; with a flair for bold, attention-grabbing communication. Learning style: Reflective & Auditory Learner; often seeking out educational insights appreciating structured advice that builds understanding.

Type a message



Search

SAMSUNG

ANNEXURE

“L”

Formal discrimination complaint lodged with HR

Date: 25 September 2025

Relevance: *Part F — Gender Identity Discrimination*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



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From: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>

Sent: 25 September 2025 16:08

To: Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>; Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>; Thando Mandondo <ThandoM@medscheme.co.za>

Subject: Formal Complaint: Handling of Harassment and Discrimination Case

Importance: High

Dear Cheryl, and team

I am writing to raise a serious concern regarding how my recent complaint of sexual orientation discrimination and harassment has been managed.

After I reported this matter, I was informed that I should have a one-on-one meeting with the individual I reported, and that she would then provide feedback to her manager. I believe this approach is inappropriate and does not align with the principles of impartiality, confidentiality, or protection from retaliation, which are essential when dealing with sensitive cases of discrimination and harassment.

Key Concerns with Current Process

Meeting privately with the accused places me in a vulnerable position and creates the risk of further harassment or intimidation.

Having the accused “report back” on the outcome of the meeting undermines impartiality and fairness in the process.



Forwarding my report directly to the accused without first ensuring neutral oversight compromises confidentiality and objectivity.

Specific Incident of Sexual Orientation Discrimination and Harassment

On 23 September 2025, my manager deliberately posted in our work teams group a version of my name from “Wilbur” to “Wilma,” clearly referencing a female name, knowing that I am gay.

This was not a typo or accidental error, but a deliberate act intended to mock and embarrass me.

The post was made after working hours and immediately before the Heritage Day public holiday on 24 September 2025.

The manager then took two consecutive days off on 25 and 26 September 2025, ensuring that I could not address or respond to the issue immediately. She is only returning on Monday.

This action caused me significant upset, humiliation, and distress, and publicly undermined my dignity and identity.

The post also affected many of my colleagues, who expressed disgust and concern over my manager’s deliberate actions.

As a result of this incident, I found myself unable to focus or be productive at work. I requested from my manager's manager if I could leave the office for the day to manage my mental health. I was referred to another team leader, who informed me that my manager had instructed that if I leave, she will log it as a half-day unpaid leave, despite my willingness to work the two hours back to support my mental health. When I objected, the team leader told me I should consult with my manager on Monday.

This clearly demonstrates how the manager's deliberate actions directly impacted my ability to perform my work, caused undue stress, and created an environment that was hostile to my mental health.

Under South African law, this constitutes sexual orientation discrimination and harassment, violating both company policy and the legal protections afforded under the Employment Equity Act 55 of 1998 and the Protection from Harassment Act 17 of 2011.



Inconsistent Application of Disciplinary Policy

Our company's disciplinary policy clearly states that any form of harassment warrants dismissal.

At a minimum, best practice requires that the accused be suspended on full pay pending investigation.

Despite this, the manager I reported has been allowed to continue in her role, while I remain in a compromising position.

This situation suggests:

The company is safeguarding the accused instead of applying its own policy consistently.

I remain exposed to further harassment, intimidation, or retaliation.

The credibility of the disciplinary process and grievance procedures is being undermined.

Requests and Expectations

I respectfully request that:

1. This matter be handled by a neutral HR representative or independent investigator, rather than the individual I reported.

13/4/2026-11:58:23 AM

2. I not be required to meet privately with the accused.

3. The manager be suspended pending investigation, in accordance with company policy.

4. The process be conducted in a way that ensures impartiality, confidentiality, and protection from retaliation.

5. I be immediately removed from this manager's team to prevent further exposure to harassment or a hostile work environment.



Legal Reminder

Under the Employment Equity Act 55 of 1998, harassment on the basis of sexual orientation constitutes unfair discrimination.

The Labour Relations Act 66 of 1995 requires fair labour practices.

The Protection from Harassment Act 17 of 2011 safeguards against conduct that could reasonably cause harm or distress.

Please confirm in writing how the company intends to proceed to ensure this complaint is managed fairly, lawfully, and in compliance with company policy and South African labour legislation.

If the matter is not addressed appropriately, I may have no option but to escalate this complaint to the CCMA or other relevant authorities.

Thank you for your urgent attention to this matter.

Sincerely,

Wilbur Matthee

 **RE_ Formal Complaint_ Handling of Harassment and Discrimination Case.eml** 13/4/2026-11:58:23 AM
131 KB

Wilbur Matthee <Wilbur.Matthee@medscheme.co.za> Fri, 05 Dec 2025 at 17:57
To: wilburmat@gmail.com <wilburmat@gmail.com>

[Quoted text hidden]

 **RE_ Formal Complaint_ Handling of Harassment and Discrimination Case.eml**
131 KB

Wilbur Matthee <Wilbur.Matthee@medscheme.co.za> Sat, 06 Dec 2025 at 22:01
To: Wilbur William Matthee <wilburmat@gmail.com>



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----- Forwarded message -----

From: Tebogo Dikobe <Tebogo.Dikobe@afrocentrichealth.com>
To: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
Cc:
Bcc:
Date: Tue, 30 Sep 2025 12:57:13 +0200
Subject: RE: Formal Complaint: Handling of Harassment and Discrimination Case

Hi Wilbur

I have shared this with Sipo and will look into it.

Regards

Tebogo Dikobe (He/Him/His)

Human Resource Officer

☎ +27(11) 758 9061

✉ Tebogo.Dikobe@afrocentrichealth.com

📍 37 Conrad St, Florida North, Roodepoort | PO Box 1101, Florida Glen, 1709

🌐 www.afrocentric.za.com

🌐 [Tebogo M Dikobe | LinkedIn](#)



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From: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
Sent: Thursday, September 25, 2025 4:11 PM
To: Tebogo Dikobe <Tebogo.Dikobe@afrocentrichealth.com>
Subject: Fw: Formal Complaint: Handling of Harassment and Discrimination Case

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From: Wilbur Matthee
Sent: Thursday, September 25, 2025 4:07:42 PM
To: Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>; Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>; Thando Mandondo <ThandoM@medscheme.co.za>
Subject: Formal Complaint: Handling of Harassment and Discrimination Case



Dear Cheryl, and team

[Quoted text hidden]

2 attachments



image004.png
29 KB



RE_ Formal Complaint_ Handling of Harassment and Discrimination Case.eml
106 KB

ANNEXURE

“M”

Mashele investigation closure report (“typo” finding)

Date: 1 October 2025

Relevance: *Part F — Gender Identity Discrimination*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



13/4/2026-11:58:23 AM

Fw: Follow-Up on Formal Complaint and Meeting Outcome

2 messages

Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
To: wilburmat@gmail.com <wilburmat@gmail.com>

Fri, 05 Dec 2025 at 17:53

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From: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>

To: Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>, Thando Mandondo <ThandoM@medscheme.co.za>, Tebogo Dikobe <Tebogo.Dikobe@afrocentrichealth.com>, Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>

Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Mapule Mashele <Mapule.Mashele@medscheme.co.za>

Bcc:

Date: Wed, 1 Oct 2025 11:09:00 +0200

Subject: Follow-Up on Formal Complaint and Meeting Outcome

Dear [Chery-DI/Team/Investigation Team],

I would like to provide a formal update following my meeting with the investigation officer, Senior Manager Ms. Mapule Mashele.

During our discussion, we acknowledged that while errors can occur, there are also instances where actions are deliberate. After reviewing all relevant evidence and context, we navigated toward the latter and reached a shared understanding of the seriousness of the incident.

As a constructive step forward, I have decided to speak directly with Mary-Ann in the presence of Ms. Mashele to express how the situation truly affected me and to seek a way to make amends.

I would like to sincerely thank the team for your swift response to my formal complaint, and for the support, professionalism, and guidance you provided throughout the reporting process.

Kind regards,

13/4/2026-11:58:23 AM



Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Care coordinator: GEMS Active Disease Risk Management

- +27 12 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock,7925
- www.medscheme.com



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 **Follow-Up on Formal Complaint and Meeting Outcome.eml**
52 KB

Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
To: Wilbur William Matthee <wilburmat@gmail.com>

Sat, 06 Dec 2025 at 22:01

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mailto:Meds_disclaimer@medscheme.co.za .

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From: Mapule Mashele <Mapule.Mashele@medscheme.co.za>

To: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>, Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>, Tebogo Dikobe <Tebogo.Dikobe@afrocentrichealth.com>, Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>

Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Thando Mandondo <ThandoM@medscheme.co.za>

Bcc:

Date: Wed, 1 Oct 2025 13:28:09 +0200

Subject: RE: Follow-Up on Formal Complaint and Meeting Outcome

Dear Wilbur,

I would like to take this moment to thank you once again for bringing this issue to our attention and, most importantly, for your willingness to engage and resolve this amicably.



@HC Team, I have engaged with both Wilbur and Mary-Anne on the complaint raised. Mary-Anne has confirmed that the incorrect spelling of Wilbur's name in the Teams chat was a typo on her part, which she regretted and has acknowledged the seriousness of. She has apologised to Wilbur, and the apology has been accepted. The issue is now considered resolved, and Mary-Anne has committed to being extra careful in the future.

Furthermore, we have agreed that the unpaid half-day leave logged for the 25th of September on Wilbur's behalf will be cancelled – Mary Anne will action and reach out to HC if she needs any support.

Wilbur has confirmed that there have been no prior issues with Mary-Anne that suggested any harassment based on sexual orientation and that this was a first occurrence. He further expressed that he does not wish to be removed from Mary-Anne's reporting line, as they work well together.

Next Steps:

- We will continue to monitor the working relationship to ensure there are no further concerns.
- Wilbur will continue to be in Mary Annes hierarchy – no changes to the reporting lines will be made.

This matter is now closed, but should any similar issues arise in future, they will be addressed promptly in line with the Prevention and Elimination of Harassment Policy.

Kind Regards

13/4/2026-11:58:23 AM

Mapule

Thank you again, Wilbur, for raising your concern in good faith and working with us towards a constructive resolution.

[Quoted text hidden]

 **RE_ Follow-Up on Formal Complaint and Meeting Outcome.eml**
58 KB



ANNEXURE

“N”

Gexus/DMS root cause analysis and protected disclosure

Date: 23 October 2025

Relevance: *Part G — Protected Disclosures*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

Good day Team

Hope my email finds you well.

Complaint details:



"Hi I have a 75 yr old Mom who is my one and only dependent. I religiously ensure she honours her blood collection months when due and not end there. Also collect her treatment every month from Nite Doc pharmacy. You sent her an email that you don't get records of her repeat. I request that you get your house in order and update your records. Stop confusing old lady.

SP Mahlangu

000655789"

Comment from the Medscheme CMM HLQ Team:

No email correspondence was sent to the member via GEXUS from the Medscheme CMM department.

Upon receipt of the complaint, the Medscheme CMM CMM HLQ Team contacted the main member to obtain details regarding the email that was sent. The following was advised:

FROM EMAIL ADDRESS: NOCGEMSBHRM@MEDSCHEME.CO.ZA

TO EMAIL ADDRESS: SBONGILEPR@GMAIL.COM

DATE AND TIME: 19/10/2025 13:28PM

SUMMARY OF EMAIL CONTENT:

- DID NOT RECEIVE REGULAR MONTHLY CLAIMS FOR CHRONIC MEDICATION

- WANT TO KNOW REASONS, THEN STIPULATES REASONS E.G. ERROR, SICK, OR FEELING WELL
NO LONGER NEEDS MEDS

13/4/2026-11:58:23 AM

The member has indicated that the communication received is incorrect, as they are claiming monthly for chronic medication.

Could you kindly confirm the following:

1. Is this email address associated with the ADRM space?
2. Was this communication sent from DMS?

If both are confirmed, I would like to request input from ADRM to enable me to respond on your behalf, as there is currently no HLQ agent assigned to our space.



As per my investigation, the last visible claim was in August 2025.

Could you also confirm whether this letter was part of a bulk communication or if it was sent by an agent?

Thank you in advance for your assistance.

Kind regards

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1. Email Address Verification



The email address NOCGEMSBHRM@MEDSCHEME.CO.ZA does not appear under the current possible email addresses accessible via Gexus.

It is an automated email address, not linked to ADRM space but the DMS system itself.

2. Source of Communication

The communication originated from DMS.

Confirmed that it was not sent by an agent but generated automatically.

3. Root Cause Analysis

Miscommunication between Gexus and DMS systems.

Member collected cholesterol medication from acute benefit from 21/11/2017 until 17/12/2024.

From January 2025, medication was claimed under chronic benefit.

Although Gexus shows a chronic claim on 14/04/2023 onwards for cholesterol medication, DMS did not receive this update.

DMS continued tracking adherence based on acute claims only, ignoring chronic claims after the benefit change.

This resulted in the member being incorrectly flagged as non-adherent, despite being compliant.

4. **Impact**

13/4/2026-11:58:23 AM

Automated correspondence was sent based on incorrect source information.

Member received misleading communication, causing dissatisfaction.

5. **Resolution Steps**

Confirmed issue stems from system integration gaps between Gexus and DMS.

Recommended actions:

Review and update communication between systems.

Ensure chronic claims are recognized by DMS for adherence tracking.



Input from the ADRM space to assist with member correspondence

Thank you for bringing this matter to our attention. We have identified that the communication you received was the result of a system error, and we appreciate your patience while we investigated.

Please be assured that your adherence status will be updated correctly going forward, and this will not affect your record.

We sincerely apologize for any inconvenience caused and thank you for your understanding. Should you have any further questions, please feel free to reach out to us.

I hope that this is sufficient feedback.

Kind regards,

Please find final feedback below.

13/4/2026-11:58:23 AM

The comms went out correctly, this member is non-adherent.

The non-adherent methodology is you need claim twice in a rolling three months member only claimed once in the past 3 months.

Sep – No claims = 0

Aug – claims = 1

Jul – no claims =0

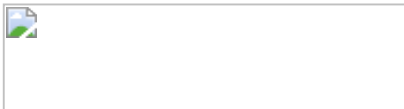
June – claims = 1

May – claims = 1



Please note a member could have claimed this month but the previous two months have not claims the member will be flagged as non-adherent – and clinically and behaviourally there is a problem that requires attention, therefore the intervention and should please be managed as such in the ADRM calls too.

Kind Regards



Mary-Ann Jones

Team Leader: GEMS Active Disease Risk Management



(021) 466 1191



Mary-annj@medscheme.co.za



37 Conrad St, Florida North, Roodepoort, 1709 | PO Box 1101, Florida Glen, 1709



www.medscheme.com



Please call member for enrollment and bring the non-adherence into the conversation with gentle probing as to why the medication was not claimed.

19/4/2026 11:58:25 AM

Kindly provide feedback when done.

Thank you



Mary-Ann Jones

Team Leader: GEMS Active Disease Risk Management

 (021) 466 1191

 Mary-annj@medscheme.co.za

 37 Conrad St, Florida North, Roodepoort, 1709 | PO Box 1101, Florida Glen, 1709

 www.medscheme.com



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13/4/2026-11:58:23 AM

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From: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
Sent: Thursday, October 23, 2025 12:40 PM
To: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>
Subject: RE: general complaint received - GEMS 000655789 Dependent 02

Dear Mary-Ann,

I wanted to provide an update on the recent complaint investigation. Based on the information received, the team indicated that the non-adherence correspondence was sent due to the member not claiming twice within a three-month period, which aligns with the non-adherence methodology for the Jul-Sep 2025 period.

However, after reviewing the evidence, I believe the complaint stems from a different issue:

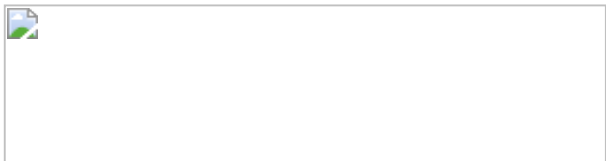
- DMS began flagging the member as non-adherent in Dec 2024, after the last acute claim.
- From Jan 2025 onward, the medication was claimed under the chronic benefit, but DMS did not recognize this change.
- As a result, automated non-adherence letters were sent on 19 May 2025 and 19 Oct 2025, despite the member being compliant.

This confirms the root cause is a miscommunication between Gexus and DMS systems, not the methodology itself. Evidence supporting this timeline has been attached for reference.

Given that the integration gap still exists, I will not contact the member at this time. Instead, I recommend that the team review this feedback and consult on the best way forward to prevent similar issues. 13/4/2026-11:58:23 AM

Thank you very much for your assistance.

Kind regards,



Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Case Manager: GEMS Active Disease Risk Management

- +27 12 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock,7925
- www.medscheme.com



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From: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>

Sent: Thursday, 23 October 2025 12:02

To: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>

Cc: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>

Subject: FW: general complaint received - GEMS 000655789 Dependent 02

Good day Wilbur



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From: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>
Sent: Thursday, 23 October 2025 12:54
To: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
Cc: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>
Subject: RE: general complaint received - GEMS 000655789 Dependent 02

Good day Wilbur

The below is noted. I am not in agreement with your feedback though.

Kind Regards



Mary-Ann Jones

Team Leader: GEMS Active Disease Risk Management



(021) 466 1191



Mary-annj@medscheme.co.za



37 Conrad St, Florida North, Roodepoort, 1709 | PO Box 1101, Florida Glen, 1709



www.medscheme.com

RE: general complaint received - GEMS 000655789 Dependent 02

18/4/2026-11:58:23 AM

1 message

Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
To: Mary-Ann Jones <Mary-AnnJ@medscheme.co.za>

Thu, 23 Oct 2025 at 15:21

Hello Mary-Ann,

This investigation outcome doesn't sit well with me, knowing that I will be excluded from further feedback or the final resolution. I revisited our communication prior to your response and believe I understand why you were not in agreement with my feedback:

You explicitly instructed me to call the member and address non-adherence, and I can see how my tone in the email, where I stated I would not contact the member at this time, may have come across as contradicting your directive. I also understand that complaints require timely resolution, and my decision may have delayed the process, potentially causing further dissatisfaction, which you were trying to avoid.



After looking at this from a neutral perspective, I realize that I should have contacted the member as requested while still escalating the integration gap for review.

Ultimately, I appreciate the opportunity you gave me to work on this case, it opened a valuable learning experience, and I truly enjoyed investigating it.

I hope that this instance of poor communication and failure to comply does not influence future requests for assistance.

Thank you for your guidance and support.

Kind regards,



Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Case Manager: GEMS Active Disease Risk Management

- +27 12 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock,7925
- www.medscheme.com

ANNEXURE

“O”

Transcript — 31 October 2025 meeting (retaliatory admission by Jones)

Date: 31 October 2025

Relevance: *Part I — Retaliatory Intent*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

ANNEXURE P: RECORD OF HOSTILE CONDUCT AND PROFESSIONAL MISCONDUCT – MARY-ANN JONES

This Annexure provides a summary of the hostile, malicious, and unprofessional conduct of Team Leader Mary-Ann Jones. The following highlights and verbatim quotes are extracted from the consultation on 31 October 2025 and the subsequent formal grievance (FG2). This conduct evidence points to a calculated pattern of behavior designed to undermine, isolate, and provoke the employee, creating an intolerable work environment intended to force constructive dismissal.



1. Hostile and Critical Communication

Ms. Jones initiated the interaction with immediate criticism and accusations, setting a tone of conflict.

- *"So I think there's a huge communication problem between me and you at the moment."*
- *"I don't think that what you are doing is really supporting what we need to achieve as a team and as a level two."*
- *"...the noise that you're creating today is another thing."*

2. Systematic Gaslighting and Denial of Facts

Ms. Jones repeatedly denied verifiable factual reality, such as the receipt of follow-up emails, to destabilize the employee's confidence.

- *"Well, but you did not inform me."*
- *"No, you didn't. You didn't. You did not."*
- *"I don't know what your intention is... but it's not right. I'm not standing for that."*

3. Retaliatory Threats and Weaponizing Past Issues

Ms. Jones maliciously referenced past resolved grievances and issued retaliatory threats of harassment claims based on minor clerical errors.

- *"I thought so. I thought you made a case of harassment against me last time. Isn't that so?"*
- *"And you must misspelled my name in an email. Go check. So I can make a case of harassment against you too."*

4. Accusations of Undermining and Insubordination

Ms. Jones utilized 'Double Binds' to label initiative as undermining while simultaneously demanding respect for her authority.

- *"You have the recording, if you need it.."*
- *"Yes, that was insubordination. Because you are telling me you're not going to do what I'm asking you to do."*

5. Pressure for Resignation / Transfer (Constructive Dismissal)

Ms. Jones explicitly solicited the employee's removal from the team, suggesting they were unhappy and should resign or transfer.

- *"If you're not happy in this team, ask to be removed from the team."*
- *"I'd rather do that. Send Mapule the email asking if you can be removed from the team because you do communicate like that."*
- *"Maybe it is like you say, you're being targeted... tomorrow it can be victimization and that state can be whatever."*
- *"I cannot continue on the way that we are continuing on as a team leader with somebody in my team."*





00:00:05 Wilbur Matthee
Okay.

00:00:07 Mary-Ann Jones
Okay.

00:00:08 Mary-Ann Jones
Great stuff.

00:00:11 Mary-Ann Jones
So I think there's a huge communication problem between me and you at the moment.



00:00:21 Mary-Ann Jones
I don't think that what you are doing is really supporting what we need to achieve as a team and as a level two.

00:00:38 Mary-Ann Jones
There is the request that I asked you to attend to.

00:00:41 Mary-Ann Jones
You said that you will not attend to it.

00:00:45 Mary-Ann Jones
You're sending escalations to CBNR without us knowing what it's about.

00:00:52 Mary-Ann Jones
We need to do reporting on those.

00:00:55 Mary-Ann Jones
If I have to get any questions, I wouldn't know.

00:00:58 Mary-Ann Jones
Maybe the intention is that I should not be knowing.

00:01:00 Mary-Ann Jones
But as a team leader.

00:01:02 Wilbur Matthee
Okay, so sorry.

00:01:03 Wilbur Matthee
Sorry.

00:01:03 Wilbur Matthee
Something that is not the intention.

00:01:06 Wilbur Matthee
First of all, immediately after I sent that message to some peewee, I informed you saying that I am sorry, but I forgot to cc you in that complaint.

00:01:20 Wilbur Matthee
Do you remember me sending that email to you?



00:01:23 Mary-Ann Jones
Sorry.

00:01:23 Mary-Ann Jones
Well, but you did not inform me.

00:01:26 Wilbur Matthee
I did.

00:01:27 Wilbur Matthee
I did inform you.

00:01:28 Wilbur Matthee
I did inform you.

00:01:29 Wilbur Matthee
As soon as.

00:01:30 Wilbur Matthee
As soon as I send that CMBR referral, I send you an email.

00:01:35 Wilbur Matthee
I just want to open my email quickly.

00:01:39 Mary-Ann Jones
And if I speak again, if you can just give me the opportunity to finish speaking, I

would appreciate that first.

00:01:46 Wilbur Matthee
All right, no problem.

00:01:48 Mary-Ann Jones
If there is.

00:01:49 Mary-Ann Jones
Because we can't all speak at the same time and then.

00:01:55 Mary-Ann Jones
Yeah, I don't know what your intention is with.

00:01:58 Mary-Ann Jones
With what is happening at the moment, but it's not right.

00:02:03 Mary-Ann Jones
I'm not standing for that.

00:02:06 Mary-Ann Jones
Hold on, let me finish.

00:02:08 Mary-Ann Jones
I was in Mapule's office.

00:02:10 Mary-Ann Jones
I asked if you now want to go to the other team.

00:02:13 Mary-Ann Jones
You said no, no, no, no, no.

00:02:15 Mary-Ann Jones
But what you are doing now, you are never minding me.

00:02:19 Mary-Ann Jones
You're undermining what I'm trying to do in the team.

00:02:22 Mary-Ann Jones
And I would really appreciate if you think about that.

00:02:25 Mary-Ann Jones



00:03:04 Wilbur Matthee

I didn't consult you first or CCU in the email.

00:03:08 Wilbur Matthee

Please let me know if you'd like me to forward the details so keep you updated on the outcomes.

00:03:12 Wilbur Matthee

You reply to me on that email, just good day, Wilbur.

00:03:17 Wilbur Matthee

That's right.

00:03:18 Wilbur Matthee

And then I said thank you for the sop.



00:03:20 Wilbur Matthee

So I did reply on it and I did get back to you.

00:03:23 Wilbur Matthee

So you should also take it from a manager's point of view that I am also learning these stuff still and I am going to make steps along the way.

00:03:31 Mary-Ann Jones

So.

00:03:33 Mary-Ann Jones

So why don't you reach out to me when you have a question, when you wondering about something, why don't you reach out to me first?

00:03:40 Wilbur Matthee

I normally do too.

00:03:41 Wilbur Matthee

Don't I normally do reach out?

00:03:43 Mary-Ann Jones

No.

00:03:43 Mary-Ann Jones

No, you didn't.

00:04:22 Mary-Ann Jones

I think maybe you need maybe take a step back and the noise that you're creating today is another thing.

00:04:34 Mary-Ann Jones

I spoke to you this morning.

00:04:35 Mary-Ann Jones

When I want to speak to you, I reach out to you and I speak to you.

00:04:39 Wilbur Matthee

All right.

00:04:40 Wilbur Matthee

I just didn't think it was fair.



00:04:42 Mary-Ann Jones

No.

00:04:42 Wilbur Matthee

Tell me this morning before I started to work that I need to be back in office when the agreement wasn't that.

00:04:49 Wilbur Matthee

That wasn't the initial agreement we had.

00:04:52 Wilbur Matthee

And that's why I asked the team if anyone else received that information.

00:04:57 Wilbur Matthee

And no one received that information.

00:04:59 Wilbur Matthee

What does that tell me then?

00:05:00 Wilbur Matthee

At the end of the day, what.

00:05:03 Mary-Ann Jones

Does it tell you?

00:05:04 Wilbur Matthee

Maybe I'm being targeted.

00:05:05 Wilbur Matthee
Maybe I was going to be.

00:05:07 Mary-Ann Jones
I thought so.

00:05:09 Mary-Ann Jones
I thought you made a case of harassment against me last time.

00:05:13 Mary-Ann Jones
Isn't that so?

00:05:15 Mary-Ann Jones
And you must misspelled my name in an email.



00:05:18 Mary-Ann Jones
Go check.

00:05:19 Mary-Ann Jones
So I can make a case of harassment against you too.

00:05:22 Wilbur Matthee
I'm not.

00:05:23 Wilbur Matthee
I don't want to go back and forth with you about.

00:05:24 Wilbur Matthee
About things that's really not valid about or to talk about.

00:05:28 Wilbur Matthee
So I want us to focus on what is really important.

00:05:31 Mary-Ann Jones
Yes.

00:05:32 Mary-Ann Jones
No, you want to.

00:05:33 Mary-Ann Jones

00:12:19 Mary-Ann Jones

But there is quite a bit of challenges and I mentioned that also when.

00:12:23 Mary-Ann Jones

When I reach out to you guys with all of this of the pilot that everybody wants.

00:12:30 Mary-Ann Jones

So yeah.

00:12:32 Mary-Ann Jones

So please just keep an open mind.

00:12:35 Mary-Ann Jones

I would appreciate if you take a step back.



00:12:37 Mary-Ann Jones

If you do decide that you do ask Mapule that you want to be out of the team.

00:12:43 Mary-Ann Jones

But I also believe that me and you as a team, I was supporting you for months on here.

00:12:48 Mary-Ann Jones

But I cannot continue on the way that we are continuing on as a team leader with somebody in my team.

00:12:57 Mary-Ann Jones

Another question I want to ask you because you're in.

00:13:00 Mary-Ann Jones

In my team, on my team, in our team, are you having meeting with the team?

00:13:06 Mary-Ann Jones

Are you scheduling meetings with the teams in the department?

00:13:12 Wilbur Matthee

So to be honest, yes.

00:13:15 Mary-Ann Jones

Yeah.

00:13:15 Mary-Ann Jones

00:14:00 Mary-Ann Jones

Should it be a union meeting or is it just a nice chill department kind of meeting?

00:14:06 Mary-Ann Jones

Should we then cancel the.

00:14:09 Mary-Ann Jones

These chilling sessions and things that we're having?

00:14:12 Mary-Ann Jones

What is the effect of this at the end of the day?

00:14:19 Wilbur Matthee

So the team discussion on the work from Home policy was a meeting that I created prior to sending this email to Mapule requesting for the policy.



00:14:33 Wilbur Matthee

It was just a short meeting that we had to discuss on what we want to put in on that.

00:14:40 Wilbur Matthee

On the email and stuff.

00:14:41 Wilbur Matthee

Wasn't a long meeting.

00:14:44 Wilbur Matthee

Yeah.

00:14:45 Mary-Ann Jones

So was that put on the timesheets of the team?

00:14:48 Mary-Ann Jones

Did I approve that?

00:14:50 Mary-Ann Jones

Was it now in October or September?

00:14:52 Wilbur Matthee

It was in a second.

00:15:00 Wilbur Matthee

Ninth month in September.

00:15:03 Mary-Ann Jones
September.

00:15:04 Mary-Ann Jones
Was that put on the time sheet?

00:15:05 Mary-Ann Jones
The time?

00:15:06 Wilbur Matthee
No.

00:15:06 Wilbur Matthee
No, I don't.

00:15:08 Mary-Ann Jones
Don't think so.

00:15:09 Mary-Ann Jones
Okay.

00:15:10 Mary-Ann Jones
Don't think so.

00:15:12 Mary-Ann Jones
Okay.

00:15:13 Mary-Ann Jones
Because if I did approve that, then that is so wrong.

00:15:16 Mary-Ann Jones
If I approve that time on that thingy timesheet.

00:15:20 Mary-Ann Jones
If it was something that untoward.

00:15:28 Mary-Ann Jones
Wilbur, I think when you come into office we need to have a person face to face.

00:15:34 Mary-Ann Jones
I like first to face to face in such instances.



00:20:32 Mary-Ann Jones
Whatever.

00:20:33 Mary-Ann Jones
We haven't spoken since we've been in the office there.

00:20:36 Mary-Ann Jones
I think it's time for us to sit and face to face.

00:20:41 Mary-Ann Jones
Besides, before or without Mapule them, if you decide you're still gonna go there, yes, you go there.

00:20:48 Mary-Ann Jones
But I think it's important that we must have some other, healthier something going on.



00:20:56 Mary-Ann Jones
But you do have the recording as proof if.

00:20:58 Mary-Ann Jones
If you need it.

00:21:00 Mary-Ann Jones
So, Wolba, I will chat to you later again.

00:21:03 Mary-Ann Jones
I did call you because I felt what was happening was a bit uncooled for you.

00:21:13 Mary-Ann Jones
She wanted clarity and I'm glad that somebody in the chat said maybe you must rediscuss it with your team leader then.

00:21:21 Mary-Ann Jones
So.

00:21:23 Mary-Ann Jones
Yeah, I did appreciate that.

00:21:26 Mary-Ann Jones
So I will speak to you again this afternoon.

ANNEXURE

“P”

Formal grievance against Jones

Date: 2 November 2025

Relevance: *Part J — Institutional Silence*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



Formal Grievance - Evidence of Malicious Intent, Constructive Dismissal, and Hostile Work Environment by Mary-Ann Jones

1 message

Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>

Sun, 02 Nov 2025 at 22:28

To: Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>, Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>, Thando Mandondo <ThandoM@medscheme.co.za>, Tebogo Dikobe <Tebogo.Dikobe@afrocentrichealth.com>
Cc: Wilbur William Matthee <wilburmat@gmail.com>

Dear HR Department and Union,



I am writing to lodge an urgent formal grievance against my team leader, Mary-Ann Jones. A review of the attached transcript from 31 October 2025 (SPK1=Mary-Ann SPK2 Wilbur) reveals a calculated pattern of behavior designed to undermine, isolate, and provoke me, creating an intolerable work environment with the apparent intent of forcing my resignation.

This is not a simple personality conflict. The evidence points to malicious intent and a strategic effort to build a false case against me.

Evidence of Malicious and Bad-Faith Conduct:

1. Strategic Entrapment and "Double Binds":

- My team leader creates no-win situations. When I show initiative (e.g., the CNBR referral), I am accused of "undermining" her and operating in secrecy, even when I proactively correct the oversight. When I seek clarity from peers on a sudden, unilateral change to my work arrangement, I am accused of "creating noise" and failing to "respect authority." This is a deliberate tactic to ensure I am always in the wrong.

2. Systematic Gaslighting:

- She directly denies factual reality. In the transcript, I state I informed her of the CNBR issue via a follow-up email, which she explicitly denies ("No, you didn't"). This is a clear attempt to destabilize my perception of events and erode my confidence.

3. Weaponizing Past Resolved Issues:

- She inappropriately and maliciously referenced a past, resolved grievance I lodged, using it to suggest a pattern of behavior on my part which she can do to now by opening a case of harassment against me. This is an obvious attempt to pre-emptively discredit me and intimidate me from raising current valid concerns.

4. Orchestrated Provocation and Narrative Building

· The sudden, out-of-hours demand to return to the office—issued only to me, contrary to agreed-upon plans—appears designed to provoke a reaction. My reasonable efforts to understand this directive were then used as the "evidence" to support her accusations of insubordination. She is actively manufacturing incidents to support a false narrative.

5. Overt Campaign for My Removal:

· She repeatedly and unprompted suggested I "ask Mapule to be removed from the team," stating our dynamic is "unhealthy" and that she "cannot continue." Given that she is the source of the conflict, these statements are not constructive but are a clear indication of her desired outcome: my removal from her team, by any means necessary.

6. Retaliation and Mockery of prior Complaint:

· Mary-Ann referenced the harassment complaint I previously lodged, and said: "You made a case of harassment against me, now I can make a case of harassment against you. You misspelled my name in an email—go look." Given that we communicate via email multiple times daily, this comment was delivered in a manner that was dismissive and condescending. This threaten a counter-complaint appears retaliatory, and it served to belittle and undermine my prior legitimate complaint rather than address it confidentially and professionally.

Conclusion and Urgent Request:

The cumulative effect of these actions is a textbook hostile work environment. The pattern of entrapment, gaslighting, and provocation indicates bad faith and malicious intent that goes far beyond poor management. I am experiencing significant stress and anxiety, and I have a genuine fear that my position is being deliberately made untenable to facilitate a constructive dismissal.

I therefore demand that HR:

1. Launch an immediate and formal investigation into the conduct of Mary-Ann Jones, focusing on the patterns of malicious behavior and intent outlined above.
2. Take immediate interim measures to remove me from her direct authority pending the outcome of the investigation.
3. Provide me with a written guarantee against any form of retaliation.
4. Confirm in writing that the unilateral change to my work arrangement is suspended until a full and fair consultation is conducted.

This situation is critical. I have attached the transcript as evidence and am prepared to provide all further documentation. I expect your immediate response to arrange a meeting.

Sincerely,

Sr. Wilbur W. Matthee

(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Case Manager: GEMS Active Disease Risk Management

- +27 12 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock,7925
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transcript1.pdf

136 KB

ANNEXURE

“Q”

Executive escalation bundle to leadership (32-page evidence bundle)

Date: 4 December 2025

Relevance: *Part J — Institutional Silence*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

RE: Urgent intervention required

1 message

13/4/2026-11:58:23 AM

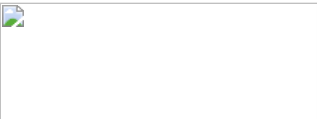
Monwabisi Kula <Monwabisi.Kula@afrocentrichealth.com> Tue, 09 Dec 2025 at 10:00
To: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>, Gerald Van Wyk <Gerald.VanWyk@afrocentrichealth.com>, Lindiwe Miyambu <Lindiwe.Miyambu@afrocentrichealth.com>, Andrew Schwulst <Andrew.Schwulst@medscheme.co.za>, Aklaaq Mahmood <aklaaqm@afrocentrichealth.com>
Cc: wilburmat@gmail.com <wilburmat@gmail.com>

Dear Wilbur,

Thank you for raising the below concerns with us , this mail serves to confirm receipt of your escalation.

While we get the urgency you're highlighting, please also appreciate that your note is quite lengthy, with 32 attachments annexed to it, so please allow us sufficient time to investigate the matter comprehensively and give you feedback as soon as possible. With the festive break upon us, I can realistically say you will get interim feedback during January.

Regards,



Monwabisi Kula
Chief Risk Officer



Office: +27 11 671 6447 | Mobile: +27 72 690 2140
Monwabisi.Kula@afrocentrichealth.com
37 Conrad St, Florida North, Roodepoort, 1709 | PO Box 1101, Florida Glen, 1709
www.afrocentric.za.com



From: Wilbur Matthee <Wilbur.Matthee@medscheme.co.za>
Sent: Thursday, December 4, 2025 12:29 PM
To: Gerald Van Wyk <Gerald.VanWyk@afrocentrichealth.com>; Lindiwe Miyambu <Lindiwe.Miyambu@afrocentrichealth.com>; Monwabisi Kula <Monwabisi.Kula@afrocentrichealth.com>; Andrew Schwulst <Andrew.Schwulst@medscheme.co.za>; Aklaaq Mahmood <aklaaqm@afrocentrichealth.com>
Cc: wilburmat@gmail.com
Subject: Urgent intervention required
Importance: High

Dear Executive Leadership,

This correspondence serves as my final and good-faith attempt to resolve, internally and without escalation, the severe and ongoing harm I have been subjected to as a direct result of the company's repeated institutional failures.

Over the past months, I have experienced serious and documented misconduct, including:

1. Unlawful salary deductions exceeding 40% of my monthly income without my consent or consultation. This during a motivated mental health admission, awaiting a bed informed by the HRBP that the deduction was taken on the principle of no work no pay which did not require consultation.
2. Continuous intimidation, retaliation, and victimisation by my direct manager despite lodging grievance. After a formal request to be temporarily removed from her team. No action.

3. Awaiting feedback from manager as discrepancies found in up to 20% reduction in some cases of my monthly actual performance metrics that led to poor outcomes even missed opportunities in performance bonuses.

13/4/2026-11:58:23 AM

4. Prolonged silence and inaction from HR and senior leadership and union despite formal complaints. Currently +30days to date no response.

These actions have caused significant financial strain, forced me into a cycle of unexpected debt which i am struggling to manage, and resulted in severe psychological distress. My capacity to work in an environment that continues to permit such conduct has reached its limit.

Given the seriousness of these failures, I am formally requesting the restorative measures:

1. Paid Recuperation Leave & Guaranteed Reintegration

A period of fully paid special leave, allowing adequate time for psychological recovery.

Upon return, a permanent and immediate transfer to a new department.

2. Financial Settlement Determined by Risk & Compliance

I request a comprehensive financial settlement, independently assessed by the Head of Risk & Compliance, covering:

Financial losses caused by the unlawful deduction.

Debt incurred as a direct result of these actions.

Compensation for emotional distress, harassment, and institutional negligence.

The company's full legal exposure for:

Unfair labour practice

Discrimination

Failure to provide a safe working environment

Reputational and operational risks should the matter escalate externally.

3. Institutional Commitments

A written, irrevocable guarantee of no retaliation, present or future.

Next Steps

I am willing to meet with this leadership group and a neutral mediator within 14 days from date 04 December 2025.

If a signed agreement in principle is not provided by 14 working days, I will immediately proceed with the following simultaneous actions: As I feel I have exhausted all avenues this far to have the incidents resolved and this is my final demand.



1. File cases with the CCMA for unfair labour practices, and discrimination.

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2. Lodge a formal complaint with the Department of Employment and Labour regarding the unlawful salary deduction.

3. Submit a complaint to the South African Human Rights Commission.

I am really at my breaking point now no motivation to come to work.

All evidence is already compiled and ready for submission.

I trust that the company will choose a constructive, internal resolution over a protracted and public legal dispute. I await your urgent response.

Kind regards,



Sr. Wilbur W. Matthee
(Nurse Specialist PGrad HSM, Btech Gen Pn (Midw,Psych,Comm))

Case Manager: GEMS Active Disease Risk Management

- +27 12 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock,7925
- www.medscheme.com



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ANNEXURE

“R”

Employer’s opposing affidavit — CCMA (Xolile Vincent Manyathi)

Date: 10 December 2025

Relevance: *Part K — Bad-Faith Affidavit*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

IN THE COMMISSION FOR CONCILIATION MEDIATION AND ARBITRATION

Case Ref. Number: **WECT23564-25**

In the matter between

WILBUR MATTHEE

Applicant

And

MEDSCHEME HOLDINGS

Respondent



NOTICE OF OPPOSITION OF CONDONATION APPLICATION

Your condonation application dated **05 December 2025**, is hereby acknowledged and refers.

In response to the condonation application by the applicant **BE PLEASED TO TAKE NOTE THAT** the Respondent hereby files its Notice of Opposition.

PLEASE TAKE FURTHER NOTE THAT the answering affidavit of **Mr XOLILE VINCENT MANYATI** will be used in support of this notice of opposition.

The respondent will accept all documents in this matter at the following email address:
IR@afrocentrichealth.com.

Signed at **Johannesburg** on the **10th** day of **December 2025**.

XOLILE VINCENT MANYATI

Respondent representative

To: Wilbur Matthee
Per email: wilburmat@gmail.com

And to: Case Management Officer
CCMA
Per email: CMS@CCMA.org.za



**IN THE COMMISSION FOR CONCILIATION MEDIATION AND ARBITRATION
HELD AT CAPE TOWN**

Case Ref. Number: **WECT23564-25**

In the matter between

WILBUR MATTHEE

APPLICANT

And

MEDSCHEME HOLDINGS

RESPONDENT



RESPONDENT'S ANSWERING AFFIDAVIT

I the undersigned **Mr. Xolile Vincent Manyathi** do hereby make an oath and say:

1. I am an adult male and the respondent in this matter. I am duly authorized to make this affidavit as a Senior Employee Relations Specialist employed by Afrocentric Health (Pty) Ltd, the holding company.
2. I am duly authorized to depose of this affidavit. The facts contained herein are both true and correct and fall within my personal knowledge, unless otherwise indicated or the contrary appears from the facts.
3. The applicant in this matter is **Mr. Wilbur Matthee**, who is employed as a care coordinator at Gems business unit under Medscheme Holdings.

4. LEGAL FRAMEWORK:

- 4.1 In terms of the Labour Relations Act 66 of 1995, as amended, if there is a dispute about unfair labour practice, the employee may refer the dispute in writing, however the referral must be made within **90 days** of the date of the act or omission which allegedly constitutes the unfair labour practice. If the employee shows good cause at any time, the council or the commission may permit the employee to refer the dispute after the 90-day time limit has expired.

4.2 Section 191 (2) stipulates that if the applicant shows good cause at any time, the council or CCMA may permit the applicant to refer the dispute after the 90-day time limit has expired. In the case of **Shoprite Checkers (Pty) Ltd CCMA & Others (1998) 19 ILJ 892 (LC) p 21-22, (1998) 5 BLLR 510 (LC)** the honourable judge summarised the law in this regard as follows: "Section 191(2) enjoins an applicant to show good cause before a commission may permit her to refer the dispute after the time limit has expired. The section makes it clear that condonation is not there merely for the asking. The applicant must tender an adequate explanation for the delay. The Commissioner must also consider this explanation. Due regard must be had to the other generally accepted requirements for the grant of condonation as contemplated in the words *good cause....*"



4.3 From the above, it is clear that before the commissioner can grant condonation, the applicant must furnish **justifiable reasons** for the late request. The fundamental question is, therefore, whether the applicant has good grounds for not referring the dispute in time and does his explanation satisfy the factors as mentioned below.

4.4 To that end, in the case of **Dlamini v Progress Housekeeping CC (2001) 22 ILJ 1024 (CCMA) 1028** the commissioner indicated that in determining whether to grant condonation or not the following factors need to be taken into account: the **degree of lateness**, the **degree of fault**, the **reasonableness of the explanation**, the **prejudice to the parties**, the **prospect of success in the main dispute**. The aforementioned factors were re-iterated in the case of **Job L. Boshop v Mac Donalds Transport NC5139 7 July 2000 CCMA**.

5. **BACKGROUND:**

5.1 The Applicant is employed as the care coordinator in Medscheme and is based at the employer's offices in Cape Town.

5.2 On the or about **July 2025** the Applicant was absent due to health reasons, the employees' leave days got exhausted due to this prolonged absence and because of previous intermittent sick leaves taken. All the employees' leave days were exhausted, including sick leave and annual leave were exhausted at the time.

5.3 The Respondent's process is that before it can apply a no work no pay principle, it firstly exhausts the employees' annual leave in lieu of sick leave when sick leave is finished, it also assists employees with special leave which in this case it also did in order to accommodate the employee. This is the case in this matter where the respondent firstly used annual leave of the

employee in lieu of sick leave absence. The employee was not paid as he was not at work and had no leave days, however this was not a deduction, but an employee did not render any services and therefore was not paid rightfully so.

6. DEGREE OF LATENESS:

6.1 Disputes of Unfair labour practice must be referred to CCMA within 90 days of the date of the act or omission.

6.2 It is interesting to note that the Applicant deliberately leaves out the information of how late his referral is. This is consistent with his untruthful character, he is aware how long is his matter out of the prescribed time frames of the CCMA, yet he does not state that information. The Applicant's referral is more than 40 days late; I submit that this is excessive, especially for someone who had all the time in the world to refer the matter to the CCMA.



6.3 According to the Applicant he got to be aware of what he needed to do in November or December due to mental issues, but the employee has been working all along since August without any mental issues, but the applicant failed to explain how he got to know of his oversight 40 days later. The employee was addressed by his management team and HR on the issue of unpaid leave. It is also difficult to accept the argument of mental issues from the applicant, as he raised the issue of the alleged victimization in August or September, maybe that was also an oversight. So, when the matter of victimisation was dealt with in August or September why did the employee not refer the matter of unfair deduction then, as clearly there were no mental problems then. The mental issues are an excuse and this whole referral is an afterthought matter. The applicant's failure to take accountability of his lateness and lack of proper pleadings about the degree of lateness on his part, shows his lack of regard for the commission, its processes, and resources.

6.4 The rationale for referring the matter within the required time frames is *inter alia*, to avoid potential prejudice that the Applicant may suffer at the hands of the Respondent. In this case it cannot be argued that the delay was occasioned by the Respondent but by the sloppiness and lack of regard of the most reasonable periods placed by the legislature.

7 REASON FOR LATENESS:

7.1 It is the employer's contention that the Applicant has not provided a compelling explanation for the lateness of his referral to the CCMA. It is interesting to note that the Applicant in his referral claims that he only became aware of what he needed to do in November 2025 but that is not

true as the employee raised another matter with Cheryl-Dawn who is a HR business partner in August/September. Applicant still does not explain how he became aware of his rights in this matter or the lack thereof, this is difficult to accept as the truth as the applicant is the member of the union and if he needed advice he would have gotten it from union representative. It is further interesting that the employee chose not to include this information in his referral affidavit to the CCMA but just makes references that he was ignored by the company.

7.2 The Respondent made the Applicant aware of the reasons why he was not fully paid on the 23rd of July 2025, when the applicant enquired about the shortfall on his salary, however the applicant only referred the matter to CCMA on the 5th of December 2025 which was more than 40 days late and more than 120 days later from the date of the act or omission.



7.3 The reasons advanced by the Applicant are not truthful, and for that reason alone the CCMA must not entertain this application. In the matter between **Molla v Shal & NO others (2007) 28 ILJ 1028 (LAC)**: Zondo JP as he then was with McCall AJA and Kruger AJA. Concurring held: [34] "I do not have the slightest hesitation in concluding that this is a case where the period of delay is excessive and the appellant's purported explanation for the delay is no explanation at all. If ever there was a case in which one can conclude that good cause has not been shown for condonation without considering the prospects of success, then this is the one. Where in an application for condonation, the delay is excessive and no explanation has been given for that delay or an "explanation" has been given but such explanation amounts to no explanation at all, I do not think it is necessary to consider the prospects of success."

7.4 The Applicant has failed to proffer plausible reasons for the delay. On this ground alone the application should be dismissed, and this abuse of the CCMA services should be condemned with the contempt that it deserves.

8 PROSPECTS OF SUCCESS:

8.1 I submit the Applicant has no prospects of success to the main case. The Respondent acted in line with the law and its own policy that when an employee is sick it will utilize sick leave, once sick leave is exhausted, and employee continues to be absent it will utilize annual leave in lieu of sick leave and once all leaves are exhausted it will then apply unpaid leave. There is nothing illegal about that and the lack of explanation from the applicant as to how the respondent acted unlawfully shows clearly that the employee does not know what he is referring to. The contractual obligation between the respondent and the applicant is that the employer will pay an employee after the employee has rendered his services and not merely for being an employee. It is very clear on what the Applicant did, that to breach the rule as he did, the

consequences thereof of are a dismissal. There is no evidence nor reasons that the applicant will provide to show that he was supposed to be paid a full salary for the month of July.

8.2 The Applicant's submission lacks explanation on the prospect of success, and a mere claim of unfair labour practice cannot be seen as a prospect of success, and the matter has been wrongly referred as well.

8.3 The applicant did not even follow an internal process of formally logging a grievance regarding the alleged unlawful deduction after it was addressed in August but decided to only communicate about it in November which is after the prescription period and then on the 4th of December wrote to the company executive members just before he referred the matter to the CCMA. The respondent submits that this is just a ploy by an applicant to force the company to pay him salary that he is not legally entitled to.



8.4 The respondent submits that it did everything to help the employee with his absence to an extent that there was special leave approved for him at the time because the respondent was acting as a responsible employer. The issue of unpaid leave was addressed by his management team (Mapule Mashale) who is the senior manager and HR team that supports the Gems business unit. The respondent will argue that the applicant was addressed on the issue of unpaid leave several times.

8.5 The respondent submits that the applicant has really no prospects of success as his reasons given around victimization and intimidation have no relevance but are used to sensationalize his dispute and have nothing to do with main dispute. It further makes it difficult even to understand as to what the real dispute of the applicant is.

8.6 The applicant has failed to discharge compelling reasons that are convincing that he has prospects of success other than mere accusations of victimization and intimidation. Therefore, the respondent submits that the condonation application of the applicant should be dismissed as there are no prospects of success should the matter go to the main case. The applicant has not even demonstrated that he has a reasonable chance of succeeding nor has he provided facts and evidence to support his claims.

9 **PREJUDICE:**

9.1 Having regard to submissions made above it is my view that the Respondent will suffer severe prejudice should the application be granted because the Respondent will be required to use resources that are Joburg based to fly to Cape Town to deal with this unsubstantiated dispute, considering that the delay has been caused by the applicant in this instance. Respondents will also have to take management and HR out of the office to bring them to the CCMA as witnesses for something that the applicant is aware of why he was not paid and that management assisted him to not be further negatively affected by his absence through approving special leave for him.

9.2 It would be unfair to now hold the Respondent to ransom given that the Applicant has erred in this regard.

9.3 It will be prejudicial for the respondent to be expected to go through the main case without any good cause shown by the applicant in terms of the prospects of success in the main.



In conclusion:

The Applicant's affidavit is riddled with untruthful submissions, meaning he has not been truthful or forthcoming and expects the CCMA to grant him condonation even though his application is just a mere ask without factual basis. This is just a fishing expedition and abuse of the system. It is therefore my considered view that this application should be dismissed with cost.

The applicant will not succeed in the main case if the reasons is just a claim of unlawful deduction and victimisation.

It is further submitted that the Applicant does not have prospects of success in the main application as if the Respondent's view that there is no unfair labour practice that the applicant has been subjected to.

It is further submitted by the respondent that whether there is good cause shown on the lateness but that cannot be the only determining factor in granting of condonation, but the applicant should satisfy all the elements which I believe he has not given sufficient evidence or explanation in terms of prospects of success and prejudice.

It is clear that the applicant will not succeed in the main if the reasons is just a claim of unlawful deduction or invalid no work no pay claim.

~~Intervall~~

Deponent

I certify that on this 10 day of December 2025, the deponent signed the declaration in my presence and declared that:

- a) I know and understand the contents hereof;
- b) I have no objection to taking this oath;
- c) I consider this oath to be binding on my conscience and uttered the words: "I swear that the contents of the declaration are true, so help me God."



Commissioner of Oaths

CS
7268154-3
Nelson J.B.

SOUTH AFRICA
COMMUNITY SERVICE
2025-12-10
FLORIDA
SUID-AFRIKAANSE POLISIEDIENS



**IN THE COMMISSION FOR CONCILIATION MEDIATION AND ARBITRATION HELD AT
CAPE TOWN**

CASE NO.: **WEC123564-25**

In the matter between:

WILBUR MATTHEE APPLICANT

and

**MEDSCHEME HOLDINGS (A Division of AfroCentric Health)
RESPONDENT**



**APPLICANT'S REPLYING AFFIDAVIT TO THE RESPONDENT'S OPPOSITION TO
CONDONATION**

I, the undersigned, WILBUR WILLIAM MATTHEE, do hereby make oath and state:

1. INTRODUCTION AND AUTHORITY

1.1. I am an adult male, the Applicant in this matter, and a Health Services Management Nurse Specialist (Care Manager) employed by the Respondent at its GEMS business unit in Cape Town.

1.2. The facts herein are within my personal knowledge, true and correct, unless otherwise stated. I have personal and direct knowledge of all events and communications referenced.

1.3. I am opposing the Respondent's application to dismiss my condonation request and will demonstrate that: (a) the harm was continuous, placing my referral within time; and alternatively, (b) compelling good cause exists for condonation.

2. CORRECTION OF FACTUAL INACCURACIES AND UNFOUNDED ATTACKS

2.1. I must correct material errors and reject unfounded attacks in the affidavit of Mr. Manyathi, which undermine its credibility.

2.2. Correction of My Position: I am not a "care coordinator" as dismissively stated, but a Health Services Management Nurse Specialist (Care Manager). This reflects a pattern of the Respondent's inattention to accuracy.

2.3. Rejection of the Irrelevant "Dismissal" Argument: My case concerns unfair labour practices, not dismissal. The Respondent's unsolicited argument that my actions warranted dismissal is highly prejudicial and attempts to improperly influence the Commissioner.



2.4. Rejection of the Inappropriate Prayer for Costs: The demand for costs in a CCMA condonation application is punitive and underscores the Respondent's aggressive posture to intimidate me, given my documented financial distress.

2.5. Rejection of Character Attacks: I categorically reject allegations of an "untruthful character." My submissions are backed by documentary evidence. The Respondent's personal attacks are a strategy to distract from their lack of substantive defence.

SECTION 3: THE RESPONDENT'S AFFIDAVIT AS ESCALATING VICTIMISATION & INSTITUTIONAL MALICE

3.1 Aggravated Attack on Protected Characteristics & Medical Condition

The Applicant is compelled to address the content and tone of the Respondent's Affidavit by Mr. Xollie Vincent Manyathi. This document does not constitute a legal defence but represents a continuation and escalation of the very unfair labour practices at issue, providing compelling grounds for enhanced, punitive damages.

3.1.1 The affidavit deliberately and maliciously dismisses my medically documented psychiatric condition—a direct consequence of the Respondent's own discriminatory and victimising conduct—as a cynical "excuse" and "afterthought". This is not legal argument; it is a direct attack on a protected characteristic (disability/medical condition) that compounds the initial discrimination.

3.1.2 By weaponising my mental health crisis—caused by their actions—against me in a formal legal proceeding, the Respondent demonstrates institutional contempt for employee wellbeing and a shocking disregard for the duty of care. This transforms a health condition into a tool for further victimisation.

3.2 Financial Intimidation & Weaponisation of Process

3.2.1 The Respondent's aggressive and unwarranted demand for costs in a routine condonation application, made with full knowledge of my documented financial destitution (as detailed in previous communications), serves no legitimate legal purpose.



3.2.2 This demand is a deliberate act of financial intimidation, calculated to exert psychological pressure and deter me from pursuing my rights. It exploits my vulnerable financial position—a position created by the Respondent's own unlawful deductions—to threaten further ruin.

3.3 Bad Faith, Personal Attacks & Abuse of Process

3.3.1 The affidavit is riddled with gratuitous personal attacks, labelling me as “untruthful” and characterising my application as an “abuse of the CCMA services”. These are not legal submissions but ad hominem assaults intended to poison the well and discredit me personally.

3.3.2 This demonstrates the Respondent's utter lack of remorse, systemic bad faith, and commitment to continued hostility. It confirms that the initial misconduct was not an aberration but reflective of an organisational culture that attacks rather than addresses complaints.

3.3.3 The affidavit itself thereby eliminates any conceivable possibility of restoring the employment relationship. It proves the victimisation is not historical but ongoing and institutionalised, perpetrated now through the Respondent's legal representatives.

3.4 This Affidavit as Independent Grounds for Maximum & Punitive Damages

3.4.1 The Respondent's choice to respond to serious discrimination allegations with further aggression in a sworn affidavit constitutes an independent, post-referral unfair labour practice (victimisation). It is a conscious decision to escalate rather than remediate.

3.4.2 This conduct warrants a significant punitive damages premium in any award. It proves the need for a deterrent sanction to signal that using legal process to further victimise complainants is unacceptable.

3.4.3 The Commissioner is urged to consider this affidavit not merely as a defence to condonation, but as powerful evidence of the Respondent's character and the incoherence for a maximum compensatory and punitive award.



4. THE TRUE NATURE OF THE DISPUTE: A CONTINUOUS PATTERN OF UNFAIR LABOUR PRACTICES

4.1. This dispute encompasses multiple, interconnected unfair labour practices forming a continuous course of conduct from July 2025 to December 2025:

- a. Unfair Labour Practice – Unlawful Deduction & Failure to Accommodate (July 2025).
- b. Unfair Discrimination & Harassment (the "Wilma" incident, 23 September 2025).
- c. Victimisation & Retaliation (doubled unpaid leave, punitive management).
- d. Institutional Bad Faith & Procedural Failure (30-day silence on grievances, whitewashing of complaints, bad-faith de-escalation).

5. THE UNLAWFUL NATURE OF THE JULY 2025 SALARY DEDUCTION

5.1. The Respondent misrepresents the July 2025 incident as a lawful "no work, no pay" application. In reality, its implementation was procedurally unfair and discriminatory.

5.2. Failure to Consult and Lack of Procedural Fairness: The Respondent deducted over 40% of my salary without any prior consultation, despite my explicit request to stagger the amount to prevent destitution. This refusal to engage constituted a lack of procedural fairness.

5.3. Failure to Provide Reasonable Accommodation: My absence was due to a documented psychiatric condition. The employer has a duty to make reasonable accommodation. My request to stagger the deduction was a minimal adjustment. The refusal, while I was in a vulnerable post-hospitalization state, constitutes discrimination based on my medical condition.

5.4. Derogatory Comments and Victimization: The manager's comment that "The Respondent is not a loan company" followed by I "would not feel the missing chunk... because I receive online deliveries weekly" was derogatory and proves the refusal to accommodate was motivated by malicious intent, converting a deduction into an act of victimisation.



6. THE CONTINUOUS HARM AND INSTITUTIONAL BAD FAITH THAT FOLLOWED

6.1. The July incident established a pattern of unfairness that continued unabated, demonstrating a continuous course of conduct:

- a. September 2025: I experienced public discrimination (the "Wilma" incident) and immediate retaliation (doubled unpaid leave).
- b. October 2025: Senior management whitewashed the discrimination complaint as a "typo," HR and union accept demonstrating institutional bad faith.
- c. November 2025: I filed a formal grievance about the hostile environment. The Respondent met this with 30 days of institutional silence. HR and union.
- d. Early December 2025 – The Escalation and De-escalation:
 - i. On 4 December 2025, after 30 days of silence from HR and the union followed by +20 days silence by senior management, I escalated the entire matter to executive leadership.
 - ii. The executive leadership de-escalated the matter back to the very department and manager accused of causing the harm.
 - iii. Faced with being returned to the source of the problem, I had no reasonable alternative and filed the CCMA referral.
 - iv. The de-escalated department contacted me to schedule a meeting notifying them of the case is now with the CCMA. I stipulated basic safeguards: a clear agenda and a recorded meeting to ensure transparency.
 - v. The department refused these basic terms, creating a procedural deadlock.

vi. I therefore formally notified the department that I could not attend without safeguards and would seek oversight elsewhere.

vii. As the next logical step, and true to my word, I formally notified the Board's Social & Ethics and Risk Committees the following day. This was a good-faith report of a material governance failure.

6.2. This sequence proves my actions were sequential, reasonable, and forced by the Respondent's conduct. The "delay" encompasses the period I was cycled through this broken internal process.

7. PRIMARY LEGAL ARGUMENT: THE HARM WAS CONTINUOUS, RESETTING THE REFERRAL CLOCK



7.1. Legal Principle: Where unfair labour practices constitute a continuing wrong, the 90-day period for referral resets for as long as the conduct persists. The last act in the continuum sets the clock *Gauteng Department of Health v Tsomakae* (GAJB 5554-16) [2017].

7.2. Application: The Respondent's misconduct was a continuous course of conduct that only culminated with the institutional bad faith and refusal to engage transparently in early December 2025. My referral is therefore within 90 days of the last confirming act of the continuing wrong.

8. ALTERNATIVE ARGUMENT: COMPELLING GOOD CAUSE FOR CONDONATION

Should the Commissioner find the continuous harm principle not fully applicable, I demonstrate compelling good cause.

8.1. Reason for Delay – Exhaustion of Remedies: Any delay was due to my good-faith exhaustion of every internal and governance channel, a process systematically thwarted by the Respondent. The CCMA referral was a last resort made promptly once this became clear.

8.2. Prospects of Success are Overwhelming: I possess direct evidence for each allegation (payslips, screenshots, emails, medical certificates). The Respondent's affidavit avoids engaging with this substance.

8.3. Prejudice: The prejudice to me—destitution, health crisis—if denied a hearing is catastrophic. Any administrative inconvenience to the Respondent is negligible.

9. THE RESPONDENT'S CLAIM OF "PREJUDICE" IS WITHOUT MERIT AND DEMONSTRATES ITS BAD FAITH

9.1. The Respondent claims in its affidavit (paragraph 9.1) that it will suffer "severe prejudice" should my application proceed because it would need to use "resources that are Joburg based to fly to Cape Town to deal with this unsubstantiated dispute."



9.2. This claim is disingenuous and reveals the Respondent's true disregard for the seriousness of this matter and the imbalance of power:

- a. ****Relative Prejudice is Wholly Mismatched:**** The Respondent, a large JSE-listed corporate entity, complains of the administrative and financial inconvenience of travel. I, an individual employee, am facing ****actual destitution, inability to afford food and medication, and severe health deterioration****. To equate corporate travel logistics with the threat to an individual's basic survival and health is not only irrational but morally indefensible. Any prejudice to the Respondent is purely financial and administrative; the prejudice to me is existential.
- b. ****The Respondent Created This Dispute Through Its Own Conduct:**** The Respondent's suggestion that it is being held to ransom is an inversion of reality. The dispute exists ****solely because of the Respondent's own actions****: its unlawful deduction, its discriminatory conduct, its victimisation, and its systemic failure to address my grievances. Having created the harm, it cannot now claim prejudice when called to account for it.
- c. ****Travel is a Normal Business Cost in a National Company:**** The Respondent, as a national healthcare administrator, operates across the country. Requiring managers to travel for a formal legal proceeding regarding serious misconduct within its ranks

is a normal cost of doing business and upholding the law. Framing this as an undue burden is an admission that it views its own legal and ethical accountability as an optional inconvenience.

D. ****Modern Technology Provides Alternatives:**** Should travel genuinely be a concern, the Respondent could have explored or proposed virtual mediation or arbitration. Its failure to do so and immediate leap to claiming “severe prejudice” underscores its strategy to use cost as a barrier to justice.

- d. ****The Claim Undermines the CCMA’s Authority:**** Implicit in the Respondent’s argument is the suggestion that an employee in one city should be barred from justice if the employer’s head office is in another. This strikes at the heart of the CCMA’s national mandate to provide accessible dispute resolution for all employees, regardless of geography.



9.3. Conclusion on Prejudice: The Respondent’s claim of prejudice is not a valid legal objection but a tactical one, designed to portray me as an inconvenience rather than a wronged employee seeking redress. The overwhelming and documented prejudice lies with me. To deny condonation based on the Respondent’s purported travel costs would be a grave miscarriage of justice, prioritising corporate convenience over an individual’s right to be heard on matters of discrimination, victimisation, and survival.

10. CONCLUSION

10.1. My referral is, on the correct legal analysis, within time due to the continuous nature of the harm. Alternatively, more than sufficient good cause exists.

10.2. To dismiss this application on a technicality would sanction the Respondent's strategy of institutional bad faith and deny justice.

11. PRAYER FOR RELIEF

I respectfully request that the Honourable Commissioner:

11.1. Find that my referral was made within the prescribed time due to the continuous harm.

11.2. In the alternative, grant condonation for the late referral.

11.3. Dismiss the Respondent's opposition.

11.4. Proceed to set the matter down for conciliation/arbitration.

11.5. Grant such further relief as deemed just.

SIGNED at DELFT on this 11TH day of December 2025.



WILBUR WILLIAM MATTHEE

APPLICANT

SWORN TO AND SIGNED BEFORE ME AT DELFT ON THIS 11TH DAY OF DECEMBER 2025, THE DEPONENT HAVING ACKNOWLEDGED THAT HE KNOWS AND UNDERSTANDS THE CONTENTS OF THIS AFFIDAVIT, THAT HE HAS NO OBJECTION TO TAKING THE PRESCRIBED OATH, AND THAT HE CONSIDERS THE OATH BINDING ON HIS CONSCIENCE.

COMMISSIONER OF OATHS

FULL NAME: DAVID JOHANNES

BUSINESS ADDRESS: →

APPOINTMENT EXPIRES:



11 DEC 2025

To whom it may concern,

It is my pleasure to write you a short motivation for Mr Wilbur William Matthee. My Name is Dr Priscilla MiJeong Ohm currently working with Mr Matthee within the Mitchell's Plain District hospital Emergency unit. The emergency unit is one that requires timeous action, fast thinking and definitive action as it has many moving parts including a high patient load, critically ill patients with undifferentiated pathology all needing urgent management. Mr Matthee carries all the above qualities. As a shift leader he allocates staff to needed areas with good skills distribution across the floor, furthermore he often fills in gaps needed as they arise to allow for continued workflow and patient care allowing for staff breaks without disruption of patient care. All shift related issues including high risk patients who have absconded, drug ordering, equipment issues as well as family communication as needed are always appropriately done or escalated with timeous feedback given to the doctor team leader. When students are allocated whether nursing, EMS or medical are within the unit during his work he allocates tasks to aid their learning and supervises each student within their patient care.



When it comes to patient care within the emergency unit, Mr Matthee shows depth of clinical knowledge which allows him to identify critically ill patients, as well as pre-empt needed management which allows for improve the efficiency of patient care within the unit. He is well versed in the management protocols commonly used within the unit and is able to remind doctors of when the next blood gases or ECGs are needed as well as the next step in management on the protocol. All emergency room doctors leave the resus patients once stabilised in Mr Matthee's care till next review time with no doubt that they will be correctly monitored, cared for and that they will be timeously informed if there is a change in condition.

One particular point of excellence for Mr Matthee is the way he is able to de-escalate any heated situation within the emergency unit whether it be with family, staff or patients with a calm tone and demeanour. He approaches all his patients with the same respect and as much as possible protects their dignity with simple actions as always closing curtains, giving patients privacy when needed and answering patient queries as well as informing family members of needed standard procedures. His method of communication and interaction with his patients has been a lesson and reminder to me to give good, individualised care and communication that maintains patient dignity and autonomy.

Regards,

Dr PMJ Ohm

ANNEXURE

“S”

Resignation letter

Date: 11 December 2025

Relevance: *Part L — Constructive Dismissal*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



FORMAL NOTICE OF RESIGNATION DUE TO AUTOMATICALLY UNFAIR CONSTRUCTIVE DISMISSAL

4 messages

Wilbur William Matthee <wilburmat@gmail.com> Fri, 12 Dec 2025 at 03:29
To: lebohang.mpumlwana@afrocentrichealth.com
Cc: ferfam.bruno@gmail.com, alice@aliceleroux.co.za, Gretchen.veldman@afrocentrichealth.com, gerald.vanwyk@afrocentrichealth.com, munisinf@global.co.za
Bcc: Sr Wilbur William Matthee <wilburmat@gmail.com>

TO:
The Board of Directors (via Company Secretary)
AFROCENTRIC HEALTH (RF) (PTY) LTD
37 Conrad Road, Florida North, 1709
CC:
Mr./Ms. Gerald Van Wyk (Chief Executive Officer)
Chair of the Audit & Risk Committee
Chair of the Social & Ethics Committee
Group Human Resources Department
DATE: 11 December 2025



Dear Board of Directors,

I, Wilbur Matthee, hereby give notice of the immediate termination of my contract of employment with Medscheme Holdings, effective 12 December 2025.

This resignation is not voluntary. I am terminating my employment because the employer, Medscheme Holdings (A Division of AfroCentric Health), has rendered the continuation of the employment relationship irretrievably intolerable through its malicious, calculated, and institutional conduct.

This termination constitutes an Automatically Unfair Constructive Dismissal in terms of the Labour Relations Act (LRA).

1. Grounds for Intolerability: Institutional Malice and Victimisation

The conditions rendering my continued employment intolerable are proven by the employer's formal legal submission in the matter of Wilbur Matthee and Medscheme Holdings (CCMA Case Ref. Number: WECT23364-25), specifically the Respondent's Answering Affidavit signed under oath on 10 December 2025.

The Affidavit contains specific statements that constitute severe and ongoing occupational detriment, victimisation, and unfair labour practice, including:

Malicious Character Attacks: The Affidavit labels me as having an "untruthful character," engaging in a "fishing expedition," and acting with "sloppiness and lack of regard" for the Commission. This ad hominem attack, under oath, fundamentally destroys the trust and confidence necessary for any employment relationship.

Abuse of Process: The document constitutes an abuse of the CCMA's services by employing tactics designed to discredit and cause further financial harm, rather than engaging in a good faith legal defence.

13/4/2026-11:58:23 AM

Institutionalised Victimization: The Affidavit explicitly dismisses my claims of victimisation and mental health issues as having "no relevance" and being used to "sensationalize" the dispute (Sections 6.3 and 8.5). This demonstrates that the victimisation is ongoing, officially sanctioned, and institutionalised through the Respondent's legal representation.

2. Legal Basis: Automatically Unfair Constructive Dismissal

The employer's conduct, as documented in the sworn Affidavit, makes this termination an Automatically Unfair Constructive Dismissal in terms of the following sections of the Labour Relations Act (LRA):

Section 186(1)(e): The employee terminated the contract of employment because the employer made continued employment intolerable.

Section 187(1)(h): The reason for the employer's conduct is directly linked to an act of retaliation against a Protected Disclosure (the P1 Material Risk Report and subsequent updates regarding discrimination and victimisation).



3. Conclusion and Formal Notice

The content of the Respondent's Opposing Affidavit eliminates any conceivable possibility of restoring the employment relationship. I consider myself to have been dismissed by the employer's malicious conduct.

I have already formally informed the Board of the governance failure related to this P1 risk and the ongoing victimisation. All further matters relating to this dispute, including the claim for maximum compensation and a demand for punitive costs (Attorney-and-Client scale), will be handled at the CCMA/Labour Court, and via my External Whistleblower disclosures to the JSE, FSCA, CMS, DEPARTMENT OF EMPLOYMENT & LABOUR - INSPECTION & ENFORCEMENT + B-BBEE, . Pending complete governance failure.

Yours sincerely,

Wilbur Matthee

Mail Delivery Subsystem <mailer-daemon@googlemail.com>
To: wilburmat@gmail.com

Fri, 12 Dec 2025 at 03:30



Address not found

Your message wasn't delivered to
Gretchen.veldman@afrocentrichealth.com
because the address couldn't be found, or is
unable to receive mail.

ANNEXURE

“T”

**CPUT Postgraduate Diploma in Occupational Health Nursing —
acceptance letter**

Date: 2026

Relevance: *Part M — Career Destruction*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

Dear Applicant

ANNEXURE: 2026 ACCEPTANCE POSTGRADUATE DIPLOMA IN OCCUPATIONAL HEALTH NURSING

Greetings on behalf of the Department of Nursing Science at the Cape Peninsula University of Technology (CPUT) and congratulations on your 2026 acceptance into the Post Graduate Diploma in Occupational Health Nursing.

Kindly note that this programme is offered face-to-face; it is not a distance learning course, and you will therefore be required to attend sessions on campus. This annexure provides important information regarding admission and orientation requirements to be met prior to your commencement of orientation at the Bellville Campus on Monday, 26 January 2026.

Should you accept the offer of study for this programme, kindly complete *Addendum D: Reply Slip and Personal Details in full* on page 14 and email to Ms Natalie Copeling at CopelingN@cput.ac.za within the stipulated timeframe. Failure to do so will result in your place being offered to the next suitable applicant. As per the South African Nursing Council (SANC) accreditation, only **60** students can be accommodated for this programme and this will be done on a first-come, first serve basis.



1. GENERAL INFORMATION

At CPUT, which is a University of Technology, students will be required to play an active role by using mobile learning throughout their studies, using their own data. It is therefore important that you have a device that allows you to connect to the Learner Management System (LMS) used to facilitate online sessions, specifically "My Classroom/Black Board Learning". Further to this, written assignments must be submitted in a typed format, and it is thus also necessary to have access to a device equipped with Microsoft Word, preferably a laptop.

2. COMMENCEMENT INFORMATION PART ONE

A **compulsory** orientation programme will take place from Monday, 26 January 2026 to Friday, 30 January 2026, commencing daily at 09:00.

PLEASE TAKE NOTE OF THE FOLLOWING STEPS TO BE TAKEN:

2.1 Step 1: Submission of Documents

Please submit hard copies of the documents below to the Department of Nursing Science between 26 January 2026 and 19 January 2026 (certification not older than three months):

- 2.1.1 Certified copy of ID (x 1)
- 2.1.2 Completed Application Form (Addendum A) for submission to SANC
- 2.1.3 Certified copy of 2025 SANC receipt as proof of registration with SANC as a professional nurse and midwife
- 2.1.4 Certified copy of Nursing Qualification(s) obtained (x 1)
- 2.1.5 Certified copy of National Senior Certificate (x 1)
- 2.1.6 Copy of 2025 Professional Indemnity (DENOSA, NEHAWU, SASOHN, etc.).
- 2.1.7 Certified copy of Marriage Certificate (if applicable) (x 1)
- 2.1.8 Completed CPUT Indemnity Form (Addendum B)
- 2.1.9 Completed Western Cape Government Indemnity Form (Addendum C)
- 2.1.10 Evidence of Hepatitis B Vaccination or Hepatitis B Immunity
- 2.1.11 Proof of payment of the SANC registration fee to register as a learner **payable to the university**; fees are subject to change therefore, kindly refer to the Government Gazette.

ANNEXURE

“U”

uFiling screenshot — ‘No Information’ for June 2025

Date: 2026

Relevance: *Part N — Post-Resignation Victimisation*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

Get Declaration	
AMERICAN RECRUITMENT SERVICES LIMITED (2020-2021)	
March 2021	
Salary:	92.00
Hours:	360
Start Work Date:	09-11-2020
End Work Date:	
Termination Reason:	
April 2021	
Salary:	93425.00
Hours:	46
Start Work Date:	09-12-2020
End Work Date:	
Termination Reason:	
May 2021	
Salary:	81533.00
Hours:	14
Start Work Date:	09-13-2020
End Work Date:	
Termination Reason:	
June 2021	
Salary:	82855.24
Hours:	14
Start Work Date:	05-12-2020
End Work Date:	
Termination Reason:	
June 2021	
Salary:	82855.24
Hours:	14
Start Work Date:	09-12-2020
End Work Date:	
Termination Reason:	
July 2021	
Salary:	90.00
Hours:	360
Start Work Date:	09-12-2020
End Work Date:	
Termination Reason:	
August 2021	
Salary:	89646.21
Hours:	49
Start Work Date:	05-12-2020
End Work Date:	
Termination Reason:	
September 2021	
Salary:	82179.49
Hours:	23
Start Work Date:	05-12-2020
End Work Date:	
Termination Reason:	
October 2021	
Salary:	87712.68
Hours:	77
Start Work Date:	05-12-2020
End Work Date:	
Termination Reason:	
November 2021	
Salary:	83664.72
Hours:	360
Start Work Date:	05-12-2020
End Work Date:	
Termination Reason:	
November 2021	
Salary:	83664.72
Hours:	160
Start Work Date:	09-12-2020
End Work Date:	
Termination Reason:	
November 2021	
Salary:	83078.92
Hours:	35
Start Work Date:	09-13-2020
End Work Date:	
Termination Reason:	
January 2022	
Salary:	81318.53
Hours:	12
Start Work Date:	05-12-2020
End Work Date:	
Termination Reason:	
February 2022	
Salary:	90.00
Hours:	360
Start Work Date:	09-13-2020
End Work Date:	
Termination Reason:	

Page 134 of 243



ANNEXURE

“V”

Certificate of Service (disputed) + Naming Convention request

Date: January 2026

Relevance: *Part N — Post-Resignation Victimisation*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



13/4/2026-11:58:23 AM

Request for Assistance: UI-19 Forms and Umbrella fund Benefit Documents

1 message

Wilbur William Matthee <wilburmat@gmail.com>
To: Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>
Bcc: Sr Wilbur William Matthee <wilburmat@gmail.com>

Wed, 07 Jan 2026 at 14:28

Dear Cheryl-Dawn,

I hope you are having a good week.

I am writing to kindly request your assistance with the necessary documentation required for my Unemployment Insurance Fund (UIF) application.



To help me meet my immediate financial needs and begin the claims process, could you please provide the completed UI-19 form and any other relevant exit documents?

Additionally, please let me know if there are any specific forms I need to complete from my side, or if you require any updated information (such as my current contact details or banking confirmation) to finalize these papers.

Thank you very much for your time and for helping me with this transition. I look forward to hearing from you.

Best regards,

Sr. W. Matthee

Nurse Specialist Health Services Management, Risk And Psychosocial Safety.

SRN. Com Health, Psychiatry, Midwifery; BTech; Bcur HNM; Post Grad HSM.



13/4/2026-11:58:23 AM

Retirement forms and Uif forms

2 messages

Wilbur William Matthee <info@ethichawks.co.za>

Wed, 14 Jan 2026 at 10:46

To: Sipokazi.Makaula@afrocentrichealth.com

Dear Sipikazi,

I trust you are well. I am just doing a follow up on my retirement claim forms and UIF forms. Could you kindly assist with these at your earliest convenience.

Much appreciated and thank you for your assistance in this matter.

Best regards

Wilbur William Matthee
RN, BCur (HMN), PG Dip HSM (NQF 8)



Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>

Thu, 15 Jan 2026 at 18:57

To: Wilbur William Matthee <info@ethichawks.co.za>

Good day ,

I hope you are well.

Please share your details Name , Surname, ID number , are you a exting employee or former employee, which business unit and your team lead/ managers name.

This will assist in me knowing how to best assist you.

Thank you.



Kind regards

Sipokazi Makaula

Human Capital Consultant Human Capital

- Sipokazi.Makaula@afrocentrichealth.com
- Searle Street, The Boulevard, Woodstock, Cape Town, 7925
- www.afrocentric.za.com

"Always aligning our leaders and people with the business purpose"



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From: Wilbur William Matthee <info@ethichawks.co.za>
Sent: Wednesday, January 14, 2026 10:46 AM
To: Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>
Subject: Retirement forms and Uif forms

This message originated from outside your organization

Dear Sipikazi,
[Quoted text hidden]



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13/4/2026-11:58:23 AM

URGENT: NOTICE OF STATUTORY NON-COMPLIANCE (UIF) AND SYSTEMIC WITHHOLDING OF EXIT DOCUMENTS

1 message

Wilbur William Matthee <info@ethichawks.co.za>

Thu, 15 Jan 2026 at 20:43

To: Sipokazi.Makaula@afrocentrichealth.com

Cc: Cheryl-DawnM@afrocentrichealth.com, Lindiwe.Miyambu@afrocentrichealth.com,

Tebogo.Makoe@afrocentrichealth.com, Monwabisi Kula <monwabisi.kula@afrocentrichealth.com>

Bcc: wilburmat@gmail.com

Dear Sipokazi,

I refer to your email dated 15 January 2026.

1. Rejection of Information Request:

Your request for my basic identity and employment details is rejected as a bad-faith stalling tactic. As a Human Capital Consultant, you have access to the Respondent's payroll records. Furthermore, the Respondent's legal representative, Mr. Xollie Vincent Manyati, authored a sworn Opposing Affidavit on 10 December 2025 (Case WECT23564-25) identifying me and my role.



2. Ignorance of Prior Requests:

Please note that I formally requested these documents from Cheryl-Dawn Modern on 7 January 2026. Her complete ignorance of that request and subsequent failure to provide the documents—despite my follow-up with you on 14 January—is a continuation of the institutional silence and victimization documented in my legal filings.

3. Evidence of UIF Non-Compliance (June 2025):

Attached is a screenshot from the uFiling portal showing that the Respondent failed to submit a declaration for June 2025. My payslip for that month reflects that UIF was deducted from my salary. This discrepancy constitutes a violation of the Unemployment Insurance Contributions Act. I require immediate proof of payment and an updated declaration for June 2025 to ensure my claim is not rejected by the Department of Employment and Labour (DEL).

4. Formal Notice to Leadership:

(To Lindiwe Miyambu, Tebogo Makoe, and Monwabisi Kula) Please be advised that I am formally filing my Founding Affidavit in the Labour Court on Tuesday, 20 January 2026. The systemic failure to provide my UI-19, retirement forms, and the missing June 2025 declaration will be included as evidence of institutional bad faith and continued occupational detriment.

5. Final Demand:

I require the following by 12:00 PM on Monday, 19 January 2026:

My completed UI-19 form;

My Service Certificate and Umbrella Fund Withdrawal Forms;

Proof of UIF payment and declaration for June 2025.

Failure to comply will result in a formal complaint to the DEL and a request for a statutory audit.



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13/4/2026-11:58:23 AM

Correction Required – Certificate of Service Misrepresentation

4 messages

Wilbur William Matthee <info@ethichawks.co.za>

Mon, 19 Jan 2026 at 12:25

To: Monwabisi Kula <monwabisi.kula@afrocentrichealth.com>, Lindiwe.Miyambu@afrocentrichealth.com
Cc: Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>, Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>, Tebogo.Makoe@afrocentrichealth.com, nthabisengn@medscheme.co.za
Bcc: wilburmat@gmail.com

Dear Sipokazi,

I acknowledge receipt of the Certificate of Service. However, I must formally object to the fact that my role has been adjusted to align with the Opposing Affidavit dated 10 December 2025, rather than reflecting my actual employment records.

The Certificate of Service is a statutory document required for UIF and fund processing. It must be accurate and consistent with payroll and HR records, not tailored to litigation pleadings. Adjusting my role to fit an affidavit constitutes misrepresentation and undermines compliance with the Basic Conditions of Employment Act.



I further note that both you and Nthabiseng were present in the recorded Teams meeting titled "Naming Convention", where the Operational Manager encouraged us to update our statistics. That meeting provides direct evidence that the employer had full knowledge of my role, identity, and responsibilities. Any deviation from those facts in the Certificate of Service is therefore unjustifiable.

I therefore request that the Benefits Team urgently issue a corrected Certificate of Service that reflects my true role and employment details, as per payroll and HR records and the recorded meeting. As a result, of this failure, that is evidenced by your not meeting your 12:00 deadline, I require correct Certificate of Service by close of business 19 January 2026.

Should this not be provided within the stipulated timeframe, I will proceed to file a formal complaint with the Department of Employment and Labour (DEL) and request a statutory audit of the employer's compliance.

Please confirm by return email before close of business.

Kind regards,

Wilbur William Matthee
RN, BCur (HMN), PG Dip HSM (NQF)

<postmaster@medscheme.co.za>

Mon, 19 Jan 2026 at 12:26

To: info@ethichawks.co.za

Delivery has failed to these recipients or groups:

tebogo.makoe@afrocentrichealth.com

A communication failure occurred during the delivery of this message. Please try resending the message later. If the problem continues, contact your email admin.

The following organization rejected your message: AM4PEPF00027A62.mail.protection.outlook.com.

13/4/2026-11:58:23 AM

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[Quoted text hidden]



Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>
To: info@ethichawks.co.za <info@ethichawks.co.za>
Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>

Dear Wilbur

Thank you for your e-mail,

Ito correspondences, from an HR perspective, I would like to confirm that we responded within the stipulated timeframe and provided all information required from our side. Furthermore the Certificate of Service issued is correct and fully aligned with our employment records as well as statutory requirements.

With regard to the naming-convention changes, please note that this business process is still in progress, and I can confirm that no changes to the Care Co-Ordinator job title for your department have been implemented at this stage. Your title is therefore accurately and correctly reflected in the Certificate of Service.

Should you require any additional documentation, we are available to assist; however, our position regarding the accuracy of the certificate remains unchanged.

Kind Regards

Cheryl-Dawn Modern

Senior HR Business Partner

+27 (011) 671 4819

cheryl-dawnm@afrocentrichealth.com

37 Conrad St, Florida North, Roodepoort | PO Box 1101, Florida Glen, 1709

www.afrocentric.za.com



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From: Wilbur William Matthee <info@ethichawks.co.za>

Sent: Monday, January 19, 2026 12:25 PM

To: Monwabisi Kula <Monwabisi.Kula@afrocentrichealth.com>; Lindiwe Miyambu <Lindiwe.Miyambu@afrocentrichealth.com>

Cc: Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>; Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>; Tebogo.Makoe@afrocentrichealth.com <Tebogo.Makoe@afrocentrichealth.com>; Nthabiseng Nkatane <nthabisengn@medscheme.co.za>

Subject: Correction Required – Certificate of Service Misrepresentation

This message originated from outside your organization



Dear Sipokazi,

[Quoted text hidden]

[Quoted text hidden]

 **image014.png**
29 KB

Wilbur William Matthee <info@ethichawks.co.za>

Mon, 19 Jan 2026 at 16:30

To: Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>

Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>

Bcc: wilburmat@gmail.com

Dear Cheryl-Dawn,

Thank you for the information provided.

You have stated that the company was compliant with the statutory timeline. However, the documents I received were incomplete, which means the company is in fact not compliant with statutory requirements.

For the record:

- I verbally accepted the updated role during the recorded Teams meeting titled "Naming Convention", attended by Sipokazi Makaula and Nthabiseng Nkatane, where the Operational Manager informed me to update my identifiers.
- I subsequently updated my professional signature to reflect the new title.
- Both the recording and my updated signature constitute legally binding confirmation of the naming convention.

Despite this, the Certificate of Service issued contradicts both the recorded meeting and my updated signature, and has been aligned with the employer's Opposing Affidavit dated 10 December 2025. This alignment is illegal, as statutory documents must reflect factual employment records, not be tailored to litigation pleadings.

As a result, the current Certificate of Service will form part of my legal proceedings. Unless a corrected version is provided by close of business today, 19 January 2026, I will proceed to file a formal complaint with the Department of Employment and Labour (DEL) and include this matter in my Labour Court affidavit.

Kind regards,



Wilbur Mat... 2025/10/15



to me ▾



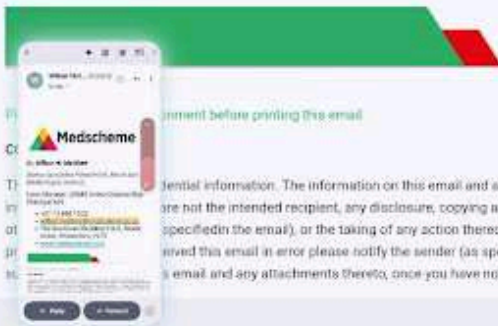
Medscheme

Sr. Wilbur W. Matthee

*(Nurse Specialist PGrad HSM, Btech Gen P
(Midw, Psych, Comm))*

Case Manager: GEMS Active Disease Risk
Management

- +27 12 466 1332
- wilbur.matthee@medscheme.co.za
- The Boulevard Building F & G, Searle street, Woodstock, 7925
- www.medscheme.com



comment before printing this email

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Reply



#

Forward





Request for Disclosure of Recorded Teams Meeting – “Naming Convention”

1 message

Wilbur William Matthee <info@ethichawks.co.za>
To: Cheryl-Dawn Modern <Cheryl-DawnM@afrocentrichealth.com>
Cc: Nthabiseng Nkatane <nthabisengn@medscheme.co.za>, Sipokazi Makaula <Sipokazi.Makaula@afrocentrichealth.com>
Bcc: wilburmat@gmail.com

Mon, 19 Jan 2026 at 17:24

Dear Cheryl, Sipokazi, and Nthabiseng.

I hereby formally request disclosure of the full and unaltered Microsoft Teams recording titled “Naming Convention”, attended by Sipokazi Makaula and Nthabiseng Nkatane. This recording is in the Respondent’s possession and constitutes material evidence directly relevant to the dispute concerning my designation and the accuracy of the Certificate of Service.



Please provide the recording by [reasonable deadline, e.g., 5 working days]. Should the Respondent fail to disclose the recording, I will proceed to seek an order compelling production in terms of Rule 6(7) of the Labour Court Rules and invite the Court to draw an adverse inference from any refusal to disclose evidence within your possession.

Kind regards,

Wilbur Matthee

Specialist Nurse Health Services Manager
SRN | BTech Psych, Midw, Community | BCur HNM | PG Dip HSM (NQF 8)

ANNEXURE

“W”

CMS referral — ‘Administrative Misconduct and Ethical Failure’

Date: 2026

Relevance: *Part O — Regulatory Confirmation*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



Administrator Misconduct, Ethical Failure & Retaliation – Request for Investigation

6 messages

Wilbur William Matthee <info@ethichawks.co.za>
To: complaints@medicalschemes.co.za
Bcc: wilburmat@gmail.com

Tue, 23 Dec 2025 at 13:02

CMS – Council for Medical Schemes

Subject: Administrator Misconduct, Ethical Failure & Retaliation – Request for Investigation



Dear CMS Compliance & Investigations Unit,

I request CMS intervention regarding:

- discrimination
- retaliation
- unlawful deductions
- institutional silence
- executive de-escalation of my PDA
- board-level governance failure

These failures compromise:

- ethical administration
- beneficiary protection
- scheme governance

- whistleblower protection
- independent investigation
- oversight of Medscheme's governance practices

13/4/2026-11:58:23 AM

Sincerely,

Wilbur William Matthee
RN, BCur (HMN), PG Dip HSM (NQF 8)

2 attachments

-  **Sworn Affidavit evidence bundle.pdf**
342 KB
-  **Regulatory Full Master bundle ready for regur final-compressed (1).pdf**
4.4 MB



Complaints <complaints@medicalschemes.co.za>
To: Wilbur William Matthee <info@ethichawks.co.za>

Fri, 09 Jan 2026 at 11:22

Good day,

We acknowledge receipt of your email.

Please note that in order for your matter to be formally registered it is required that you complete and submit the documentation and/or correspondence as seen below. We request that you submit your complaint together with the following documents, if applicable: In a single email.

- A completed complaint form which we have attached herein for your attention;
- If acting on behalf of a member/beneficiary, please attach a letter of consent, written and signed by the member /beneficiary of the medical scheme.

Your matter will only be attended to upon receipt of all the requested documents.

The complaint should be e-mailed to complaints@medicalschemes.co.za

We are looking forward to your response.

Kind Regards,

13/4/2026-11:58:23 AM

Polite

Complaints

Council for Medical Schemes
t:
e: complaints@medicalschemes.co.za
Disclaimer
[Quoted text hidden]



3 attachments

-  **Sworn Affidavit evidence bundle.pdf**
342 KB
-  **Regulatory Full Master bundle ready for regur final-compressed (1).pdf**
4.4 MB
-  **UPDATED-COMPLAINT-FORM-AND-CHECKLIST_APRIL-2024-ft.docx**
706 KB

Wilbur William Matthee <info@ethichawks.co.za>
To: Complaints <complaints@medicalschemes.co.za>

Mon, 12 Jan 2026 at 21:12

Dear Polite,

I acknowledge receipt of your email and the attached documents.

As a direct result of my employer’s conduct and ongoing retaliation, I am currently in a humanitarian crisis and destitute. Due to an unlawful 40% salary deduction, I was unable to pay the necessary fees to register as a specialist nurse, legally barring me from practicing in 2026.

Furthermore, the conciliation meeting on January 9, 2026, resulted in an insulting offer of only one month's remuneration. This follows a year of discrimination, victimization, and retaliation after I reported P1 risks and made protected disclosures. I am now facing severe prejudice, imminent eviction, and homelessness.

I will complete and return the required documents as soon as possible.

Kind regards,

Sr. Wilbur Matthee

Sr. Wilbur Matthee

Specialist Nurse Health Services Manager
SRN | BTech Psych, Midw, Community | BCur HNM | PG Dip HSM (NQF 8)

[OBJ]

[Quoted text hidden]

Wilbur William Matthee <info@ethichawks.co.za>
To: Complaints <complaints@medicalschemes.co.za>

Tue, 20 Jan 2026 at 15:47

Dear Polite,



I am writing in my capacity as a registered nurse, in strict adherence to my professional and ethical obligations as outlined by the South African Nursing Council (SANC).

Pursuant to our earlier correspondence, please find attached the following requested documents in support of the formal complaint being lodged:

1. Facts of the complaint – including supporting email evidence.
2. Dispute resolution steps undertaken – with proof of escalation and correspondence.
3. Recourse required – as outlined in the attached summary.

These documents have been prepared in compliance with SANC's Code of Ethics and Professional Standards, which mandate the reporting of conduct that jeopardises patient safety, data integrity, or ethical healthcare administration.

Should you require any further information or clarification, please do not hesitate to contact me.

Thank you for your attention to this serious matter.

Yours sincerely,

Attachments:

- Complaint Summary & Evidence.pdf
- Escalation Proof.pdf
- Required Recourse.pdf

Sr. Wilbur Matthee

Specialist Nurse Health Services Manager
SRN | BTech Psych, Midw, Community | | PG Dip HSM (NQF 8)

[Quoted text hidden]

2 attachments

13/4/2026-11:58:23 AM

 **UPDATED-COMPLAINT-FORM-AND-CHECKLIST_APRIL-2024-ft.pdf**
243 KB

 **General complaint was called insurbordanation .pdf**
235 KB

Complaints <complaints@medicalschemes.co.za>
To: Wilbur William Matthee <info@ethichawks.co.za>

Wed, 04 Feb 2026 at 12:11

Good day,

We acknowledge receipt of your email,



Regrettably, we will not be able to proceed with the registration and investigation of your complaint because it does not fall within our jurisdiction as the Council for Medical Scheme (CMS). The CMS is mandated by the Medical Schemes Act register and investigate complaints against Medical Schemes, Administrators, Brokers and Managed Health Care Facilities.

Please note that complaints against Health Professionals are regulated by the Health Professions Council of South Africa (HPCSA). We hereby recommend that you refer the attached complaint to the Health Professions Council of South Africa (HPCSA) in writing. Please see below the HPCSA contact details: <https://www.hpcsa.co.za/>

Client Care Centre

Professionals and members of the public are encouraged to call the HPCSA Client Care Centre for all general enquiries including annual fees, registrations, compliments, service delivery, certificates of status, CPD, ethical queries and undesirable business practices.

Telephone	Postal Address	Physical Address	Self-Service
(+27) 12 338 9300/01	PO Box 205 Pretoria 0001	553 Madiba Street Madiba Street Arcadia 0083	Visit our online portal on the following link: https://hpcsaonline.custhelp.com/
NATURE OF QUERY		E-mail	
Certificate of status		hpcsaacs@hpcsa.co.za	
Complaints against practitioners		Legalmed@hpcsa.co.za	
Education & Training		Education&Training@hpcsa.co.za	
Service Delivery		servicedelivery@hpcsa.co.za	

Kind regards,

[Quoted text hidden]

13/4/2026 11:58:23 AM
Fri, 06 Feb 2026 at 10:54

Wilbur William Matthee <info@ethichawks.co.za>
To: Complaints <complaints@medicalschemes.co.za>

Thank you for the feedback.

Kind regards

Sr. Wilbur Matthee

[Quoted text hidden]

 **image001.png**
41 KB



TO: The Registrar / Commissioner, Council for Medical Schemes**E:** complaints@medicalschemes.co.za**REF:** CMS-REQUEST-3 | High Court Case No. HCGS951842 | B-BBEE Commission Case No. 5/02/2026**DATE:** 2 April 2026**RE: FORMAL FOLLOW-UP — JURISDICTIONAL RE-ENGAGEMENT REQUIRED
SYSTEMIC GEXUS-DMS INTEGRATION FAILURE CONSTITUTES
MALADMINISTRATION UNDER THE MEDICAL SCHEMES ACT 131 OF 1998**

Dear Registrar / Commissioner,

I write in formal follow-up to my correspondence of 20 March 2026 (CMS-REQUEST-3), which updated your office regarding the unremediated Gexus-DMS system integration failure at Medscheme Holdings (Pty) Ltd and its nexus to my urgent Section 162 Director Delinquency Application (HCGS951842, Western Cape High Court).

This letter serves to place the CMS on formal notice that the jurisdictional declination issued on 4 February 2026 — directing this matter to the Health Professions Council of South Africa — was premised on a characterisation of the disclosure as a complaint against a health professional. That characterisation does not reflect the substance of the matter now before this office. The disclosure concerns a systemic technical integration failure in Administrator infrastructure. As set out below, that failure falls squarely and inescapably within the CMS's statutory mandate.

I respectfully submit that a failure to engage substantively with this correspondence, in the context of a documented Priority 1 integration gap that continues to affect GEMS beneficiaries, would itself constitute a dereliction of the Council's statutory duty under the Medical Schemes Act 131 of 1998 ("the MSA").

1. REFRAMING THE DISCLOSURE: FROM PROFESSIONAL CONDUCT TO ADMINISTRATOR MALADMINISTRATION

The CMS's February 2026 declination was appropriate insofar as it related to any complaint against a health professional. That referral to HPCSA and SANC stands without objection.

However, the primary subject of this file is categorically distinct. The Gexus-DMS integration failure is a defect in Administrator infrastructure — the technical systems and processes through which Medscheme discharges its obligations as an Accredited Administrator under the MSA. Specifically:

- The Disease Management System (DMS) continued to flag GEMS member 000655789 Dependent 02 as non-adherent from December 2024 onwards, despite the member transitioning to the chronic benefit from January 2025 onward.
- The Gexus claims system recorded chronic benefit claims from April 2023 onward. DMS did not receive or recognise this update.
- As a result, automated non-adherence letters were dispatched on 19 May 2025 and 19 October 2025 — clinically incorrect communications sent to a compliant, elderly beneficiary.
- A Root Cause Analysis (RCA) confirmed the integration gap as a structural design flaw — not an isolated clerical error — affecting all members undergoing the acute-to-chronic benefit transition.

This is not professional misconduct. It is systemic Administrator maladministration within the precise meaning of Section 1 of the MSA, and it falls unambiguously within CMS jurisdiction.

2. THE CMS'S STATUTORY DUTIES — ENGAGED AND UNMET

Four distinct statutory obligations are triggered by this disclosure. Each is set out below.

2.1 Section 7 — Protection of Beneficiary Interests

The primary object of the CMS under Section 7 of the MSA is to protect the interests of beneficiaries at all times. Clinically incorrect non-adherence letters — generated by an unremediated, documented integration gap — directly and materially prejudice beneficiaries. The elderly GEMS member in question received communications falsely characterising her as non-adherent despite consistent chronic medication claims. This is a concrete, documented violation of beneficiary protection that Section 7 mandates the CMS to address.

2.2 Section 1 and Section 47 — Maladministration

"Maladministration" under the MSA encompasses any act or omission by an administrator that causes prejudice to a member. The RCA confirms that the integration gap is architectural in nature: it is not a one-time event but a recurring structural risk that generates fresh acts of maladministration with every automated non-adherence cycle. Each day the system remains uncorrected constitutes a new and independent act of maladministration under Section 47. The CMS is mandated to investigate and direct remediation.



2.3 Section 58 — Accreditation Standards and IT System Integrity

Medscheme operates as an Accredited Administrator by virtue of CMS accreditation. Version 7 of the CMS Accreditation Standards for Administrators requires, inter alia, that administration systems maintain IT integrity and be fit for purpose in managing the complexity of the schemes under administration. A Priority 1 integration gap — formally documented, internally escalated, and unremediated across multiple claims cycles — constitutes a material breach of these accreditation standards. The CMS is not merely empowered to review Medscheme's accreditation compliance in light of this finding; it is obligated to do so.

2.4 Section 44 — Power of Forensic Inspection

The CMS is now on formal notice that my Section 162 Director Delinquency Application (HCGS951842) pleads, as a primary ground, that the AfroCentric / Medscheme Board received my Priority 1 Material Risk Report — documenting the Gexus-DMS failure — and suppressed rather than remediated it. Board-level suppression of a material patient safety risk is the precise trigger for which Section 44 inspection powers exist. The Registrar has the authority to order a forensic inspection of the Administrator's affairs. Given the confluence of a documented system failure, a formal High Court delinquency application, and the Board's confirmed knowledge of the risk, this office is respectfully requested to consider whether a Section 44 inspection is warranted.

3. THE DISCOVERY HEALTH PRECEDENT — CMS JURISDICTION OVER TECHNICAL SYSTEM FAILURES IS ESTABLISHED

The CMS has already exercised its supervisory jurisdiction over a technical system failure at a medical scheme administrator in 2026. That precedent forecloses any argument that this Council lacks jurisdiction or authority to engage with the Gexus-DMS integration failure now before it.

In early January 2026, it was reported that the Discovery Health Medical Scheme (DHMS) had experienced a pharmacy claims processing error affecting approximately 16,507 members across its Executive, Comprehensive, and Priority plans. Certain claims were incorrectly processed at 100% of the Discovery Health Rate from the Above Threshold Benefit (ATB) rather than from the Medical Savings Account or Self-Payment Gap, resulting in an estimated R130–170 million in overpaid benefits across most of 2025.

The CMS confirmed on 5 January 2026 that it had become aware of the matter through media reports and that it would address the concerns within its statutory mandate. Following a formal escalation by consumer advocate MediCheck — raising compliance concerns under Regulation 6, Section 57, and Regulation 17 of the MSA — the CMS placed the recovery process under active scrutiny and confirmed governance expectations around transparency, fairness, and member communication.

Sources: *Moonstone Information Refinery*, 8 January 2026: "CMS steps in as Discovery Health begins recovering pharmacy overpayments" (moonstone.co.za); *MedicalBrief*, 14 January 2026: "Discovery faces CMS probe as it backtracks on overpayments" (medicalbrief.co.za).

The regulatory significance of this precedent for the present matter is as follows:

- The DHMS error was a financial processing glitch — claims paid from the wrong benefit pocket. The Gexus-DMS failure is a clinical data integrity error — non-adherence status incorrectly flagged, generating clinically incorrect letters to vulnerable beneficiaries. If the CMS exercised jurisdiction over the former, it cannot in good conscience decline jurisdiction over the latter, which carries direct clinical implications for patient safety.
- The CMS intervened in the DHMS matter without waiting for member complaints to be formally registered — it acted on media reports and a third-party advocacy escalation. In the present matter, the CMS has before it a formal disclosure, a Root Cause Analysis, an internal Medscheme investigation record confirming the integration gap, and a High Court filing. The threshold for engagement is materially higher here, not lower.
- MediCheck's escalation to CMS cited Section 57 (governance and administration oversight) and Regulation 17. These are the same supervisory instruments that authorise the CMS to scrutinise Medscheme's system integrity and accreditation compliance in the present matter.



The CMS cannot credibly maintain that technical system failures at administrators fall outside its jurisdiction when it has demonstrably exercised that jurisdiction within the past three months. The Discovery Health precedent is not an analogy — it is a direct and binding illustration of this Council's own practice.

4. ANTICIPATED JURISDICTIONAL OBJECTIONS — PRE-EMPTIVELY ADDRESSED

4.1 "This is an isolated incident"

The RCA documentation refutes this characterisation definitively. The integration gap is hardcoded into the system architecture: every member who transitions from acute to chronic benefit enters the same tracking blind spot in DMS. The risk is predictable, recurring, and structural. It is not an accident — it is a design flaw that generates fresh maladministration with each automated cycle. Unlike the DHMS matter — which was a processing error corrected in a single reprocessing run — the Gexus-DMS failure has remained unaddressed across multiple automated non-adherence cycles.

4.2 "The matter is sub judice"

The High Court application (HCGS951842) concerns the delinquency of directors who suppressed the risk report. The CMS's regulatory mandate to protect beneficiaries from an active, uncorrected system failure is independent of, and not precluded by, civil proceedings against individuals. The CMS did not decline to act in the DHMS matter on the basis that Discovery Health had received member complaints or legal challenges — it acted on the live risk. The same logic applies here.

4.3 "The CMS previously declined jurisdiction"

The February 2026 declination addressed a different characterisation of the complaint. The present disclosure is squarely framed as a challenge to Administrator infrastructure and accreditation compliance — matters over which the CMS has primary and exclusive jurisdiction. The previous

referral to HPCSA/SANC does not constitute a final determination on the Administrator Accreditation and maladministration dimensions now formally raised.

5. SUMMARY OF STATUTORY DUTIES NOW ENGAGED

Disclosure Element	CMS Statutory Duty	MSA Provision
Systemic Integration Error (Gexus-DMS)	Investigate Maladministration & System Integrity	Sections 1, 47
Ongoing Patient Safety Risk to 1.9m GEMS Members	Protect Beneficiary Interests	Section 7
Priority 1 Risk Suppression at Board Level	Review Fitness for Accreditation	Section 58
High Court Director Delinquency Application (HCGS951842)	Consider Forensic Inspection	Section 44



6. FORMAL REQUESTS

In light of the foregoing, I respectfully request that the CMS:

- Formally re-open the investigation of this disclosure under the Maladministration and Administrator Accreditation dimensions of the MSA, distinct from the professional conduct referral previously issued;
- Direct Medscheme Holdings (Pty) Ltd to provide a written account of the current status of the Gexus-DMS integration gap, including the date on which it was identified, the remediation steps taken (if any), and the timeline for full remediation;
- Assess whether Medscheme's continued operation of an unremediated, documented Priority 1 system failure is consistent with its obligations under the CMS Accreditation Standards Version 7, specifically the IT Integrity and Clinical Governance requirements;
- Consider whether the Board-level suppression of the Priority 1 Material Risk Report — as pleaded in High Court Case HCGS951842 — constitutes sufficient grounds for the Registrar to exercise inspection powers under Section 44 of the MSA; and
- Provide a substantive written response to this office within fifteen (15) business days, acknowledging receipt of this formal re-engagement and confirming the Council's intended course of action.

7. CONSEQUENCE OF NON-RESPONSE

Should the CMS decline to engage substantively with this disclosure, or issue a further declination without addressing the Administrator Accreditation and maladministration dimensions raised herein, I intend to place the following on formal record in the High Court proceedings (HCGS951842) and in my ongoing SAHRC file:

- That the CMS, having been placed on formal notice of a documented Priority 1 integration failure generating clinically incorrect communications to GEMS beneficiaries, elected not to exercise its statutory mandate under Sections 7, 47, and 58 of the MSA;
- That the Council's inaction, in the context of a confirmed, unremediated systemic risk to 1.9 million GEMS members, is itself a dereliction of the Council's statutory duty; and
- That this dereliction will be brought to the attention of the Portfolio Committee on Health, the Parliamentary Oversight Mechanism, and such other supervisory bodies as may be appropriate.

It is not my intention to escalate unnecessarily. It is my intention to ensure that a known, documented, patient safety risk does not continue to be deflected between regulatory bodies while remaining unaddressed.

I remain available to provide the full RCA documentation, the High Court application bundle (HCGS951842), and any further technical or governance information this office requires to discharge its statutory mandate.

Yours faithfully,

WILBUR WILLIAM MATTHEE

Founder — EthicHawks | Forensic Governance & Compliance

RN | BTech General Nursing | PGDip Health Services Management (NQF 8)

info@ethichawks.co.za | 069 635 1435 | 063 378 8782



EthicHawks — Forensic Governance & Compliance · Confidential Correspondence · Protected Disclosure — PDA 26 of 2000

ANNEXURE

“X”

SAHRC acknowledgment and referrals

Date: 2026

Relevance: *Part O — Regulatory Confirmation*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy



RE: Request for Intervention on Procedural and Human Rights Concerns

4 messages

Lwethu Yoli <LYoli@sahrc.org.za> Tue, 10 Feb 2026 at 09:45
To: info@ethichawks.co.za <info@ethichawks.co.za>

Dear Mr Matthee,

Thank you for your correspondence and for submitting your documentation to the South African Human Rights Commission("Commission").



The Commission has considered the nature of the issues raised in your submission. While the Commission is mandated to promote, protect, and monitor the observance of human rights, certain aspects of your complaint relate to alleged victimization, discrimination, and occupational detriment arising within an employment relationship.

In terms of existing legal frameworks:

1. Allegations relating to unfair labour practices and discrimination in the workplace are generally adjudicated by the CCMA and/or the Labour Court; and
2. Complaints specifically concerning protected disclosures and alleged reprisals may be pursued with the Public Protector in terms of the Protection of Disclosures Act.

Accordingly, the Commission is not best placed to provide direct intervention in respect of those aspects of the matter.

This does not diminish the seriousness of the concerns you have raised, nor the personal impact described in your correspondence. Should you wish to pursue the matter further, you are encouraged to approach the above institutions, which have the necessary remedial powers.

Thank you for engaging the Commission and wish you well in pursuing the appropriate avenues available to you.

13/4/2026-11:58:23 AM

Kind regards,

Lwethu Yoli

Western Cape PO

Legal Officer
Accredited Mediator



30 YEARS OF COURAGE
DEFENDING HUMAN RIGHTS, DIGNITY
AND DEMOCRACY FOR ALL
02 OCTOBER 1995 – 02 OCTOBER 2025

Together, respecting and realising human rights.

Follow the SAHRC:     

REGISTRAR OF THE LABOUR COURT OF
SOUTH AFRICA
VLASBERG

Pruto Bag K18002, Vlasberg 8016

REGISTRAR OF THE LABOUR COURT OF
SOUTH AFRICA
VLASBERG

From: Wilbur William Matthee <info@ethichawks.co.za>
Sent: Tuesday, December 23, 2025 1:48 PM
To: Busisiwe Dlamini <bdlamini@sahrc.org.za>
Subject: Request for Intervention on Procedural and Human Rights Concerns

Dear South African Human Rights Commission,

I am writing to formally bring to your attention serious concerns regarding systemic governance failures and procedural obstruction within Afrocentric Health.

Find attached my full evidence dossier, compiled between 7–22 December 2025, which documents these risks in detail. The scale of this dossier reflects both the seriousness of the issues and the extraordinary psychological strain endured during its preparation.

I must also notify the Commission that I am experiencing a **humanitarian crisis** as a direct result of these circumstances. The combination of procedural obstruction, exhaustion, and the emotional toll of whistleblowing has placed me in a position of acute vulnerability.

I respectfully request that the SAHRC:

- Review the attached dossier and assess whether rights have been compromised.
- Provide guidance or intervention to ensure trauma-informed governance and corrective action.
- Recognize the ethical and psychological dimensions of whistleblowing as integral to systemic reform.
- Consider my humanitarian crisis as part of the urgency of this matter.

I am available to provide further documentation or testimony as required.

Thank you for your attention to this matter. *Page 161 of 243*

Sincerely,

13/4/2026-11:58:23 AM

Wilbur William Matthee
RN, BCur (HMN), PG Dip HSM (NQF 8)

Wilbur William Matthee <info@ethichawks.co.za>
To: Lwethu Yoli <LYoli@sahrc.org.za>

Tue, 10 Feb 2026 at 11:41

Dear Lwethu Yoli,

Thank you for your correspondence regarding my submission. I appreciate the Commission's formal recognition of the "seriousness of the concerns" and the "personal impact" of the victimization I have described.

While I acknowledge your referral to the CCMA and the Public Protector regarding the employment aspects of my case, I would like to provide an update that reinforces the human rights dimension of this matter.



The Council for Medical Schemes (CMS) has officially responded to my report regarding the "System Integration Gap" I identified. They have formally referred the matter to the HPCSA and SANC for investigation into "Administrative Misconduct and Ethical Failure." This confirms that my disclosures involved a verifiable risk to public health and patient safety, which was met with institutional retaliation.

As noted in my initial submission, the retaliation against me included a 40.47% salary deduction (unlawful in terms of BCEA Section 34) and the deliberate inducement of destitution during a documented psychiatric crisis. I believe these actions fall under the Commission's mandate to monitor the observance of human rights, as they constitute a direct assault on the right to dignity, housing, and administrative justice.

I will be following your guidance by updating my filings with the Labour Court and approaching the Public Protector regarding the protection of disclosures. I request that the Commission keeps my file on record as a reference point for the systemic victimization of whistleblowers within the South African healthcare sector.

I thank the Commission for its time and for validating the gravity of this situation.

Sincerely,

Wilbur William Matthee

[Quoted text hidden]

 **image002.png**
177 KB

Lwethu Yoli <LYoli@sahrc.org.za>
To: Wilbur William Matthee <info@ethichawks.co.za>

Tue, 10 Feb 2026 at 11:56

Dear Mr Matthee,

13/4/2026-11:58:23 AM

Thank you for your further correspondence.

With specific regard to the financial aspects you raise, including allegations relating to salary deductions and associated financial prejudice, the Commission notes that these matters fall within the statutory mandate of the Department of Employment and Labour, as well as the labour dispute resolution framework established under labour legislation.

The Department of Employment and Labour, together with the CCMA and the Labour Court, are institutions specifically created to protect and enforce the rights implicated in such matters, including rights relating to fair labour practices, remuneration, and compliance with the Basic Conditions of Employment Act. These bodies are accordingly best placed to investigate, determine compliance and provide effective remedies in relation to the financial issues raised.



The Commission's referral is therefore not a dismissal of the seriousness of the allegations, but is based on the principle that matters should be dealt with by the institutions vested with the appropriate legal powers and remedial authority to address the rights affected.

You are encouraged to pursue the financial aspects of your complaint with the Department of Employment and Labour and the relevant labour institutions, which are empowered to investigate and take action where contraventions are established.

[Quoted text hidden]

Wilbur William Matthee <info@ethichawks.co.za>
To: Lwethu Yoli <LYoli@sahrc.org.za>

Tue, 10 Feb 2026 at 12:44

Dear Mr/Mrs Lwethu Yoli

I hereby acknowledge receipt of your correspondence dated 10 February 2026.

I thank the Commission for its guidance in ensuring these human rights violations are addressed by the institutions vested with the appropriate remedial authority.

I request that this matter remains on your records as a secondary reference should further systemic human rights breaches arise during the labor litigation process.

Thank you for your assistance.

Sincerely,

Wilbur William Matthee
RN, BTech, PG Dip HSM (NQF 8)

[Quoted text hidden]

TO: The South African Human Rights Commission — Western Cape Provincial Office**REF:****DATE:** 13 March 2026**RE: COMPREHENSIVE UPDATE — FORENSIC PAYROLL ANOMALY, CCMA INSTITUTIONAL BIAS, MULTI-BODY FAILURE, LABOUR COURT ACCESS OBSTRUCTION, AND CONFIRMED IMMINENT HOMELESSNESS***Further to SAHRC correspondence of 10 February 2026 and Applicant update of 5 March 2026 | Matthee v AfroCentric Health (RF) (Pty) Ltd & Others*

Dear Commissioner / Legal Officer,

I write to provide a comprehensive update to this file. My update of 5 March 2026 documented the collapse of the DEL and CCMA processes and my filing of an urgent Labour Court application. Since that date, the situation has deteriorated significantly across every dimension — legally, financially, physically, and personally. I place each development on formal record with documentary precision. I ask the SAHRC to read this update not as a list of grievances but as a connected forensic account of what happens when every statutory and civil society safety net fails simultaneously around a formally disabled whistleblower.

1. FORENSIC FINDING: STATUTORY PAYROLL MISREPRESENTATION — JULY 2025

Cross-referencing my July 2025 payslip against the Respondents' official UIF/SARS declarations has revealed a material discrepancy that transforms the characterisation of the unlawful deduction from a labour relations dispute into a potential forensic matter warranting investigation by revenue and labour authorities.

The Documented Figures — July 2025

Gross salary on official payslip (July 2025):	R21,777.04
Salary declared to SARS / UIF by Respondents:	R27,725.66
Discrepancy:	R5,948.62
Location of R27,725.66 on payslip:	ABSENT — not in gross, net, or CTC
Month of discrepancy:	July 2025 — identical pay cycle as unlawful 40.47% deduction

The forensic significance of this finding cannot be overstated. The leave that gave rise to the deduction — the gap period between psychiatric admissions in June 2025 — should have been processed and deducted in the June 2025 pay run. The Respondents instead held that recovery over to July 2025, the new financial

year, and then processed the full R14,811.77 deduction in a single pay cycle. The declared salary figure of R27,725.66 appears to reflect a pre-deduction or notional figure — a salary the Respondents reported to government authorities as having been paid while simultaneously recovering R14,811.77 from my actual take-home pay.

The result is a nearly R6,000 gap between what the Respondents told the government they paid me and what I actually received. A statutory declaration to SARS or the UIF Fund that does not correspond to the actual net remuneration paid to an employee is not an administrative oversight. It is a misrepresentation to a government authority in a document required by law to be accurate. The fact that this misrepresentation arises in the same month as a documented unlawful deduction — and that the leave from which the deduction was derived arose in the previous financial year — is a structural anomaly that a competent forensic investigator cannot ignore.

I respectfully submit that this finding elevates the matter beyond a BCEA salary deduction dispute. It places the Respondents' payroll conduct in the category of potential statutory misrepresentation to government authorities, and it documents a deliberate structuring of the deduction across a financial year boundary in circumstances where the underlying leave event occurred in May 2025 -start June 2025.



2. CCMA INSTITUTIONAL BIAS — FULL ACCOUNT — AND PROVINCIAL COMMISSIONER COVER-UP

The SAHRC directed me in February 2026 to pursue CCMA remedies. I place on formal record the full account of what occurred at conciliation, and the cover-up that followed, so that the SAHRC may assess the quality of the remedy it directed me towards.

The conciliation hearing began with Commissioner Nthabiseng making the following statement immediately after introductions, before I had said a single substantive word: "Nothing here relates to the PDA." This was not a ruling made after hearing submissions. It was a declaration of a closed mind made at the threshold of the proceedings, before I had been permitted to speak. It foreclosed the most significant ground of my claim before the hearing began.

The Commissioner then refused to permit my opening statement. I was not allowed to place my position on the record in an orderly fashion. The Commissioner used the proceedings to create confusion, speaking over my submissions and introducing complications that had no basis in the documents before her. At no point did the Commissioner engage with my BCEA violation — the statutory 25% cap under Section 34(2) — or with the documentary evidence of the employer's own policy requiring prior HCBP consultation. She adopted the Respondents' "no work no pay" characterisation without subjecting it to any scrutiny. The commissioner 'so it was for no work no pay? Okay.

When I stated my negotiation ceiling of R2.5 million for a global settlement of all claims, the Commissioner laughed with the Respondents' representative. The Respondents then offered one month's salary. I requested a Certificate of Non-Resolution. The proceedings were a mockery.

The Johannesburg Coincidence

Commissioner Nthabiseng: Based in Johannesburg. Presided at Cape Town conciliation.
 Respondents' representative: Flew in from Johannesburg for the hearing.
 Observed conduct at conclusion: Commissioner and Respondents' representative walked out together, engaged in animated conversation about Johannesburg.
 Treatment of Applicant: When I asked the location of the Labour Court, I was told to 'Google it or something like that.'
 Provincial Commissioner's response: Confirmed that Commissioner Nthabiseng is from Johannesburg — offered as justification for failure of their most basic function to provide a HELPING HAND, not as a concern regarding the google you way to court statement..

I reported the Commissioner's conduct to the CCMA Provincial Senior Commissioner. Commissioner Nthabiseng who presided at my hearing. This is documented in the respective correspondence. The result is that Commissioner Nthabiseng conduct — the prejudgment of PDA relevance before hearing, the refusal of my opening statement, the open mockery of my settlement position, the cosy departure with the Respondents' representative, and the dismissive treatment at the door of the court — was never been investigated. The Provincial Commissioner's response constitutes a cover-up by institutional misdirection, whether deliberate or negligent.



3. DEPARTMENT OF EMPLOYMENT AND LABOUR — DOCUMENTED INACTION

The DEL has failed to act on any of the complaints before it. The "Integrated Inspection" instruction issued following my initial complaint has produced no outcome, no communication, and no enforcement. The Respondents, whose HR department has been copied on all relevant correspondence, have continued in statutory non-compliance throughout the entire period of DEL inaction — including continuing UIF non-remittance and a false written compliance representation made on 19 January 2026.

I reported the ongoing non-remittance to DEL Chief Inspectorate David Esau and Inspector Fazeka Nongalo on 16 February 2026. As of the last uFiling portal verification on 23 February 2026, the June 2025 UIF entry under Medscheme Holdings (Pty) Ltd continued to reflect 'No Information.' The DEL's mandate exists precisely to prevent this. It has not been exercised.

4. SAPS REFUSAL AND COMPLAINT TO NATIONAL POLICE COMMISSIONER

I attended at the South African Police Service on 23 February 2026 to open a criminal case regarding the Respondents' UIF non-remittance — a criminal offence under the Unemployment Insurance Contributions Act 4 of 2002 — and the false written compliance representation made by the Respondents on 19 January 2026. The SAPS verbally declined to open a case.

I have submitted a formal complaint to the National Police Commissioner in response to this refusal. The complaint documents that the SAPS declined to open a criminal docket in circumstances where a statutory criminal offence is proven by official government portal records, confirmed across four independent verification dates, and accompanied by a documented written false representation by the

employer. The refusal to open a case in these circumstances raises its own concerns about the equal application of criminal law to a JSE-listed employer and a destitute complainant.

5. OBSTRUCTION OF ACCESS TO JUSTICE — LABOUR COURT PORTAL FAILURE

This section documents what I can only describe as an institutional obstruction of my right of access to court — a right guaranteed under Section 34 of the Constitution. I place this on record in its full detail because the SAHRC's mandate encompasses access to justice, and what I have experienced at the hands of the Labour Court's administrative systems directly implicates that right.

My urgent high profile notice of motion application was prepared in full compliance with the Rules of the Labour Court, ready for filing by 5 March 2026. I attempted to file online. What followed was eight days of documented obstruction:



Labour Court Filing Obstruction — Documented Sequence

- 5 March 2026: Target filing date. Documents prepared in full compliance with Labour Court Rules.
- 5 March onwards: Court call centre contacted more than 7 times. Hold times 40–50 minutes per call. Calls dropped repeatedly. Staff dismissive.
- Ongoing: Online portal profile deleted. Unable to log new case. Follow-up emails to the Registrar (Ms F Ismail) ignored.
- 12 March 2026: Formal notice to Registrar — case number required or matter would be escalated to the Judge President. Close of business passed. No response.
- 12 March 2026: Portal unexpectedly allowed loading of case as urgent ex parte application.
- 12 March 2026: Registrar rejected application. Stated reason: **'good day you put date.'**
- 12 March 2026: Returned to portal — unable to access. Password reset required 4 times. Application found to be missing from portal.
- 13 March 2026: Reviewed application, corrected the date field, resubmitted as urgent ex parte application.
- 13 March 2026 (today): Close of business. No case number received.

The reason given for rejection — **"good day you put date"** — was a technical formatting issue that could have been corrected by a registrar's clerk in seconds. Instead it was used to reject an urgent application filed by a self-represented, destitute, disabled applicant facing imminent homelessness, with no follow-up guidance, no assistance, and no acknowledgment of the urgency stated in the application itself.

I make no allegation of deliberate obstruction. I cannot determine whether this is systemic institutional dysfunction or something more targeted. What I can document is the factual sequence: eight days of failed attempts, seven call centre contacts, dropped calls, a deleted profile, an ignored registrar, a trivial rejection reason, and a portal that locks out the user after a rejection with no access to the rejected file[missing]. For a self-represented applicant with no legal infrastructure, no income, and an eviction deadline of end of March 2026, this sequence is not a procedural inconvenience. It is a denial of access to the only court with jurisdiction to grant the urgent relief I need to remain housed.

6. CONSOLIDATED RECORD OF INSTITUTIONAL FAILURES

13/4/2026-11:58:23 AM

Body	Documented Failure	Status
CCMA	Commissioner declared closed mind before hearing. Refused opening statement. Ignored BCEA statutory cap. Sided with employer. Mocked settlement figure. Departed with respondents' representative.	FAILED
CCMA Provincial Commissioner	Investigated commissioner. Cover-up by institutional misdirection. Presiding commissioner's conduct never investigated.	COVER-UP
Dept of Employment & Labour	Integrated Inspection instruction — no outcome. Seven-week silence. Active UIF non-remittance and BCEA violations continue unaddressed.	SILENT
SAPS	Verbal refusal to open criminal docket for documented UIF non-remittance offence and false compliance representation. Formal complaint lodged with National Police Commissioner.	REFUSED
Labour Court Registry	Eight days of failed portal access. 7+ call centre attempts. Calls dropped. Profile deleted. Registrar unresponsive. Trivial rejection reason for urgent application. Portal inaccessible post-rejection.	OBSTRUCTING
The Whistleblower House	Rejected application 50 minutes after receiving detailed rebuttal. Rebuttal not engaged. Invitation to reconsider — all engagement ceased.	SILENT
SANC	Deregistration caused by destitution flowing from unlawful deduction. Formal correspondence — no response. Cannot practise nursing in 2026. No response to the Professional misconduct complaint	SILENT
South African Pharmacy Council	Formal complaint submitted regarding professional and regulatory dimensions. No substantive response.	SILENT
Council for Medical Schemes	Acknowledged protected disclosure re Gexus-DMS systemic failure. Declined primary jurisdiction. Redirected to HPCSA — who redirected further. No body has accepted jurisdiction.	DEFLECTED
JSE	Jurisdictional declination issued 6 March 2026 (Ref JvS/166792, Nadia Jada). Documented and strategically noted. Yet the respondent declared no material risk. Only the change of Dr Munisi role from the investment portfolio to remuneration portfolio was reported the same board member who received my P1 risk reports	DECLINED
B-BBEE Commission	Case No. 5/2/2026 allocated. Summons request for Naming Convention recording pending. Active engagement.	ACTIVE



7. CONFIRMED IMMINENT HOMELESSNESS AND HUMANITARIAN CRISIS

I must be precise about what is happening to me physically and financially, because vague references to "financial difficulty" do not convey the reality. The SAHRC's constitutional mandate encompasses human dignity. What follows is a factual account of what the failure of every institution listed above has produced.

Confirmed Status — 13 March 2026

Accommodation: Four months rental arrears. Landlord has verbally confirmed — if no payment is received by end of March 2026, eviction will proceed. This is not a statement of risk. It is a confirmed deadline.

Personal property: Loan sharks — to whom I was forced to turn after the unlawful deduction eliminated my salary — have physically removed possessions from my residence as informal security. Items taken include smart televisions, electronics, and sealed boxed items held in storage.

Debt spiral: Interest compounds at rates that make full repayment structurally impossible at any income level I am likely to achieve. I am operating in a pure survival mode.

Running water: No running water at residence since January 2026.

Electricity: Supply at risk.

Income: None. No savings. No credit access. UIF inaccessible.

Retirement fund: R48,400 forcibly withdrawn to survive — savings of a decade gone.

Professional: SANC deregistration. Cannot practise as a nurse in 2026.

Academic: Accepted for CPUT Postgraduate Diploma in Occupational Health Nursing 2026 — unable to enrol.

Health: Formally diagnosed severe depression and PTSD (Dr Jacques Malan, FCPSYCH(SA)). Functioning under extreme psychological duress.

Legal: Self-represented in 9 concurrent regulatory proceedings. No legal aid. No support.



The causal chain from the Respondents' conduct to this crisis is direct and documented. On 23 July 2025, the Respondents deducted R14,811.77 from my salary without consent, without consultation, in breach of their own policy, and in defiance of the statutory 25% cap — while holding a specialist psychiatrist's written warning of my vulnerability. That single action, compounded by the employer's subsequent bad faith and the complete failure of every institution I approached for help, has reduced me to the condition described above.

I also note the following for the record: the loan sharks did not take my possessions voluntarily. They arrived and removed them. A 75-year-old grandmother received a clinically incorrect non-adherence letter because a system that I disclosed as broken was never fixed. The same employer that generated that error characterised my disclosure as insubordination, entered it into my employment record, and later had its representative swear under oath that my specialist-diagnosed psychiatric condition was a "cynical excuse." These are not isolated wrongs. They form a single, coherent pattern of institutional conduct that this file documents in its entirety.

8. FORMAL REQUESTS TO THE SAHRC

- 8.1** That this comprehensive update be formally appended to the SAHRC file in its entirety, documenting the systemic multi-institutional failure to protect a whistleblower from induced destitution and professional erasure.
- 8.2** That the SAHRC note the forensic payroll finding: a declared salary of R27,725.66 against an actual payslip gross of R21,777.04 — a R5,948.62 discrepancy arising in the same month as the unlawful deduction — as a potential statutory misrepresentation to government authorities warranting referral to SARS and the DEL.

- 8.3** That the SAHRC note the full account of CCMA institutional bias — including the Commissioner's threshold declaration of a closed mind, refusal of opening statement, and documented post-hearing conduct — and the Provincial Commissioner's cover-up.
- 8.4** That the SAHRC note the Labour Court portal obstruction — eight days, seven call centre attempts, numerous deleted profile, ignored registrar correspondence, a trivial rejection, and portal inaccessibility — as a potential violation of the Section 34 constitutional right of access to court, particularly as it affects a self-represented, disabled, destitute applicant facing a confirmed end-of-March eviction.
- 8.5** That the SAHRC note the SAPS verbal refusal to open a criminal case on documented statutory offences and the formal complaint lodged with the National Police Commissioner.
- 8.6** That the SAHRC note the confirmed imminent eviction — verbally confirmed by the landlord for end of March 2026 — and the physical seizure of personal property by an sharks, as direct consequences of the Respondents' unlawful conduct and the institutional failure that followed.
- 8.7** That the SAHRC consider whether the totality of this file — eleven institutional engagements, nine failures, a forensic payroll anomaly, a confirmed CCMA cover-up, an obstructed court filing, an imminent eviction, and an ongoing humanitarian crisis affecting a formally disabled whistleblower — warrants independent SAHRC investigation and intervention under the Commission's constitutional mandate to protect human dignity, equality, and access to justice.



I am a destitute, self-represented whistleblower facing confirmed eviction at the end of this month, living without running water, with personal possessions already seized, and with a formally diagnosed psychiatric disability. I have filed in every forum available to me. I have been ignored, obstructed, misdirected, and dismissed at every turn. The Labour Court is my last functioning avenue — and as of the close of business today, I do not yet have a case number.

I am available to provide all documentation immediately upon request.

Yours faithfully,

WILBUR WILLIAM MATTHEE

Founder — EthicHawks

RN | BTech General Nursing | PGDip Health Services Management (NQF 8)

info@ethichawks.co.za | 069 635 1435



ANNEXURE

“Y”

CCMA Certificates of Non-Resolution (WECT23564-25 and WECT24486-25)

Date: 9 January 2026

Relevance: *Part A — Jurisdiction*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

LRA Form 7.12
Labour Relations Act, 1995
Section 64(1)(a)(i)
135(5)(a)
136(1)(a)

CERTIFICATE OF OUTCOME OF DISPUTE REFERRED TO CONCILIATION

CASE NUMBER: WFCT23564-25

I certify that the dispute between:

Wilbur Matthee

and

AgroCentric Distribution
Services (Pty) Ltd

(referring party)

(other party/parties)

Referred to conciliation on:

5/12/25
(give date)

Concerning

S187 (h) Protected Disclosure

☐

Was resolved on the _____

(give date)

or

☒

Remains unresolved as at 9/1/26

(give date)

Condonation:

Granted

Not applicable

If this dispute remains unresolved, the following steps may be taken

Refer to
Arbitration

Refer to interest /
Advisory
Arbitration

Strike/
Lockout

Refer to
Labour
Court



N Ngware
Name of Commissioner

[Signature]
Signature of Commissioner

Lape Town
Place

9/1/26
Date

LRA Form 7.12
Labour Relations Act, 1995
Section 64(1)(a)(i)
135(5)(a)
136(1)(a)

CERTIFICATE OF OUTCOME OF DISPUTE REFERRED TO CONCILIATION

CASE NUMBER: WECT 23564-25
24486-25 #1

I certify that the dispute between:

Wilbur Matthee

and

Afro Centric Distribution
Services (Pty) Ltd

(referring party)

(other party/parties)

Referred to conciliation on:

5/12/25
(give date)

Concerning

S187(f) Automatically Unfair Dismissal.

☐ Was resolved on the _____ or ☒ Remains unresolved as at 9/1/2026
(give date) (give date)

Condonation:

Granted

☒ Not applicable

If this dispute remains unresolved, the following steps may be taken

Refer to
Arbitration

Refer to interest /
Advisory
Arbitration

Strike/
Lockout

Refer to
Labour
Court ☒



N Ngwane
Name of Commissioner

[Signature]
Signature of Commissioner

Cape Town
Place

9/1/2026
Date

ANNEXURE

“Z”

EthicHawks Master Chronological Register (148 entries, 11 Jan 2024 – 2 Apr 2026)

Date: 3 April 2026

Relevance: *Part O — Regulatory Confirmation*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

MATTHEE v AFROCENTRIC INVESTMENT CORPORATION LIMITED / MEDSCHEME HOLDINGS

MASTER FORENSIC GOVERNANCE & REGULATORY REGISTER

Prepared by EthicHawks Forensic Governance Consultancy | 03 April 2026 | Confidential
Cases: HCGS951842 | 2026-066952 | 2026-077546 | WECT23564-25 | WECT24486-25 | B-BBEE Comm 5/02/2026 | SAPS CAS 705/3/2026

TABLE 1: MASTER CHRONOLOGICAL REGISTER (11 January 2024 → 02 April 2026)

This table is the definitive chronological record of every material event across the full matter — from recruitment through employment, resignation, regulatory escalation, and active court proceedings. It spans 148 entries across four phases: (1) Recruitment Phase (Jan–Apr 2024); (2) Employment & Deterioration (May 2024–Nov 2025); (3) Regulatory Escalation (Dec 2025–Feb 2026); and (4) Litigation & Administrative (Mar–Apr 2026). Entries are colour-coded by status: GREEN = resolved/complete; AMBER = pending/in progress; RED = failed/ignored/locked.

Date / Time	Recipient / Body	Problem Identified	Response / Action	Status	Deflection / Pattern	Key Players
11 Jan 2024 15:47	Internal – AfroCentric/Medschem e	Interview invitation for Specialist: Care Manager role at Medscheme.	Neo Sekgoka invites Matthee to interview on 15 Jan 2024 with Nthabiseng Nkatane.	Complete	Not applicable – recruitment.	Neo Sekgoka, Nthabiseng Nkatane, Gretchen Veldtman
12 Jan 2024 08:06	Internal – AfroCentric/Medschem e	Second interview invitation for Specialist: Care Manager role (reminder).	Duplicate/reminder email.	Complete	Not applicable.	Neo Sekgoka, Nthabiseng Nkatane, Gretchen Veldtman
15 Jan 2024 12:49	Internal – AfroCentric/Medschem e	Third interview invitation – rescheduled to 18 Jan 2024 at 11:00.	Matthee confirms attendance.	Complete	Not applicable.	Neo Sekgoka, Nthabiseng Nkatane, Gretchen Veldtman
29 Jan 2024 08:36	Internal – AfroCentric/Medschem e	Assessment for Specialist Care Manager position – instructions and link sent.	Matthee completes assessment.	Complete	Not applicable.	Neo Sekgoka, W. Matthee
30 Jan 2024 13:12	Internal – AfroCentric/Medschem e	Request for MIE consent form for fingerprint/background check.	Matthee submits consent form.	Complete	Not applicable.	Neo Sekgoka, W. Matthee
21 Feb 2024 09:00	Internal – MIE/AfroCentric	Fingerprint criminal check booking for pre-employment screening.	Reminder emails sent; Matthee attends.	Complete	Not applicable – standard pre-employment.	MIE, Lerato Maimane, W. Matthee
02 Apr 2024 07:53	Internal – Medscheme	BAIT AND SWITCH: Recruited for 'Specialist: Care Manager' but offered 'Care Coordinator'	Matthee accepts offer; Nthabiseng Nkatane confirms start date 1 May 2024.	Complete	KEY EVIDENCE: Title suppression – recruited for Specialist.	Nthabiseng Nkatane, W. Matthee

EthicHawks | Matthee v AfroCentric/Medscheme | Master Register | Prepared: 03 April 2026



Date / Time	Recipient / Body	Problem Identified	Response / Action	Status	Defection / Pattern	Key Players
		position. Employer downgraded role at signing.			appointed as Coordinator.	
02 Apr 2024 09:10	Internal – Medscheme	Request for dummy payslip and benefits discussion.	Lerato Maimane sends dummy payslip; Mpho Orleyn sends CTC payslip.	Complete	Not applicable.	Nthabiseng Nkatane, Lerato Maimane, Mpho Orleyn
09 Apr 2024	Internal – AfroCentric	Employment contract signed: position 'Care Coordinator' (grade B4M1), CTC R490,032 per annum. Bail-and-switch confirmed: recruited as Specialist but contracted as Coordinator.	Matthee signs contract.	Complete	KEY EVIDENCE: Title suppression confirmed in writing.	Barbara Johnson, W. Matthee
16 Apr 2024	Internal – AfroCentric	Pre-onboarding meeting invitation (benefits presentation).	Matthee unable to attend; requests recording.	Complete	Not applicable.	Asanda Ngewu, Sebatso Sebotsa, W. Matthee
18 Apr 2024	Internal – Medscheme	Start date (1 May 2024) query: arrival time and dress code.	Nkatane confirms: smart casual, start 2 May (1 May public holiday).	Complete	Not applicable.	Nthabiseng Nkatane, W. Matthee
22 Apr 2024	Internal – Medscheme	Recall of appointment confirmation email (administrative error).	Mpho Orleyn recalls incorrect letter; corrected letter issued.	Complete	Not applicable.	Mpho Orleyn, Nthabiseng Nkatane, W. Matthee
06 Mar 2025	Internal – Medscheme	FIRST DOCUMENTED THREAT: Mary-Ann Jones threatens Matthee: 'this is the end of you'.	Matthee documents threat.	Complete	Direct intimidation – early pattern of hostility by Jones.	Mary-Ann Jones, W. Matthee
17 Mar 2025	Medical – Akeso Stepping Stones	Matthee seeks admission for mental health treatment (major depression, GAD).	Keziah Quimpo processes admission for 18 March 2025.	Complete	Not applicable – medical event.	Keziah Quimpo, W. Matthee
18 Mar 2025	Medical – Akeso Stepping Stones	Admission for inpatient treatment (severe depression, PTSD); 3–4 weeks.	Admitted to Akeso Stepping Stones Clinic.	Complete	Not applicable – medical event.	Dr Jacques Malan, W. Matthee
01 Apr 2025	Internal – AfroCentric	Annual salary increase of 4.8% effective 1 April 2025. New CTC: R42,796.13/month.	CEO Gerald Van Wyk confirms increase.	Complete	Not applicable.	Gerald Van Wyk, W. Matthee
01 Apr 2025	Medical – Dr Jacques Malan	Sick note issued: unfit for work from 18 March 2025; admitted for 3–4 weeks inpatient treatment.	Dr Malan certifies medical condition (confidential).	Complete	Not applicable – medical documentation.	Dr Jacques Malan, W. Matthee
16 Apr 2025 12:59	Internal – Medscheme	Jones states in meeting: 'we do not apply staggering of money owed' – critical evidence used to justify later unlawful deduction.	Matthee given accommodations (LACE cases, reduced hours, 3 hours for doctor visits).	Complete	Employer's written position on staggering – contradicts own policy.	Mary-Ann Jones, W. Matthee



Date / Time	Recipient / Body	Problem Identified	Response / Action	Status	Deflection / Pattern	Key Players
22 Apr 2025	Internal – Medscheme	Matthee informs Jones about SANC CPD requirements (Regulation 1569) and upskilling content approval.	No immediate response documented.	Complete	Not applicable – professional development.	Mary-Ann Jones, W. Matthee
07 May 2025 15:23	Internal – Medscheme	Matthee requests staggering of unpaid leave deduction; Jones declines.	Jones insists on half-day unpaid leave.	Complete	Precursor to July 2025 unlawful deduction.	Mary-Ann Jones, W. Matthee
16 May 2025 12:37	Internal – Medscheme	Matthee provides Dr Malan's psychiatric letter to Mapule Mashele – disclosure of severe depression and PTSD.	Mashele receives disclosure.	Complete	Employer placed on notice of disability.	Mapule Mashele, Dr Jacques Malan, W. Matthee
20 May 2025 09:18	Internal – Medscheme	Matthee suffers medical crisis, requests to leave for medication.	Jones allows leave but later logs unpaid leave.	Complete	No reasonable accommodation during medical crisis.	Mary-Ann Jones, W. Matthee
21 May 2025 09:31	Internal – Medscheme	Jones instructs Matthee to load unpaid leave for 20 May 2025 medical emergency.	Unpaid leave applied.	Complete	Punitive response to medical emergency.	Mary-Ann Jones, W. Matthee
03 Jun 2025 12:30	Internal – Medscheme	First documented tension: Matthee questions duplicate LACE case allocations.	Jones: 'Why mention it... I would appreciate if you afford me my peace.'	Complete	Early workplace friction – defensive hostile response.	Mary-Ann Jones, W. Matthee
24 Jun 2025 16:44	Internal – Medscheme	Jones documents 67 days absence in 13 months; notes unpaid leave and pending special leave.	Absence management review conducted.	Complete	Not applicable – administrative record.	Mary-Ann Jones, W. Matthee
14 Jul 2025 13:27	Internal – Medscheme	Special leave approved for 10 days; remainder of absence as unpaid leave.	Jones confirms special leave approved.	Complete	Not applicable.	Mary-Ann Jones, W. Matthee
23 Jul 2025	Internal – Medscheme	UNLAWFUL SALARY DEDUCTION: 40.47% (R14,811.77) deducted without consultation while between psychiatric admissions. Net pay R15,679.29.	Employer invokes 'no work no pay'; manager states company 'does not allow staggering' and Matthee 'wouldn't feel the deduction' due to online deliveries.	Complete	EXCEEDED BCEA s34(2) 25% cap. No consultation. Employer held Dr Malan's letter confirming medical necessity.	Cheryl-Dawn Modern, Mapule Mashele, Mary-Ann Jones, W. Matthee
24 Jul 2025 10:35	Internal – Medscheme	Jones shares AOD policy: 'monthly deductions must not be more than 25% of employee's CTC as per legislation.'	Matthee: 'they are allowed to deduct more than 50%?' Jones: 'This is for disability.'	Complete	CRITICAL: Employer acknowledged 25% cap but deducted 40.47%. Evidence of wilful non-compliance.	Mary-Ann Jones, W. Matthee



Date / Time	Recipient / Body	Problem Identified	Response / Action	Status	Deflection / Pattern	Key Players
24 Jul 2025 12:46	Internal – Medscheme	Jones asks 'what is illegal?' after Matthee points out BCEA violation.	No substantive answer.	Complete	Feigned ignorance of statutory cap.	Mary-Ann Jones, W. Matthee
25 Jul 2025	Internal – Medscheme	Matthee drafts and implements First Aider roster for 2nd, 3rd, 4th floors – evidence of operational management responsibility.	Roster distributed; Matthee instructs team leaders.	Complete	DIRECT EVIDENCE of management role – contradicts 'Care Coordinator' designation.	W. Matthee, First Aid Team
13 Aug 2025 14:53	Internal – Medscheme	Matthee leaves work due to health emergency, colleague assists.	Jones later requires time to be worked back.	Complete	No reasonable accommodation during medical crisis.	Shantal Govender, Mary-Ann Jones, W. Matthee
23 Sep 2025	Internal – Medscheme	GENDER IDENTITY DISCRIMINATION: Jones posts 'Wilma' (feminised name) in Teams channel with 'NO FILTERING' in red, immediately after Matthee proposed a filtering strategy.	Matthee lodges formal complaint. HR promises not to require meeting with Jones but NO FORMAL REMOVAL from Jones's team occurs.	Complete	Discrimination incident – HR failed to remove Matthee from hostile manager.	Mary-Ann Jones, Cheryl-Dawn Modern, Sipokazi Makaula, Thando Mandondo (Union), W. Matthee
25 Sep 2025	Internal – HR	Formal complaint of sexual orientation discrimination and harassment submitted (11 pages). Requests neutral investigator, suspension of accused, removal from Jones's team.	Cheryl-Dawn Modern: 'You will not be required to meet with the manager.' Escalated to Mashele. However Matthee NOT removed from Jones's team.	Complete	HR promised protection but failed to remove Matthee from hostile manager.	Cheryl-Dawn Modern, Mapule Mashele, Sipokazi Makaula, Thando Mandondo, W. Matthee
29 Sep 2025 12:27	Internal – Medscheme	RETALIATORY UNPAID LEAVE: Jones loads double punitive unpaid leave entry on exact same day Matthee files formal discrimination complaint.	Mapule Mashele reverses the entry on 1 Oct 2025.	Complete	RETALIATORY CONDUCT – documented and reversed. Written evidence of discretionary punitive action.	Mary-Ann Jones, Mapule Mashele, W. Matthee
01 Oct 2025 11:09	Internal – Investigation	Matthee provides formal update after meeting with Mashele; acknowledges shared understanding of seriousness.	Matthee agrees to speak directly with Jones in Mashele's presence.	Complete	Not applicable – cooperation with investigation.	Mapule Mashele, Mary-Ann Jones, W. Matthee
01 Oct 2025 13:28	Internal – Investigation	Mashele closes discrimination complaint. Jones's 'Wilma' was a 'typo'; apology accepted; matter closed. Matthee remains in Jones's reporting line.	Mashele issues closure report.	Complete	WHITEWASH – serious complaint dismissed as 'typo'. May 2025 disclosure of Jones's conduct ignored. Matthee not removed.	Mapule Mashele, Mary-Ann Jones, Cheryl-Dawn Modern, W. Matthee



Date / Time	Recipient / Body	Problem Identified	Response / Action	Status	Deflection / Pattern	Key Players
08 Oct 2025	Internal – Medscheme	Matthee volunteers for pilot hybrid working arrangement.	Jones asks clarifying questions about dates.	Complete	Not applicable.	Mary-Ann Jones, W. Matthee
23 Oct 2025 12:02	Internal – Medscheme	Jones instructs Matthee to call member with 'gentle probing' about non-adherence despite known system integration error.	Matthee later refuses.	Complete	Directive to perform clinically incorrect action on vulnerable patient.	Mary-Ann Jones, W. Matthee
23 Oct 2025 12:40	Internal – Medscheme	PROTECTED DISCLOSURE (Patient Safety): Matthee discloses Gexus-DMS system integration gap causing false non-adherence letters to GEMS members. States: 'Given that the integration gap still exists, I will not contact the member at this time.'	Jones: 'The below is noted. I am not in agreement with your feedback though.' Matthee excluded from resolution process.	Complete	Disclosure rejected; insubordination framing begins. PDA-protected disclosure met with dismissal.	Mary-Ann Jones, Lisa Mari (cc'd), W. Matthee
23 Oct 2025 12:54	Internal – Medscheme	Jones: 'I am not in agreement with your feedback though.' Matthee's clinical assessment overridden.	Clinical judgment dismissed without clinical basis.	Complete	Professional judgment dismissed.	Mary-Ann Jones, W. Matthee
23 Oct 2025 15:21	Internal – Medscheme	Matthee concedes under employer pressure: 'I realize that I should have contacted the member... I hope this instance of poor communication does not influence future requests.'	Jones acknowledges forced apology.	Complete	Constructive acquiescence – forced apology for raising patient safety concern.	Mary-Ann Jones, W. Matthee
31 Oct 2025	Internal – Medscheme	RECORDED RETALIATORY ADMISSION: Jones states: 'You made a case of harassment against me, so I can make a case of harassment against you. You misspelled my name in an email – go look.' Also warns: 'think of the ripple effect' of his actions.	Matthee records the interaction.	Complete	SMOKING GUN: Direct evidence of PDA breach, retaliatory intent, and veiled threat.	Mary-Ann Jones, W. Matthee
02 Nov 2025	Internal – HR/Union	Formal 3-page grievance against Jones: malicious intent, gaslighting, weaponising past issues, orchestrated provocation, campaign for removal, retaliation. Demands investigation, removal from team, written guarantee against retaliation.	No response documented.	Pending	30+ DAYS INSTITUTIONAL SILENCE BEGINS.	Cheryl-Dawn Modern, Sipokazi Makaula, Thando Mandondo, Tebogo Dikobe, W. Matthee
02 Nov 2025 22:55	Internal – HR	Request for review of July 2025 salary deduction – lack of written consent; staggering refused.	No response documented.	Pending	Ignored – part of institutional silence.	Cheryl-Dawn Modern, W. Matthee



Date / Time	Recipient / Body	Problem Identified	Response / Action	Status	Deflection / Pattern	Key Players
11 Nov 2025 17:15	Internal – HR	manager's inappropriate comment about online deliveries. Matthee formally disputes July 2025 deduction; reports manager's statements about staggering.	Cheryl-Dawn Modern acknowledges and requests time to look into matter.	Pending	Acknowledged but no resolution.	Cheryl-Dawn Modern, W. Matthee
20 Nov 2025	Internal – Medscheme	Time in lieu request for year-end functions.	Jones declines citing departmental adherence policy.	Complete	Selective policy application.	Mary-Ann Jones, W. Matthee
03 Dec 2025 17:26	Internal – Medscheme	Jones issues directive: no overtime, calls ≤30 minutes, work outside hours 'for your own account'.	Matthee challenges selective application of policies.	Complete	Selective enforcement; unreasonable restrictions.	Mary-Ann Jones, W. Matthee
04 Dec 2025 12:29	Executive Leadership – Medscheme/AfroCentric	Final internal escalation (32 attachments); unlawful deduction, intimidation/retaliation, 30+ days silence, performance discrepancies. Requests paid leave, financial settlement, written guarantee.	CRO Monwabisi Kula (09 Dec): acknowledges; feedback only in January due to 'festive break'.	Pending	DELAY TACTIC – 'festive break' response to urgent humanitarian crisis.	Gerald Van Wyk, Lindiwe Miyambu, Monwabisi Kula, Andrew Schwulst, Aklaaq Mahmood, W. Matthee
05 Dec 2025	CCMA	First CCMA referral (WECT23564-25) – Unfair Labour Practice (unlawful salary deduction, discrimination, victimisation).	Case scheduled for Com/Arb on 9 Jan 2026.	Scheduled	Not applicable.	W. Matthee, CCMA
05 Dec 2025 16:42	Internal – Medscheme	Matthee requests meeting conditions: recording, agenda, written responses. Bongani Luvuno claims only recently made aware.	Luvuno proposes engagement on 10 December.	Pending	Procedural delay – executive claims ignorance despite 30+ days silence.	Bongani Luvuno, Mapule Mashele, Gretchen Veldtman, W. Matthee
09 Dec 2025	Board of Directors – AfroCentric	Board escalation (38 attachments); discrimination, retaliation, unlawful deduction, 30+ days silence, King IV breaches. Requests independent investigation, manager suspension, board settlement.	Emails to Shadi Chauke and Lebohang Mpumwana (typo) fail – invalid addresses.	Partial failure	Technical deflection – invalid email addresses for key directors.	Nkateko Munisi, Alice le Roux, Bruno Fernandes, Shadi Chauke (invalid), Lebohang Mpumwana, W. Matthee
09 Dec 2025	Chief Risk Officer – Monwabisi Kula	Urgent plea: paid special leave, removal from manager's line, hardship payment.	Kula: asks for patience; 48-hour deadline unreasonable; commits to feedback 'in reasonable time'.	In progress	Delay tactic – 'takes time' / other critical things.	Monwabisi Kula, W. Matthee



Date / Time	Recipient / Body	Problem Identified	Response / Action	Status	Deflection / Pattern	Key Players
10 Dec 2025	CCMA	Employer (Xolile Manyati/Medscheme legal) files Opposing Affidavit in WECT23564-25, characterising diagnosed depression as 'cynical excuse', 'untruthful character', 'sloppiness', 'fishing expedition', 'abuse of system'. Requests dismissal with costs.	Affidavit served on CCMA and Matthee.	Complete	BAD FAITH LITIGATION – personal attacks under oath.	Xolile Manyati (Medscheme legal), W. Matthee
10 Dec 2025	Board/Executives	Matthee formally confirms executive-level resolution exhausted; redirects governance aspects to Board Committees.	No response documented.	Pending	Passed internally – executives to Board Committees.	W. Matthee, Lebhang Mpumwana
10 Dec 2025	Board/Executives	Urgent update: Opposing Affidavit filed – institutional bad faith, factual misrepresentation, attack on character, request for costs. Acknowledgment requested by 12 Dec 12:00.	No response documented.	Pending	Ignored – no acknowledgment by requested deadline.	Xolile Manyati, Board Chairs, Lebhang Mpumwana, W. Matthee
11 Dec 2025	Board of Directors – AfroCentric	Formal notice of resignation due to automatically unfair constructive dismissal. Cites opposing affidavit as final intolerable act.	Email to Gretchen.veldman@ fails – address not found.	Complete	Technical deflection – invalid email address.	Board of Directors, Lebhang Mpumwana, W. Matthee
11 Dec 2025	CCMA	Matthee files Replying Affidavit to employer's opposition.	Email to CMS@ccma.org.za fails – mailbox full (QuotaExceededException).	Failed	Technical deflection – CCMA mailbox full.	W. Matthee, CCMA
12 Dec 2025	Internal – Medscheme	Resignation effective – employment terminated due to intolerable conditions.	Resignation submitted.	Complete	Not applicable – constructive dismissal complete.	W. Matthee
16 Dec 2025	Board Audit & Risk Committee	Maximum Severity GRC Escalation: suppression of findings, retaliation against investigator, King IV/ISO/OHSALRA breaches. 24-hour compliance window.	CRO Kula (17 Dec): asks Matthee to stop emailing directors; warns impatience 'does not serve you well'.	Complete	GATEKEEPING – CRO blocks Board-level communication.	Monwabisi Kula, Nkateko Munisi, Alice le Roux, Bruno Fernandes, W. Matthee
17 Dec 2025	Board/Executives	Matthee retracts GRC escalation (sent on public holiday, invalidating timeline). Underlying concerns remain.	No response documented.	Complete	Not applicable – retracted by Matthee.	W. Matthee, Board members
18 Dec 2025	Board of Directors	CRO: 'can I please ask you to stop sending these e-mails to our directors? Sending further e-mails concerns remain.'	Matthee declines, asserting right to escalate material risks to oversight committees.	Complete	GATEKEEPING – CRO attempts to suppress	Monwabisi Kula, W. Matthee

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19 Dec 2025	CCMA	only gives them an indication of impatience and unreasonableness on your part. Second CCMA referral (WECT24486-25) – Automatically Unfair Constructive Dismissal (protected disclosure retaliation).	Case scheduled for Con/Arb on 9 Jan 2026.	Scheduled	Board-level governance escalation.	W. Matthee, CCMA
22 Dec 2025	Bruno Fernandes – Lead Independent Director	Humanitarian notice: two months arrears on rent, facing eviction due to executive retaliation. Requests hardship funds. Independent investigation; acknowledgment by 12:00 PM 23 Dec 2025.	No response documented.	Pending	Ignored – no acknowledgment received.	Bruno Fernandes, Lebohang Mpumlwana (cc'd), W. Matthee
23 Dec 2025	Board of Directors	Final humanitarian notice: demands financial relief; independent investigation; acknowledgment by 12:00 PM; otherwise regulatory escalation to SAHRC/JSE/CMS/IFSCA/B-BBEE/DEL.	No response. Matthee proceeds with all regulatory complaints as notified.	Complete	Ignored – Board failed to respond to humanitarian notice.	Board of Directors, Bruno Fernandes, Lebohang Mpumlwana, W. Matthee
23 Dec 2025	SAPS/NPA – Criminal Referral	Final notice: criminal referral for Medical Schemes Act (s66), OHSA (s26 & 8), B-BBEE Act (Reg 2.10). Deadline: 12:00 PM 24 Dec 2025.	No response documented.	Pending	Criminal justice process separate.	W. Matthee, SAPS, NPA
23 Dec 2025	Council for Medical Schemes (CMS)	Administrator misconduct, ethical failure, retaliation, Gexus/DMS integration gap generating false non-adherence reports to GEMS patients.	CMS acknowledges; requests complaint form and consent letter.	Pending	Not yet deflected at initial stage.	W. Matthee, Polite (CMS Complaints)
23 Dec 2025	SAHRC – Initial	Systemic governance failures, procedural obstruction, humanitarian crisis, right to dignity, housing, administrative justice.	No formal acknowledgement initially. Busisiwe Dlamini later responds requesting province.	Pending	Not yet deflected.	W. Matthee, Busisiwe Dlamini
23 Dec 2025	DEL – Initial	Unlawful salary deductions (>40%), gender discrimination, retaliation, constructive dismissal.	No immediate response documented.	Pending	Not yet deflected.	W. Matthee, DEL IES
23 Dec 2025	JSE – Initial	Board governance failure, P1 Material Risk Report ignored, retaliation via affidavit, King IV breaches.	Ncumisa Sinyanya acknowledges receipt same day.	Pending	Engaged – acknowledged.	Ncumisa Sinyanya (JSE Investigations), W. Matthee

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26 Dec 2025	Employer – CCMA Objection	Employer objects to Con-Arb in WECT24486-25 under Rule 17(a) (delay tactic).	Matthee objects to delay; notes humanitarian crisis (2 months arrears, eviction, no food/medication); requests expedited arbitration.	Pending	Employer delay tactic – Rule 17(a) objection.	Zera Rankutu, Ronel Vorster, W. Matthee
26 Dec 2025	CCMA	Matthee submits evidence dossier; notifies CCMA of SAHRC/JSE/CMS/FSCA/B-BBEE/DEL escalations.	Dossier attached to case file.	Complete	Not applicable.	W. Matthee, Ronel Vorster
07 Jan 2026 14:28	Employer – HR	Request for UI-19 form and exit documents for UIF application.	Cheryl-Dawn Modern – no response (ignored).	Pending	Ignored initial request.	Cheryl-Dawn Modern, W. Matthee
09 Jan 2026 12:00	CCMA	Conciliation/Con-Arb hearings held for both cases. Certificates of Non-Resolution issued.	Commissioner Nhabiseng presides. Matthee later files bias complaint.	Complete	Conciliation failed – matters proceed to Labour Court.	Commissioner Nhabiseng, Vilay Mc (employer rep), W. Matthee
10 Jan 2026 02:11	CCMA – Bias Complaint	Formal bias complaint against Commissioner Nhabiseng: pre-judgment ('nothing here relates to protected disclosure'), 'Google it' remark, misdirection on jurisdiction.	Maggie Kuntu (12 Jan): escalated to region; outcome within 14 working days.	Complete	CCMA complaints process engaged.	Commissioner Nhabiseng, Maggie Kuntu, Retha Ludeke, W. Matthee
14 Jan 2026	Employer – HR	Follow-up on retirement claim and UIF forms.	Sipokazi Makaula (15 Jan) requests identity verification – info already known.	In progress	Delay tactic – requesting basic info already on file.	Sipokazi Makaula, W. Matthee
15 Jan 2026 20:43	Employer – HR/Leadership	Matthee rejects stalling; provides notice of UIF non-compliance (June 2025 gap); demands docs by 19 Jan 12:00.	Email to Tebogo Makoe@ fails – 'Access denied'.	Partial failure	Email blocked – technical deflection.	Sipokazi Makaula, Tebogo Makoe, Lindiwe Mlyambu, Monwabisi Kula, W. Matthee
18 Jan 2026	Employer – HR	Employer responds: UIF declared (screenshots), UI-19 attached, Certificate of Service attached. Makaula: UI-19 only if resignation qualifies.	Documents provided but Certificate disputed – role downgraded.	Complete	Documents provided but disputed for accuracy.	Sipokazi Makaula, Seababatsa Sebotsa, W. Matthee
19 Jan 2026 12:25	Employer – HR/Leadership	Matthee formally disputes Certificate of Service: 'Care Coordinator' vs 'Specialist: Care Manager' (NQF 8); refers to recorded 'Naming Convention' Teams meeting.	Cheryl-Dawn Modern (WRITTEN ADMISSION): Certificate correct; naming convention changes 'still in progress'; no changes implemented for Matthee's department.	Complete	EMPLOYER REFUSES CORRECTION – written admission of ongoing naming convention process.	Cheryl-Dawn Modern (WRITTEN ADMISSION), Nhabiseng Nkane, Sipokazi Makaula, W. Matthee



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19 Jan 2026	Employer – HR	Matthee requests disclosure of recorded 'Naming Convention' Teams meeting as material evidence.	No response documented.	Pending	Ignored – employer withholds recording.	Cheryl-Dawn Modern, Nhlabiseng Nkatane, Sipokazi Makaula, W. Matthee
20 Jan 2026	JSE – Update	Failed conciliation, employer blocking financial relief, weaponising litigation, deepening humanitarian crisis.	Investigations acknowledges receipt; will review and revert.	Pending	Engaged – acknowledged.	Nadia Jada (JSE), W. Matthee
28 Jan 2026	CCMA	Vusumzi Landu responds to bias complaint: denies bias; warns against recording; confirms CCMA lacks jurisdiction for protected disclosure/automatically unfair dismissal arbitration.	Landu closes complaint. Matthee accepts; commits to Labour Court within 90 days.	Complete	Passed to Labour Court – CCMA confirms lack of jurisdiction.	Vusumzi Landu (Provincial Senior Commissioner), Aziza Tallep, W. Matthee
04 Feb 2026	CMS – Follow-up	CMS declines jurisdiction – complaint involves health professionals, not schemes/administrators. Refers to HPCSA.	No formal finding of misconduct. File closed on jurisdiction.	Complete	Passed to HPCSA.	CMS Complaints Unit, W. Matthee
10 Feb 2026	SAHRC – Formal Assessment	Alleged victimisation, discrimination, occupational detriment.	Lwethu Yoli: SAHRC not best placed – refers to CCMA/Labour Court and Public Protector. Financial aspects → DEL.	Complete	Passed to CCMA/Labour Court, Public Protector, and DEL.	Lwethu Yoli (SAHRC), W. Matthee
10 Feb 2026	HPCSA & SANC – Initial	Professional misconduct against Mary-Ann Jones and Mapule Mashele (nurses).	HPCSA: 'No jurisdiction over nurses' – refers to SANC.	Pending	HPCSA passed to SANC. SANC ignored complaint.	LegalMed (HPCSA), W. Matthee
10 Feb 2026	DEL – UIF & BCEA	False UIF declaration (June 2025 gap), fraudulent Certificate of Service, suppression of OHSA duties.	No acknowledgement for 6 days.	Pending	Ignored until follow-up.	David Esau, Fezeka Ngalo, Sipokazi Makaula
11 Feb 2026	HPCSA – Second Complaint	Complaint against HPCSA-registered Medical Directors and Lisa Mari (pharmacist).	HPCSA: states Lisa Mari falls under SAPC. Email to info@hpcsa.co.za blocked by mail flow rule.	Complete	Passed to SAPC. Blocked email – institutional access barrier.	LegalMed, W. Matthee
16 Feb 2026	DEL – Follow-up	48-hour deadline expired; June 2025 UIF gap still 'No information'; no case reference or inspector assigned.	Fezeka Ngalo responds same day: requests physical address, then allocates integrated inspection.	In progress	Engaged – Ngalo acted promptly.	Fezeka Ngalo, David Esau, Buziswa Ngqweleka, Bongive Binase, Hlanganisa Dalasile
16 Feb 2026	Minister of Health / NDOH	Ministerial escalation re SANC's non-response.	Emsie Grundling acknowledges (17 Feb). Veronica Taschi requests full correspondence	In progress	NDOH actively requesting documents – engaged.	Minister Aaron Motsoaledi, Veronica Taschi, Emsie Grundling

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16 Feb 2026	SAPC	Unprofessional conduct against Lisa Mari – dismissed system integration gap. Auto-ticket 5211-7743.	(18 Feb) Matthee provides attachments. 26 Mar 2026: SAPC asks to resend attachment. Emails to registrar and conduct department timed out.	Pending	Server unreachable / delayed response.	Lisa Mari, Isaac Nkanyane, Felicia Ngoveni, W. Matthee
16 Feb 2026	SANC	CMS→HPCSA→SANC referral chain; SANC failed to respond.	No response (6+ days silence). Ministerial escalation filed.	Pending	Ignored complaint.	W. Matthee, SANC Registrar, Minister of Health
16 Feb 2026	CCMA Ethics & Fraud Hotline	Follow-up on commissioner bias complaint.	Submission reviewed; referred to CCMA investigators. Ref: 350111. Minimum 21 days for feedback.	In progress	Properly referred – not a deflection.	Ethics & Fraud Hotline, CCMA investigators
19 Feb 2026	The Whistleblower House (TWBH)	Matthee submits request for whistleblower assistance.	Tsholofelo Mahlangu: referred to Marco van der Walt (Schroter Attorneys) for validity and scope assessment.	Pending	Not applicable – evaluation in progress.	Tsholofelo Mahlangu (TWBH), Marco van der Walt
23 Feb 2026	SAPS – Delft Police Station	Matthee attempts to open criminal case for statutory fraud (UIF Contributions Act, BCEA) against Medscheme. Presents full documentary evidence.	Station Commander refuses without reviewing evidence; states 'there is no case'; claims to decide prosecutorial merit; characterises request as 'attitude problem'.	Failed	UNLAWFUL REFUSAL – Station Commander usurped NPA function; refused to examine evidence; intimidated complainant.	Station Commander (Delft SAPS), W. Matthee
24 Feb 2026	SAPS – Provincial Commissioner (WC)	Formal complaint against Delft Station Commander: unlawful refusal, failure to examine evidence, usurpation of NPA function, intimidation.	Multiple email addresses failed. wc-provcomm@saps.gov.za successful.	In progress	Technical deflection – multiple invalid SAPS email addresses.	Provincial Commissioner (WC SAPS), W. Matthee
02 Mar 2026	Schroter Law / TWBH	Van der Walt provides preliminary legal opinion: Matthee does not qualify for PDA protection; constructive dismissal threshold not met.	Matthee provides detailed point-by-point rebuttal. TWBH eventually declines: 'does not meet eligibility criteria'.	Complete	TWBH declined assistance based on Schroter preliminary opinion.	Marco van der Walt, Tsholofelo Mahlangu, W. Matthee
04 Mar 2026	OCJ Court Online	Account activation for eRegistry.	Account activated; password reset confirmed.	Complete	Not applicable.	OCJ Court Online, W. Matthee
05 Mar 2026	Labour Court – OCJ Portal	Urgent application: BCEA s77A – unlawful salary deduction (40.47%); urgent interim relief sought.	Manual filing via Labour Court online portal.	Pending	Not yet deflected.	Farzaana Ismail, Tsakani Soundy, W. Matthee



Date / Time	Recipient / Body	Problem Identified	Response / Action	Status	Deflection / Pattern	Key Players
06 Mar 2026	JSE – Formal Assessment	Complaint re AfroCentric governance, King IV breaches, retaliation.	Jowdienne van Schalkwyk: JSE determines matters fall outside Listings Requirements – refers to CCMA/Labour Court/Equality Court.	Complete	Passed to CCMA/Labour Court/Equality Court – within JSE's legal mandate.	Jowdienne van Schalkwyk, Nadia Jada (Head: Investigations), W. Matthee
12 Mar 2026	OCJ Court Online	First BCEA case initiating document received (Ref: HCGs943758) against AfroCentric Health (RF) (Pty) Ltd.	Document received; pending approval.	Pending	Not applicable.	OCJ Court Online, Registrar's Office
13 Mar 2026	OCJ Court Online	Initiating Document rejected – confusing reason: 'good day you put date.'	Rejection notification.	Failed	Poorly explained rejection – access barrier to self-represented litigant.	OCJ Court Online, W. Matthee
13 Mar 2026	SAHRC – Formal File Update	Forensic payroll discrepancy, CCMA institutional bias, obstructed Labour Court filing, SAPS refusal, eviction imminent.	Awaiting feedback – no response documented.	Pending	No response to 13 Mar update.	W. Matthee, Busisiwe Dlamini, Lwethu Yoli
16 Mar 2026	DEL – Certificate of Service	Employer's Certificate misrepresents role as 'Care Coordinator' instead of 'Specialist: Care Manager' (NQF 8).	Matthew Holmes: Certificate matches UI declaration. DEL accepts employer's version.	Complete	DEL accepts employer's version without independent verification – passes to CCMA.	Matthew Holmes (Team Leader: BCEA), Bongilewe Binase, W. Matthee
17 Mar 2026	DEL – Salary Deduction	40.47% salary deduction without consent.	Matthew Holmes: DEL lacks authority under Section 70(b) – earnings above Minister's threshold. Suggests CCMA.	Complete	DEL cannot act – passed to CCMA.	Matthew Holmes, W. Matthee
17 Mar 2026	DEL – UIF Referral	UIF non-compliance at Medscheme.	Matthew Holmes: referred to UIF unit, Granville Van Wyk assigned as contact.	In progress	Passed internally – BCEA team to UIF unit.	Matthew Holmes, Granville Van Wyk, Thinabethu Mazibuko
17 Mar 2026	Board of Directors – Section 162 Notice	Formal service of Section 162 Director Delinquency Application. Respondents: B. Fernandes, N. Munisi, A. Le Roux, G. Van Wyk, LSM Mpumliwana. Urgent High Court proceedings Rule 6(12).	No response documented. Board reminded of SENS announcement obligation.	Pending	Ignored – no response. Now litigation respondents.	Bruno Fernandes, Nkateko Munisi, Alice le Roux, Gerald van Wyk, Lebohang Mpumliwana, W. Matthee
19 Mar 2026	SAPS – Delft Police Station	Criminal case registration following Brigadier Naidoo's intervention.	SMS: 'Case registered on 2026-03-19 at DELFT Station. Ref nr CAS 705/3/2026'.	Complete	Case registered after Brigadier Naidoo's intervention.	SAPS Delft, Brigadier Naidoo, W. Matthee



Date / Time	Recipient / Body	Problem Identified	Response / Action	Status	Deflection / Pattern	Key Players
20 Mar 2026	CMS – Follow-up	Uncorrected Gexus/DMS integration error – ongoing patient safety risk. High Court filing HCGS951842 referenced.	CMS auto-reply 2 Apr 2026: matter receiving attention.	Pending	Not yet deflected – awaiting substantive response.	W. Matthee, CMS Commissioner
23 Mar 2026	OCJ Court Online	Initiating Document approved and filed after multiple rejections. Case No. 2026-066952 (Section 162 Director Delinquency). Urgent Application scheduled for 25 Mar 2026.	Case filed; application scheduled; Practice Notes filed.	Complete	Not applicable.	OCJ Court Online, W. Matthee
25 Mar 2026 10:00 & 14:15	High Court – WC Division (Urgent Court 4)	Urgent Section 162 Director Delinquency Application – Matthee v Fernandes & Others, Case 2026-066952. Matthee self-represented; SC Graham for Respondents. Struck from roll. Unauthorised third party incident.	Judge Anderson presides. Scale C costs order granted against Matthee. Grounds for Rule 42 rescission identified.	Complete	Hearing held – rescission application and LPC complaint intended.	Judge Anderson, Senior Counsel Graham, W. Matthee (self-rep)
25 Mar 2026	SAPS – Delft/Woodstock	Case transfer confirmation.	SMS: 'DELFT ref nr CAS 705/3/2026 was transferred to WOODSTOCK station.'	Complete	Active criminal case – properly transferred.	SAPS Delft, SAPS Woodstock, W. Matthee
26 Mar 2026	High Court – Registrar's Office	Request for official audio recording and certified transcript for Rule 42 rescission application, Judicial Conduct Tribunal and LPC complaints. To Nicola Hanekom (Acting Chief Registrar) and Nosicelo Chwethiso.	No response from Hanekom or Chwethiso.	Pending	Ignored – no acknowledgment from Registrar's office.	Nicola Hanekom, Nosicelo Chwethiso, W. Matthee
26 Mar 2026	Inlexso – Transcription Service	Parallel request for audio recording and transcript.	Belinda Botes acknowledges; states 'I'll request the audio and revert.' Then goes on bereavement leave 31 Mar – 6 Apr.	In progress	Engaged – but no backup system when Botes goes on leave.	Belinda Botes, Tina Jorgenson, W. Matthee
01 Apr 2026	Director of Court Operations – R David	Formal complaint: Registrar's office non-response; Inlexso breakdown; no backup system; prejudice to self-represented litigant.	Awaiting response.	Pending	Not yet deflected.	R David (Director of Court Operations), W. Matthee
02 Apr 2026	OCJ Court Online	New BCEA case filed. Case No: 2026-077546. Matthee v Afrocentric Health (RF) (PTY) LTD.	Initiating Document approved and filed.	Complete	Not applicable.	OCJ Court Online, W. Matthee



Date / Time	Recipient / Body	Problem Identified	Response / Action	Status	Deflection / Pattern	Key Players
02 Apr 2026	CMS – Formal Follow-up	Gexus-DMS integration failure constitutes maladministration under Medical Schemes Act. Discovery Health precedent cited.	Auto-reply received: awaiting substantive response.	Pending	Not yet deflected.	CMS Commissioner, W. Matthee



TABLE 2: INTERNAL CHRONOLOGY — ISOLATED EMPLOYMENT EVENTS (Key Events)

This table isolates the internal employment chronology from the broader regulatory record. It traces the sequence of protected disclosures, employer responses, retaliatory conduct, and governance escalations that constituted the cause of action. Each entry includes the legal significance of the event for the LRA, PDA, EEA, and BCEA claims. This table is the primary reference for constructing the automatically unfair dismissal and protected disclosure retaliation narratives.

Date	Event	Response / Outcome	Legal Significance
11 Jan 2024	Recruitment Phase (Jan–Apr 2024): Matthee recruited and assessed for 'Specialist: Care Manager' role. Three interview invitations, assessment, background check completed.	Recruitment process complete; Matthee invited to proceed to offer.	Establishes legitimate expectation of Specialist role – foundation of B-BBEE fronting and title suppression argument.
02 Apr 2024	BAIT AND SWITCH: Offer made for 'Care Coordinator' role despite recruitment for 'Specialist'. Contract signed 9 Apr 2024 at grade B4M1, CTC R490,032 pa.	Matthee accepts; Nkatane confirms 2 May 2024 start date.	KEY DOCUMENT: Contract contradicts recruitment documentation. Core B-BBEE fronting evidence.
06 Mar 2025	FIRST DOCUMENTED THREAT: Jones threatens Matthee: 'this is the end of you'.	Matthee documents threat.	Establishes early hostile intent by Jones – contextualises all subsequent discrimination.
18 Mar 2025	First psychiatric admission: Akeso Stepping Stones (severe depression, PTSD), 3–4 weeks inpatient.	Dr Malan certifies medical condition.	Employer placed on constructive notice of disability from this point.
16 Apr 2025	Jones states: 'we do not apply staggering of money owed' – contradicts HCBP Policy.	Matthee given reduced-hours accommodation.	Employer's own statement contradicts policy – used to justify July 2025 unlawful deduction.
16 May 2025	Mental health disclosure: Dr Malan's letter (severe depression, PTSD) provided to Mapule Mashele.	Mashele receives disclosure; no immediate adverse action.	Employer placed on formal written notice of disability – held before July 2025 deduction.
23 Jul 2025	UNLAWFUL SALARY DEDUCTION: 40.47% (R14,811.77) without consultation while between admissions.	Employer invokes 'no work no pay'; manager denies staggering.	BCEA s34(2) violated. Employer held Dr Malan's letter. Bad faith established.
24 Jul 2025	Jones shares AOD policy showing 25% cap; acknowledges cap exists but defends 40.47% deduction.	Jones: 'This is for disability.' Matthee: what is illegal?	Critical admission: employer knew of statutory cap and violated it knowingly.
23 Sep 2025	GENDER IDENTITY DISCRIMINATION: Jones posts 'Wilma' in Teams channel with 'NO FILTERING' in red, immediately after Matthee proposed filtering strategy.	HR investigates. Matthee NOT removed from Jones's team.	EEA s6 violation. HR promises not fulfilled. Matthee left in hostile environment.
25 Sep 2025	Formal 11-page discrimination complaint: requests neutral investigator, suspension of accused, removal from team.	HR promises protection but does not act on removal request.	Protected activity under LRA s187(1)(f). Employer's failure to act = constructive complicity.
29 Sep 2025	RETALIATORY UNPAID LEAVE: Jones loads double punitive unpaid leave on exact same day Matthee files formal complaint.	Reversed by Mashele 1 Oct 2025.	Direct evidence of retaliation. Written proof of discretionary punitive conduct.

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Date	Event	Response / Outcome	Legal Significance
01 Oct 2025	Investigation closed: 'Wilma' dismissed as 'typo'. Matthee not removed from Jones's reporting line.	Whitewash finding issued.	Institutional whitewash. Matthee left in hostile environment. May 2025 disclosure ignored.
23 Oct 2025	PROTECTED DISCLOSURE (Patient Safety): Gexus-DMS integration gap identified; Matthee refuses to contact vulnerable patient on clinically incorrect grounds.	Jones overrides clinical judgment; labels refusal 'insubordination'.	PDA-protected disclosure. Insubordination charge = retaliatory adverse action under PDA s3.
31 Oct 2025	RECORDED RETALIATORY ADMISSION: Jones: 'You made a case of harassment against me, so I can make a case of harassment against you.'	Matthee records interaction.	SMOKING GUN: Causal nexus between protected complaint and retaliatory intent. PDA s3 breach confirmed.
02 Nov 2025	Formal 3-page grievance against Jones: malicious intent, gaslighting, retaliation. 30+ days institutional silence begins.	No response documented.	Constructive dismissal ingredient. 30+ days silence = futility of internal remedies.
04 Dec 2025	Final executive escalation (32 attachments). Requests paid leave, financial settlement, written guarantee.	CRO: feedback only in January due to 'festive break'.	Executive-level delay during humanitarian crisis. CRO's 'festive break' response is a governance failure.
09 Dec 2025	Board escalation (38 attachments). Final humanitarian notice. Some emails bounce due to invalid addresses.	CRO Kula asks Matthee to stop emailing directors.	Gatekeeping by CRO = suppression of governance escalation. Board non-response = Section 162 grounds.
10 Dec 2025	Employer's Opposing Affidavit: depression as 'cynical excuse'; conduct as 'untruthful and sloppy'; seeks costs.	Affidavit served on CCMA and Matthee.	BAD FAITH LITIGATION: Personal attacks under oath. Grounds for striking out and costs application.
12 Dec 2025	RESIGNATION (Constructive Dismissal): employment terminated due to objectively intolerable conditions.	Resignation submitted.	Section 186(1)(e) LRA: objectively intolerable = constructive dismissal. PDA s7 protection applies.
22-23 Dec 2025	Final humanitarian notice to Board and Lead Independent Director (Bruno Fernandes). No response → regulatory escalation triggered.	No response. Matthee proceeds with all regulatory complaints.	Board's silence = final confirmation of institutional failure. Triggers regulatory chain.



TABLE 3: KEY PATTERN IDENTIFICATION

This table synthesises twelve structural governance and legal patterns identified across the full correspondence record. Each pattern represents an independent cause of action or evidentiary foundation for the active proceedings. The patterns are arranged in order of legal materiality, beginning with the foundational title suppression and progressing through the regulatory deflection carousel and judicial access barriers.

Pattern	Description	Key Evidence / Examples	Legal Significance
1. Bait-and-Switch / Title Suppression	Matthee recruited as 'Specialist: Care Manager' but contracted as 'Care Coordinator' (B4M1). Role title systematically suppressed across employment records, Certificate of Service, and UIF declarations.	Employment contract (Apr 2024); Chery-Dawn Modern written admission (19 Jan 2026); Interview invitation documentation (Jan–Apr 2024); Certificate of Service (Jan 2026)	B-BBEE fronting grounds (Commission Case 5/02/2026); EEA occupational detriment; BCEA Certificate of Service misrepresentation grounds.
2. Protected Disclosure Retaliation Sequence	Each formal complaint or protected disclosure was followed by a demonstrable retaliatory act, creating a documented causal chain.	Mar 2025 complaint → 'end of you' threat; Jul 2025 deduction after May 2025 disability disclosure; Sep 2025 complaint → same-day retaliatory leave; Oct 2025 patient safety disclosure → insubordination charge; Oct 2025 complaint → recorded retaliatory admission	PDA s3 breach established across multiple protected disclosures. LRA s187(1)(f) automatically unfair dismissal grounds.
3. Institutional Whitewash	Formal complaints were dismissed with procedurally inadequate findings, minimising serious conduct and permitting the hostile environment to continue.	'Wilma' dismissed as 'typo' (Oct 2025); May 2025 disclosure of Jones's conduct ignored in October investigation; Matthee left in Jones's team despite being granted protection	Bad faith in employer's internal processes; undermines any argument that internal remedies were available or adequate.
4. 30+ Days Institutional Silence	Formal grievance of 2 November 2025 received no response for over 30 days, constituting a structural failure of internal remedy.	02 Nov 2025 grievance → no response; 02 Nov salary deduction dispute → no response; 11 Nov escalation → only acknowledged without resolution	Constructive dismissal ingredient (LRA s186(1)(e)); demonstrates futility of internal remedies for purposes of urgent Labour Court relief.
5. Executive Gatekeeping	CRO Monwabisi Kula actively blocked Board-level escalation, requesting Matthee stop emailing directors and characterising governance escalation as 'impatience'.	17 Dec 2025: 'can I please ask you to stop sending these e-mails'; 18 Dec 2025: gatekeeping email repeated	King IV governance failure; suppression of protected escalation to oversight committees; evidence for Section 162(5)(c) delinquency application.
6. Regulatory 'Passed Buck' Carousel	Every regulatory body except NDOH referred Matthee to another forum, creating a systemic access-to-justice failure.	CMS→HPCSA; HPCSA→SANC; HPCSA→SAPC; JSE→CCMA/Labour Court; SAHRC→CCMA/Labour Court/PP/DEL; DEL (BCEA)→CCMA; DEL (salary)→no action	Systemic failure of regulatory oversight; supports Biowatch challenge to costs order; documents futility argument.
7. Technical Barrier Pattern	Invalid email addresses, full CCMA mailboxes, inaccessible SAPS channels and failed deliveries systematically delayed or prevented engagement.	CCMA mailbox full (3 occasions Dec 2025); FSCA email invalid; JSE regulation@ blocked; HPCSA email blocked; SAPS multiple invalid addresses;	Constitutes systemic obstruction of access to justice for self-represented litigants; relevant to rescission application and access-to-justice argument under Constitution s34.



Pattern	Description	Key Evidence / Examples	Legal Significance
8. Bad Faith Litigation	Employer's legal team deployed personal attacks in sworn affidavit rather than substantive legal opposition.	Registrar email invalid: Inlexso no backup system 10 Dec 2025 Opposing Affidavit: 'cynical excuse'; 'untruthful character'; 'abuse of system'; 'fishing expedition'; seeks costs	Relevant to costs applications; grounds for striking out; admissible as evidence of employer's retaliatory intent
9. Document Misrepresentation (Certificate of Service)	Certificate of Service systematically downgraded Mathee's role from 'Specialist: Care Manager' (NQF 8) to 'Care Coordinator', contradicting appointment letter and Naming convention	Cheryl-Dawn Modern (19 Jan 2026): written admission that naming convention 'still in progress'; DEL (16 Mar 2026): accepted employer's version without verification	Core B-BBEE fronting allegation; BCEA Certificate misrepresentation; CCMA/Labour Court claim grounds.
10. UIF Non-Compliance	June 2025 UIF contributions deducted but not remitted or declared, creating a verifiable gap in uFiling records.	uFiling shows 'No Information' for June 2025; employer screenshot disputed; DEL UIF unit referral (Granville Van Wyk, 17 Mar 2026)	UIF Contributions Act criminal violation; active SAPS case CAS 705/3/2026.
11. Board Non-Response / Governance Collapse	Board failed to respond to multiple escalations including final humanitarian notice and is now a Respondent in delinquency proceedings.	22 Dec and 23 Dec 2025; humanitarian notices – no response; 14 Mar 2026 Rule 6 Notice – no response; 17 Mar 2026 Section 162 application – no response	Section 162(5)(c) delinquency grounds; SENS non-disclosure; wilful misconduct allegation; King IV Chapter 2 failure.
12. OCJ Portal Obstruction	Five submissions and four rejections of initiating documents, including vague and inconsistent rejection reasons, created material barriers to court access.	12–23 Mar 2026: rejections for 'good day you did put the date'; jurisdiction (Labour Court); annexures; urgency date	Access-to-justice failure (Constitution s34); relevant to Rule 42 rescission grounds; self-represented litigant prejudice.



TABLE 4: UPDATED AWAITING FEEDBACK REGISTRY

This table reflects the current status of all outstanding matters as at 03 April 2026. Urgency ratings are assigned as follows: **CRITICAL** = immediate action required within 48 hours (deadline-sensitive or rights-impairing); **HIGH** = action required within 7 days; **MEDIUM** = monitor and follow up within 14 days. The 'Recommended Next Step' column provides the specific procedural action required.

Regulator / Body	Last Action	Status	Urgency	Recommended Next Step
High Court (2026-066952) – Section 162	25 Mar 2026 – Scale C costs order, struck from roll	Awaiting audio/transcript for rescission	CRITICAL	File Rule 42 rescission; obtain Inexso transcript; file LPC complaint re unauthorised third party
High Court (2026-077546) – BCEA Claim	02 Apr 2026 – BCEA case filed (WWWBCEAWC2026)	Awaiting hearing date	HIGH	Monitor OCJ portal; prepare particulars of claim; confirm Labour Court registry receipt
SAPS – Woodstock (CAS 705/3/2026)	25 Mar 2026 – case transferred; new case number pending	Awaiting new case number	HIGH	Follow up with Woodstock SAPS (021-4423100); confirm detective contact
Director of Court Operations – R David	01 Apr 2026 – formal complaint submitted	Awaiting response	HIGH	Escalate if no response by 7 Apr 2026 (Botes return date)
High Court Registrar (Hanekom/Chwethiso)	26 Mar 2026 – audio recording request	No response (ignored)	CRITICAL	Escalate via R David; consider PAJA s11 request; Rule 30A notice
Inexso (transcription service)	02 Apr 2026 – will resubmit 7 Apr 2026	Awaiting High Court IT	HIGH	Confirm by Belinda Botes on 7 Apr; request written quote and turnaround time
DEL – UIF Unit (Granville Van Wyk)	17 Mar 2026 – referred internally by Holmes	Awaiting UIF assessment	HIGH	Follow up directly with Granville Van Wyk; provide uFiling June 2025 gap extract
SAHRC (Diamini / Yoli)	13 Mar 2026 – forensic payroll update sent	Awaiting feedback	MEDIUM	Follow up; reference Labour Court proceedings and imminent eviction
CMS Commissioner	02 Apr 2026 – auto-reply only	Awaiting substantive response	HIGH	Escalate with HCGS951842 reference; request formal investigation under MSA s47
SAPC (Nkanyane / Ngoeni)	26 Mar 2026 – asked to resend attachment (Ref 5211-7743)	Awaiting further response	MEDIUM	Resend with read-receipt; note historical email timeout pattern
SANC Registrar	16 Feb 2026 – complaint filed	No response (ignored)	MEDIUM	Escalate to National Chief Nursing Officer and Provincial Director of Nursing
JSE (Jowdlene van Schaikwyk)	Post-6 Mar 2026 – JSE engaging company	Awaiting company response	HIGH	Monitor SENS announcements by AfroCentric; provide updated court filing details
B-BBEE Commission (Case 502/2026)	17 Mar 2026 – ccd on Section 162 notice	Awaiting investigation progress	HIGH	Provide Cheryl-Dawn Modern admission (19 Jan 2026) and Certificate of Service as key exhibits



Regulator / Body	Last Action	Status	Urgency	Recommended Next Step
NDOH / Minister of Health	18 Feb 2026 – Taschl requested full correspondence	In progress (NDOH engaged)	MEDIUM	Provide updated SANC complaint status; send any new nursing regulatory correspondence
Board of Directors / Section 162 Respondents	17 Mar 2026 – Section 162 served; no response	Ignored – now litigation respondents	CRITICAL	Monitor SENS announcements; challenge Biowatch costs order; prepare for next court date
CCMA Investigators (Ref 350111)	16 Feb 2026 – 21-day window (expired –9 Mar)	No feedback – likely closed	MEDIUM	Follow up with reference number; confirm whether investigation concluded by Landu



TABLE 5: CRITICAL DOCUMENTS REFERENCED IN CORRESPONDENCE

This table catalogues every key document referenced across the full correspondence record and assigns each to its legal purpose in the active proceedings. Documents marked **URGENT** require immediate action to prevent evidentiary loss or procedural prejudice. The table serves as the primary document registry for trial preparation, regulatory submissions, and the Rule 42 rescission application.

Document / Item	Relevance	Status / Location
Interview invitation documentation (Jan–Apr 2024)	Establishes recruitment for 'Specialist: Care Manager' role – foundational document for bait-and-switch / title suppression argument and B-BBEE fronting allegation.	Filed – Neo Sekgoka correspondence; Nthabiseng Nkatane correspondence
Employment contract (09 Apr 2024)	Contract designates 'Care Coordinator' (grade B4M1) despite Specialist recruitment – core evidence of title suppression and potential B-BBEE fronting.	Filed – Barbara Johnson / W. Mathee signed contract
First documented threat: 'this is the end of you' (06 Mar 2025)	Establishes early hostile intent by Jones; contextualises subsequent discrimination and retaliation.	Filed – Matthee's contemporaneous documentation
Akeso Stepping Stones admission records (Mar–Apr 2025)	Confirms inpatient psychiatric treatment (severe depression, PTSD); establishes period of incapacity for no-work-no-pay challenge.	Filed – Dr Jacques Malan / Akeso records
Dr Jacques Malan letter (16 May 2025)	Confirms severe depression and PTSD; employer held this letter before July 2025 deduction – establishes bad faith and knowledge of disability.	Filed – KEY EXHIBIT for BCEA and LRA claims
Jones 'no staggering' statement (16 Apr 2025)	Jones's written position that company does not allow staggering – directly contradicts HCBP Policy and employer's later defence.	Filed – Meeting feedback documentation
HCBP Policy (employer's own policy)	Employer's own policy treats recovery as deduction subject to 25% cap and mandatory prior consultation – contradicts employer's position.	Filed – Contradicts employer's defence
AOD Policy (25% cap acknowledgement, 24 Jul 2025)	Jones shares policy confirming 25% cap; then defends 40.47% deduction – direct evidence of wilful non-compliance.	Filed – Critical: employer knew of cap
Payslip showing 40.47% deduction (Jul 2025)	Net pay R15,679.29 – confirms deduction exceeded BCEA s34(2) cap. Amount: R14,811.77.	Filed – KEY EXHIBIT for BCEA s77A claim
First Aider roster (25 Jul 2025)	Matthee drafts and implements management-level roster – direct contradiction of 'Care Coordinator' designation; evidences Specialist-level responsibility.	Filed – Management role evidence
'Wilma' Teams message (23 Sep 2025)	Gender identity discrimination incident – screenshot/system evidence required.	Filed – EEA s6 violation evidence
Formal discrimination complaint (25 Sep 2025)	11-page complaint with specific procedural requests – establishes PDA chain and employer's obligations.	Filed – Anchor document for retaliation chronology



Document / Item	Relevance	Status / Location
Retaliatory unpaid leave entry (29 Sep 2025)	System record: entry loaded on same day as formal complaint; reversed after escalation – written proof of discretionary punitive conduct.	Filed – Direct evidence of retaliation
Mashele investigation closure (01 Oct 2025)	Whitewash document: dismisses discrimination as 'typo'; Matthee not removed from Jones's team; May 2025 disclosure ignored.	Filed – Evidence of institutional whitewash
Gexus-DMS root cause analysis (23 Oct 2025)	Confirms systemic integration gap – not isolated glitch; affects all members transitioning between benefit categories.	Filed – Supports PDA 'general public interest' threshold
Recorded retaliatory admission (31 Oct 2025)	Jones: 'You made a case of harassment against me, so I can make a case of harassment against you.' Audio recording.	Filed – SMOKING GUN for PDA retaliation claim
Formal grievance (02 Nov 2025)	3-page grievance against Jones – no response = 30+ day institutional silence; establishes futility of internal remedies.	Filed – Constructive dismissal predicate
Executive escalation (04 Dec 2025)	32 attachments; CRO responded with 'festive break' delay – establishes exhaustion of internal remedies.	Filed – Exhaustion of internal remedies
Cheryl-Dawn Modern written admission (19 Jan 2026)	Written confirmation that naming convention changes 'still in progress' – supports B-BBEE fronting and Certificate of Service misrepresentation.	Filed – KEY EXHIBIT for B-BBEE Commission Case 5/02/2026
Employer's Opposing Affidavit (10 Dec 2025)	Personal attacks under oath: 'cynical excuse'; 'untruthful and sloppy'; seeks costs – bad faith litigation evidence.	Filed – Grounds for costs application and striking out
Resignation letter (11–12 Dec 2025)	Cites opposing affidavit as final intolerable act; constitutes constructive dismissal under LRA s186(1)(e).	Filed – LRA s187(1)(f) automatically unfair dismissal
CCMA Certificates of Non-Resolution (09 Jan 2026)	WECT23564-25 and WECT24486-25 – jurisdictional gateway to Labour Court proceedings within 90 days.	Filed – Jurisdictional gateway documents
uFiling extract showing June 2025 gap	Confirms UIF contributions deducted but not declared; 'No Information' for June 2025.	Filed – Active SAPS case CAS 705/3/2026
SMS confirmation CAS 705/3/2026 transfer (25 Mar 2026)	SAPS SMS confirming case transfer from Delft to Woodstock – active criminal investigation.	Filed – Criminal case confirmation
Audio recording of Delft Station Commander (23 Feb 2026)	Records unlawful refusal to open criminal case; Station Commander usurping NPA prosecutorial function.	Filed – Grounds for IPID complaint
High Court audio recording (Case 2026-066952 – 25 Mar 2026)	Official record required for Rule 42 rescission application and Judicial Conduct Tribunal complaint.	URGENT – Audio not yet obtained; Registrar non-responsive; Inlexso request pending
Director of Court Operations complaint (01 Apr 2026)	Documents Registrar non-response and Inlexso backup system failure – relevant to Rule 42 rescission application.	Filed – Supports access-to-justice argument

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Document / Item	Relevance	Status / Location



CONCLUSION: STATUTORY AND CONSTITUTIONAL FAILURES – COMPREHENSIVE ANALYSIS

The following analysis addresses how the documented institutional failures — across the employer, regulatory bodies, and judicial administration — constitute violations of the Labour Relations Act, Protected Disclosures Act, Employment Equity Act, Basic Conditions of Employment Act, Companies Act, and the constitutional rights of the whistleblower as a self-represented litigant. These failures are not isolated incidents; they form a systemic pattern of institutional resistance to accountability that is itself a governance failure of the highest order.

1. Labour Relations Act (LRA), Act 66 of 1995

Applicable Sections: s185 (right not to be unfairly dismissed); s186(1)(e) (constructive dismissal); s187(1)(f) (automatically unfair dismissal – protected disclosure retaliation); s5 (freedom of association and protected activity)

Institutional Failure Analysis: The employer's conduct across the period May–December 2025 constitutes automatically unfair constructive dismissal. The documented sequence — disability disclosure (May) → unlawful deduction (July) → discrimination complaint (September) → patient safety disclosure (October) → recorded retaliatory statement (October) → institutional silence (November) → opposing affidavit attacks (December) — establishes the causal chain required under s187(1)(f). The employer's failure to apply the CCMA's outcome, remove Matthee from the hostile reporting line, or address the 30+ day institutional silence demonstrates that the employer's conduct was calculated to make continued employment intolerable. The CRO's gatekeeping communications (17–18 December 2025) evidence active suppression of the governance escalation.

2. Protected Disclosures Act (PDA), Act 26 of 2000 (as amended by Act 5 of 2017)

Applicable Sections: s1 (definition of protected disclosure); s3 (protection against occupational detriment); s6 (protected disclosure to employer); s7 (protected disclosure to external body)

Institutional Failure Analysis: Matthee made at least two disclosures that qualify as protected under the PDA: the Gexus-DWS system integration gap (patient safety, 23 October 2025) and the systemic UIF non-compliance and B-BBEE fronting (disclosed through formal grievances and regulatory complaints). The immediate response — insubordination charge, exclusion from the resolution process, and Jones's recorded statement ('You made a case of harassment against me, so I can make a case of harassment against you') — constitutes occupational detriment under s1 of the Act. The Schroter Law preliminary opinion that the threshold for PDA protection was not met has been substantively rebutted; the system integration disclosure satisfies the 'general public interest' threshold in s1(vii) and the employer's institutional response satisfies the 'occupational detriment' definition. The employer's failure to investigate protected disclosures, the whitewash investigation outcome, and the opposing affidavit's personal attacks further constitute detriment under the Act.

3. Employment Equity Act (EEA), Act 55 of 1998

Applicable Sections: s6(1) (prohibition of unfair discrimination); s6(3) (harassment as unfair discrimination); s11 (burden of proof); s10 (obligation to eliminate unfair discrimination)

Institutional Failure Analysis: The 'Wilma' incident (23 September 2025), Jones's documented pattern of hostile conduct from March 2025, and the employer's whitewash investigation (1 October 2025) constitute unfair discrimination on the grounds of gender identity and sexual orientation under s6(1). The investigation's failure to remove Matthee from the hostile reporting line after a formal complaint — despite HR's promise that he would not be 'required to meet with the manager' — constitutes a failure to take all reasonable steps to prevent harassment under s60. The title suppression (recruited as Specialist, contracted as Coordinator) may also constitute discrimination in the terms and conditions of employment. Under s11, the burden shifts to the employer to show that the discrimination was fair; given the documented conduct, no such justification is available.

4. Basic Conditions of Employment Act (BCEA), Act 75 of 1997

Applicable Sections: s34 (deductions from remuneration); s34(2) (25% cap on deductions); s35 (employer duties re deductions); s77A (civil claim for statutory breach)

Institutional Failure Analysis: The July 2025 salary deduction of 40.47% (R14,811.77) exceeded the statutory 25% cap in s34(2) without consultation, without written consent, and in full knowledge of Matthee's disability. Jones's contemporaneous acknowledgment of the 25% cap (24 July 2025: 'monthly deductions must not be more than 25%') while



defending the deduction ("This is for disability") constitutes wilful non-compliance. DEL's determination that it lacks jurisdiction under s70(b) (earnings threshold) does not extinguish the civil claim under s77A of the BCEA, which has now been filed as Case No: 2026-077546 in the Western Cape High Court. The Certificate of Service misrepresentation — downgrading role from Specialist to Coordinator — constitutes a further s34A violation.

5. Constitution of the Republic of South Africa, Act 108 of 1996

Applicable Sections: s9 (equality and non-discrimination); s10 (human dignity); s14 (privacy); s23 (labour rights); s34 (access to courts); s195 (public administration values — applicable to regulatory bodies)

Institutional Failure Analysis: The systemic failure of regulatory bodies to provide effective remedy — the 'passed buck' carousel across CMS, HPCSA, SANC, SAPC, JSE, SAHRC, and DEL — constitutes a failure of the constitutional obligation to ensure that rights in the Bill of Rights are not infringed and can be enforced (s7(2)). Matthee's right to access courts (s34) was directly impaired by: the CCMA's repeated mailbox full failures; the OCAJ portal's multiple procedurally vague rejections; the Registrar's failure to provide the court recording; and the Delft Station Commander's unlawful refusal to open a criminal case. His right to human dignity (s10) was violated by the opposing affidavit's characterisation of his diagnosed mental illness as a 'cynical excuse'. The employer's failure to reasonably accommodate his disability implicates s9(3). The Blowatch principle (Blowatch Trust v Registrar, Genetic Resources 2009 (6) SA 232 (CC)) provides the constitutional basis for challenging the Scale C costs order: where a private litigant litigates to enforce constitutional rights against a private party that has suppressed public-interest protected disclosures, a punitive costs order is constitutionally disproportionate.

6. Companies Act (No. 71 of 2008) – Section 162

Applicable Sections: s162(5)(c) (delinquency for gross negligence, wilful misconduct, breach of fiduciary duty); s76 (directors' standard of conduct); s77 (liability for breach); King IV Report on Corporate Governance

Institutional Failure Analysis: The Board of Directors' failure to respond to Matthee's formal governance escalations — including the Priority 1 Material Risk Report delivered 9 December 2025 with 38 attachments, the subsequent humanitarian notices of 22–23 December 2025, the Rule 6 Notice of 14 March 2026, and the formal Section 162 application notice of 17 March 2026 — constitutes breach of fiduciary duty and gross negligence under s76 and s162(5)(c). The Board's failure to disclose material litigation (the Section 162 delinquency application and the B-BBEE Commission investigation with R580m exposure) to the market via SENS constitutes a separate wilful misconduct allegation under the Companies Act and JSE Listings Requirements. The CRO's gatekeeping communications and the employer's bad faith affidavit suggest that the Board's non-response was deliberate rather than inadvertent.

OVERALL SYSTEMIC CONCLUSION

The pattern documented across these tables reveals that Wilbur Matthee faced not a single institutional failure but a cascading, interlocking series of structural obstructions. At the internal level, the employer deployed the full arsenal of corporate deflection: whitewash investigations, institutional silence, gatekeeping, retaliatory conduct, document misrepresentation, and bad faith litigation. At the regulatory level, every external body except NDOH deflected to another forum, creating an access-to-justice vacuum. At the judicial administration level, OCAJ portal rejections, Registrar silence, and the Inlexso backup system failure compounded the prejudice to a self-represented litigant navigating unprecedented governance terrain without legal aid.

The whistleblower protection framework of the PDA exists precisely for cases such as this: where a person with direct clinical and governance knowledge identifies a material risk, attempts to escalate it through every available channel, and is systematically punished for doing so. The documented causal chain — disclosure → detriment → institutional silence → regulatory carousel → judicial obstruction — constitutes a case study in the structural vulnerabilities that the PDA, LRA, EEA, and Constitution were designed to address.

The active SAPS criminal investigation (CAS 705/3/2026), the pending Labour Court BCEA claim (Case 2026-077546), the B-BBEE Commission investigation (Case 5/02/2026), the intended Rule 42 rescission application in High Court Case 2026-066952, and the ongoing JSE SENS disclosure review represent the remaining institutional pathways through which the record documented in these tables may achieve its intended purpose: accountability, remedy, and systemic correction.

Compiled by: EthicHawks Forensic Governance Consultancy | info@ethichawks.co.za | Cape Town, South Africa | 03 April 2026
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ANNEXURE

“Z1”

The Initial Disclosure – Duplicate LACE Files and Early Suppression – 6 Mar 2025)

Date: 6 March 2024

Relevance: *Part G — Protected Disclosure*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
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Hi Mary-ann, i think this is really a waste of time i could have used to call members it seems that all the LACE cases that you have allocated was also allocated to other members i had to open these files on all application gexus, ums, and dms which is time consuming i would like to know if i can claim this time back on my timesheet ?

[illegible]

im not assuming or insinuating that this is done deliberate because how could it. im just concern about the time consumed fishing through these profiles



MJ

Hi Wilbur, one question. Why mention it if it is not believed to be so? Please, I come to work to work and to work in peace. I would appreciate if you afford me my peace. Thanks

Hi Mary-Ann ,I mentioned it because I did not want you to jump to conclusion such as this as it could have been taken the wrong way. I also did not want you to think that i am accusing you of anything it was just to provide clarity. just to be crystal clear its not believed.

standing on different 6.3.2025
comes to stalk me. This is a form
of bullying/intimidation. 13/4/2026 11:58:23 AM

↳ Came to my desk to make small talk
about my umbrella at my desk. Then
jumped straight to the message.
I explained to her my initial response
is to provide clarity as things could
easily be taken the wrong way.
She talked in a very aggressive
manner "stating" that the words I used
was unmemory and I could have just
straight asked her why she is
giving me duplicate files or did
you give me duplicate files "I found
that this more cause for drama
Explained to her that I only did this
to provide clarity as my initial response
could be viewed accusatory. She remained
on the defense and walked away
As she walk away she threatened
me stating this is the 2nd time you:
Isp I told her she do not feel my msg's.

ANNEXURE

“P1 RISK”

NQF Mandate / SAQA Statutory Obligation to Report (ELO mapping; qualification transcript)

Date: 8 April 2026

Relevance: Part G — Protected Disclosures



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Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

IN THE LABOUR COURT OF SOUTH AFRICA, CAPE TOWN

Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd

ANNEXURE P1 RISK

THE NQF MANDATE, THE PROFESSIONAL DEVELOPMENT DISTINCTION,
AND THE RESPONDENTS' DISREPUTE OF THE SAQA, THE NQF ACT, AND THE
PROTECTED DISCLOSURES ACT

RESERVED FOR ORAL TESTIMONY • FINAL ANNEXURE TO STATEMENT OF CLAIM

**SAQA Qualification ID 118316 • NQF Level 8 • CPUT • 90% Distinction in Professional
Development**

**1. PURPOSE OF THIS ANNEXURE**

This Annexure is placed before the Honourable Court as the capstone document of the Applicant's Statement of Claim. It does not repeat the factual record — that record stands fully in the Statement of Claim, the supporting bundle, and Annexures C, D, and E. Its purpose is singular: to demonstrate, with statutory and academic precision, that every act for which the Respondents punished the Applicant was not merely lawful. It was mandated.

The Applicant holds a Postgraduate Diploma in Health Services Management (SAQA Qualification ID 118316, NQF Level 8, Cape Peninsula University of Technology), awarded with a 90% Distinction in Professional Development. The Exit Level Outcomes of that qualification, as registered by the South African Qualifications Authority and gazetted under the National Qualifications Framework Act 67 of 2008, impose specific professional obligations on the holder. This Annexure maps those obligations to the specific acts the Respondents characterised as misconduct.

The argument is this: the Respondents did not merely dismiss an employee. They employed a statutory specialist, extracted the full value of his qualifications, and then systematically destroyed him for discharging the very obligations those qualifications impose. In doing so, they brought into disrepute the SAQA, the NQF Act 67 of 2008, the Protected Disclosures Act 26 of 2000, and the constitutional values of accountability and transparency that these instruments were enacted to protect.

2. THE STATUTORY ARCHITECTURE: WHY NQF EXIT LEVEL OUTCOMES ARE BINDING PROFESSIONAL OBLIGATIONS**2.1 The NQF Act 67 of 2008 — Qualifications as Instruments of Public Policy**

The National Qualifications Framework Act 67 of 2008 establishes the NQF as a national policy instrument of the Republic of South Africa. Its stated objects include facilitating access and progression within education and career paths, enhancing the quality of education and training, accelerating redress of past unfair discrimination, and contributing to the full personal development of each learner and to the social and economic development of the nation.

Qualifications registered on the NQF are not private credentials between an institution and a graduate. They are public instruments of national development, carrying Exit Level Outcomes that define what the holder is required to be able to do in the interests of the public, the profession, and the State. Section 13 of the NQF Act mandates SAQA to register qualifications on the NQF and to ensure that they serve the public interest. The holder of such a qualification, when employed in a role that activates those competencies, is not merely an employee discharging a job description. He is a statutory agent of the governance function that qualification was enacted to produce.

SAQA Qualification ID 118316 — Postgraduate Diploma in Health Services Management — is a registered NQF Level 8 qualification. Its Exit Level Outcomes are public instruments of national health policy. The Applicant is not merely an employee — he is a credentialled statutory agent of the clinical governance function the South African Nursing Council, SAQA, and Parliament designed this qualification to produce.



2.2 The Specific Exit Level Outcomes the Applicant Was Discharging — Mapped to the Respondents' Conduct

The following table maps the relevant Exit Level Outcomes of SAQA ID 118316 to the specific acts the Respondents characterised as insubordination, misconduct, or disruptive behaviour. Each item in the third column is an act of professional compliance. Each was met with suppression, penalisation, or appropriation.

ELO	SAQA Exit Level Outcome (SAQA ID 118316)	Applicant's Act in Direct Discharge of That Outcome
ELO 2	Demonstrate accountability in managing specialised nursing services; identify legislative requirements; resolve ethical-legal dilemmas; advocate for safe and quality care.	Identification and formal reporting of the Gexus-DMS Priority 1 clinical integration failure — a statutory governance obligation, not an employment choice.
ELO 5	Comprehensive, critical understanding of the management process; apply evidence-based solutions; ground work-related behaviour in management processes.	Root Cause Analysis of the Gexus-DMS failure, delivered as a formal Priority 1 Material Risk Report on 9 December 2025 with 38 evidentiary attachments.
ELO 6	Function effectively as a nurse manager; critically review complex issues using evidence-based solutions, strategic planning, and theory-driven arguments; draw a strategic plan for managing risks and adverse events.	Design and development of the CPD programme (April 2025) — a direct product of evidence-based strategic management competency. Silent departmental implementation by management (October 2025) confirms its clinical and organisational value.
ELO 7	Formulate and implement an effective strategic plan for a defined healthcare service, taking cognisance of internal strengths, weaknesses, and external threats.	The CPD programme was a strategic learning intervention targeting a documented departmental competency gap. Its architecture reflects formal strategic planning methodology applied to a real departmental need.
ELO 8	Demonstrate coherent and in-depth understanding of principles of	Identification of the forensic R5,948.62 discrepancy between uFiling salary declarations

ELO	SAQA Exit Level Outcome (SAQA ID 118316)	Applicant's Act in Direct Discharge of That Outcome
	budgetary governance in a health service.	and payslip gross earnings — direct application of budgetary governance and financial oversight competency.
Professional Development (90% Distinction)	Master the design and delivery of professional development frameworks; apply mentorship architecture; sustain lifelong learning for nursing units; develop policy for professional development programmes.	The CPD programme IS this ELO in applied form. A 90% Distinction at NQF Level 8 confirms formal, externally assessed mastery of professional development design. The Applicant's authority to create CPD programmes is not aspirational — it is the highest credentialled competency in the room.

The table above is not a list of things the Applicant chose to do. It is a list of things the Applicant was required to do by the qualification the Respondents knowingly hired him to hold. There is no version of these events in which the Respondents can characterise any item in the third column as misconduct without simultaneously characterising the SAQA-registered Exit Level Outcome in the second column as an employment liability.



3. THE 90% DISTINCTION IN PROFESSIONAL DEVELOPMENT: THE APPLICANT AS THE DEFINITIVE CPD AUTHOR

3.1 What the Professional Development ELO Requires at NQF Level 8

The Professional Development module within SAQA Qualification ID 118316 is not a generic skills subject. At NQF Level 8, its Associated Assessment Criteria require the holder to demonstrate the following competencies — each of which is directly and specifically relevant to the CPD programme the Applicant designed and the Respondents misappropriated:

- Assess the degree of self-ownership in developing professionally to remain a competent practitioner;
- Develop a framework to demonstrate the process of self-development in the professional career;
- Create a plan to establish academic and professional relationships to sustain lifelong learning amongst nurses;
- Debate the purpose of mentorship in nursing units and apply a model for implementing mentorship for novices;
- Use performance appraisal principles to monitor professional development within the programme context; and
- Distinguish between the mentor and mentee role, and develop a unit-level policy for a mentoring contract.

These are not competencies the Applicant demonstrated in an abstract academic exercise. The CPD programme he designed in April 2025 is the applied, real-world product of precisely these outcomes. It was constructed using the frameworks, the mentorship architecture, and the evidence-based professional development logic that the NQF Level 8 assessment criteria require. The Applicant did not come up with a training idea. He applied a formally assessed, nationally registered, NQF Level 8 competency framework to a real departmental need.

3.2 90% Distinction at NQF Level 8: This Is Not a Passing Grade — It Is a Mark of Mastery

The Applicant achieved a 90% Distinction in Professional Development. In the context of a postgraduate qualification at NQF Level 8 — the highest sub-degree level in the South African framework — a distinction at this level does not describe a student who understood the subject. It describes a practitioner who demonstrated mastery: the ability to synthesise, critically evaluate, and apply the principles of professional development at the ceiling of the national framework.

A 90% Distinction in Professional Development at NQF Level 8 means the following:

1. The Applicant is formally credentialled — at the highest sub-degree level in South Africa — to design, structure, and implement professional development programmes for nurses.
2. That competency was externally assessed, moderated, and confirmed by CPUT against the national standard registered by SAQA.
3. Any suggestion by the Respondents that the Applicant's CPD programme was unauthorised, informal, or amateur is contradicted by the national qualification framework of the Republic of South Africa.
4. When management silently implemented the Applicant's CPD programme in October 2025 without attribution, they did not merely infringe his copyright. They extracted the product of his highest-performing academic competency — a competency they could not replicate and for which he held the only formal credential in the room.



3.3 The Paradox of Appropriation: Management Confirmed the Programme's Value by Stealing It

The chronology of the CPD programme is a forensic admission. The Applicant designed and submitted the programme in April 2025. Management rejected it. In October 2025 — without informing the Applicant, without attribution, and without authorisation — management implemented it across the department.

This sequence is unambiguous. The decision to implement rather than further reject the programme is an implicit concession that it was professionally sound, strategically appropriate, and organisationally valuable. The decision to do so without attribution is an attempt to deny the Applicant the professional recognition that his NQF Level 8 qualification, and specifically his 90% Distinction in Professional Development, entitled him to receive.

The Respondents cannot simultaneously argue that the Applicant's CPD programme lacked professional legitimacy, and benefit from its departmental implementation. They made their position clear in October 2025 when they deployed it organisation-wide. Having confirmed its value through adoption, they are required to account for the appropriation — and for the deliberate erasure of a credentialled specialist's intellectual contribution.

4. THE PRIORITY 1 RISK REPORT: AN ACT OF STATUTORY COMPLIANCE, NOT INSUBORDINATION

4.1 The Qualification Mandated the Report

ELO 2 of SAQA ID 118316 expressly requires the holder to identify and apply legislative and regulatory requirements pertaining to healthcare management and to advocate for safe and quality care. ELO 5 requires the application of evidence-based solutions grounded in management processes. ELO 6 requires the holder to draw a strategic plan for managing risks and adverse events in a healthcare service and to develop a quality assurance programme on a risk identified in a unit.

When the Applicant identified the Gexus-DMS integration failure — a Priority 1 clinical governance risk generating clinically incorrect non-adherence letters to elderly GEMS beneficiaries — and delivered a formal Priority 1 Material Risk Report with 38 attachments to the board on 9 December 2025, he was not acting outside his employment mandate. He was fulfilling it at the highest level of professional competency his qualification required.



4.2 The 90% Distinction Explains Why He Saw What Others Did Not

Risk identification at the systems level — the ability to trace an architectural integration flaw across two enterprise systems and construct a Root Cause Analysis that demonstrates its structural, recurring nature — is not a generalist skill. It is a specialist competency. It is precisely the kind of systems thinking that ELO 6 and ELO 7 at NQF Level 8 are designed to produce.

The Applicant did not stumble across a technical anomaly. He applied the strategic management framework of his postgraduate qualification to a pattern of clinical data errors, identified the architectural gap, conducted a Root Cause Analysis, quantified the risk to 1.9 million GEMS beneficiaries, and delivered a structured, evidenced report within the governance chain. This is not what a disgruntled employee does. This is what a formally trained, nationally credentialed, 90% distinction-holding NQF Level 8 health services management specialist does when he encounters a Priority 1 patient safety risk.

The Respondents did not hire a clerk who overstepped. They hired a specialist whose qualification mandated exactly this response to exactly this kind of risk. When they suppressed the report (Monwabisi Kula, 18 December 2025), they were not managing an insubordinate employee. They were suppressing the output of the NQF framework — the product of a national qualification designed by the South African Nursing Council, registered by SAQA, and enacted under the authority of Parliament.

5. THE RESPONDENTS' CONDUCT AS INSTITUTIONAL DISREPUTE

5.1 Disrepute of SAQA and the NQF Act

SAQA exists to ensure NQF qualifications serve the public interest. The NQF Act's object includes the full personal development of each learner and the social and economic development of the nation. These objects are rendered meaningless if the holder of an NQF Level 8 qualification is systematically punished for acting in accordance with its Exit Level Outcomes.

The Respondents hired the Applicant at NQF Level 8. They derived the corporate benefit of that qualification — the risk identification capability, the governance expertise, the clinical management oversight — for as long as it suited them. When the Applicant exercised that expertise in a direction that threatened the Respondents' interests, they characterised his fulfilment of the NQF mandate as insubordination, suppressed his report, deducted 40.47% of his salary without consent, misappropriated his CPD programme, and constructively dismissed him.

The message transmitted by this conduct to every NQF Level 8 specialist in the South African health sector is not ambiguous: your qualification is welcome when it produces value for your employer, and weaponised against you when it produces accountability. This is not a private employment dispute. It is a direct assault on the credibility of the national qualifications system.

5.2 Disrepute of the Protected Disclosures Act 26 of 2000

The PDA exists to protect employees who, in good faith, disclose information they reasonably believe relates to impropriety in an employer's conduct. The Applicant's Priority 1 Material Risk Report disclosed a documented, Root-Cause-Analysis-confirmed clinical governance failure affecting vulnerable beneficiaries. It was delivered through the prescribed governance channel. It was filed with 38 supporting attachments. It is the textbook protected disclosure the PDA was enacted to shelter.



The Respondents responded with a salary deduction, a hostile work environment, the suppression of the report, and the appropriation of the Applicant's intellectual property. Eleven regulatory bodies were subsequently approached. Nine failed, deflected, or refused to engage. The Labour Court portal obstructed access to filing for eight days across seven attempted submissions. SAPS declined to open a criminal docket for documented statutory offences.

If the PDA means that a formally diagnosed, NQF Level 8 specialist who files a clinically documented, architecturally confirmed Priority 1 patient safety report through the prescribed governance chain is entitled to be evicted, deregistered, stripped of his retirement savings, and left without running water while self-representing across nine concurrent regulatory proceedings — then the PDA does not protect anyone. It creates a theoretical right while guaranteeing a practical catastrophe for anyone courageous enough to exercise it.

5.3 Disrepute of the South African Nursing Council and the National Health Framework

SAQA Qualification ID 118316 was specifically mandated by the South African Nursing Council as the national replacement for all legacy nursing management diplomas, following SANC Circular 7 of 2016. SANC designated this qualification as the benchmark for nurse management specialists from 2020 onward. The Applicant holds this qualification at distinction level. He was, by SANC's own designation, precisely the kind of specialist the South African health system needs.

The Respondents' conduct culminated in the Applicant's SANC deregistration — not for professional misconduct, but as the downstream financial consequence of an unlawful 40.47% payroll deduction. The institution that created the qualification, and the national system that depended on its holders, lost a credentialled specialist because a JSE-listed administrator chose suppression over accountability.

6. THE CONSTITUTIONAL DIMENSION

Section 195 of the Constitution prescribes accountability, transparency, and responsiveness to the needs of the public as foundational values governing public administration and entities exercising public functions. Medscheme Holdings (Pty) Ltd administers GEMS — the Government Employees Medical Scheme — and in that capacity exercises a quasi-public function affecting 1.9 million beneficiaries. The Applicant's Priority 1 Material Risk Report was, in substance, a direct exercise of these constitutional values: he identified a risk to public beneficiaries, documented it, and reported it through prescribed channels.

The Respondents' response demonstrates that a JSE-listed corporate administrator, entrusted with health administration for government employees, treats constitutional accountability obligations as a threat to be neutralised rather than a duty to be discharged. This Court is respectfully asked to treat that characterisation as the finding it is.



THE COURT IS RESPECTFULLY INVITED TO CONSIDER THE FOLLOWING:

If the outcome of this matter is that the Respondents face no meaningful consequence for suppressing a Priority 1 clinical safety report, misappropriating a formally credentialed specialist's intellectual property, and destroying that specialist through an unlawful payroll deduction and constructive dismissal — then every NQF Level 8 specialist employed in the South African health sector will have received a definitive answer to the question they dare not ask aloud:

“If you see a risk and you report it, it will be the end of you.”

That answer will not remain within the walls of AfroCentric. It will travel through every clinical risk committee, every governance meeting, every professional development programme in every health institution in this country. The NQF mandate will become a liability to be hidden rather than an obligation to be discharged. The Protected Disclosures Act will be understood — correctly, on this record — as a statute that promises protection and delivers destitution.

This Court has the power to ensure that is not the answer South Africa's health governance specialists receive. The Applicant does not ask this Court for sympathy. He asks for a determination on the law — that the NQF Act means what it says, that the PDA protects what it promises, and that a JSE-listed corporate administrator cannot employ a nationally credentialed specialist, extract the value of his qualification, suppress his risk report, steal his intellectual property, and then plead the fiction of reasonable employer conduct while he sits without running water, without income, and without professional registration.

WILBUR WILLIAM MATTHEE

Founder — EthicHawks | Forensic Governance & Compliance

RN | BTech General Nursing | PGDip Health Services Management (NQF Level 8, SAQA ID 118316)

90% Distinction in Professional Development | Cape Peninsula University of Technology

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Signed at CAPE TOWN on 8 APRIL 2026

WILBUR WILLIAM MATTHEE | Applicant / Deponent



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SOUTH AFRICAN QUALIFICATIONS AUTHORITY

REGISTERED QUALIFICATION:

Postgraduate Diploma in Health Services Management

SAQA QUAL ID		QUALIFICATION TITLE		
118316		Postgraduate Diploma in Health Services Management		
ORIGINATOR				
Cape Peninsula University of Technology				
PRIMARY OR DELEGATED QUALITY ASSURANCE FUNCTIONARY		NQF SUB-FRAMEWORK		
-		HEQSF - Higher Education Qualifications Sub-framework		
QUALIFICATION TYPE	FIELD		SUBFIELD	
Postgraduate Diploma	Field 09 - Health Sciences and Social Services		Promotive Health and Developmental Services	
ABET BAND	MINIMUM CREDITS	PRE-2009 NQF LEVEL	NQF LEVEL	QUAL CLASS
Undefined	120	Not Applicable	NQF Level 08	Regular-Provider-ELOAC
REGISTRATION STATUS		SAQA DECISION NUMBER	REGISTRATION START DATE	REGISTRATION END DATE
Reregistered		EXCO 0821/24	2021-08-03	2027-06-30
LAST DATE FOR ENROLMENT		LAST DATE FOR ACHIEVEMENT		
2028-06-30		2031-06-30		



In all of the tables in this document, both the pre-2009 NQF Level and the NQF Level is shown. In the text (purpose statements, qualification rules, etc), any references to NQF Levels are to the pre-2009 levels unless specifically stated otherwise.

This qualification does not replace any other qualification and is not replaced by any other qualification.

PURPOSE AND RATIONALE OF THE QUALIFICATION

Purpose:

This Postgraduate Diploma in Health Services management aims to produce high-quality graduates, who are able to function with advanced intellectual and practical competencies, in complex specialised areas of nursing practice as managers and leaders, in providing evidence-based care together with other team members. In addition, qualifying learners contribute to the continuous development of nursing management and healthcare services through the development of scientific and professional knowledge for the advancement of nursing practice, research, management and education. Qualifying learners will work as clinical nurse specialists in nursing management in the health sector. They are independent practitioners in their own right in the area of speciality.

This qualification provides a career path for professional nurses who want to remain in a clinical context, but who would like to specialise in nursing management and healthcare services, to add to their depth of knowledge, skills, attitudes and values (competencies). Persons learning towards this qualification become experts in their areas. The sector needs qualified nurse manager specialists, both to improve practice and to manage healthcare services in an environment of scarce resources, to address the needs of the community.

The overall purpose of the qualification is to produce:

An expert and competent professional nurse, implementing evidence-based and business practises as a strategic leader in planning, organising, managing human and financial resources, quality management, and projects within a multidisciplinary team in units of a range of settings, while complying with ethical, legal and professional frameworks to serve the health

Furthermore, the qualification aims to:

- produce high-quality nurse managers (specialists) who are able to be competent specialist nursing practitioners in a range of health service settings;
- provide nurse managers (specialists) with a range of skills, knowledge, values and attitudes that will enable them to set trends and fulfil leadership roles in a meaningful and sustained contribution to health services;
- equip nurse managers with a developed sense of equity, justice and service ethics that will ensure that they work in an accountable manner irrespective of their chosen workplace;
- offer a wide range of transferable skills for application in other professions, disciplines and general life, including:

The qualifying learners will have the:

- capacity to assess and implement health and other policies.
- ability to affect areas of policy change, development and implementation.
- ability to plan and implement and manage projects of a varied nature.
- ability to work independently and as part of a team.

Rationale:

The Postgraduate Diploma in Health Services Management is a new qualification by the South African Nursing Council (SANC) to be offered at all tertiary nursing departments in South Africa from 2020. Circular 7 of 2016 of SANC indicates the phasing out of legacy nursing qualification of all nursing education institutions, which are also all current post-basic courses. The end date of enrolment for the legacy qualification is the end of 2019.



At present the institution does not offer this specialisation in the Nursing Sciences field, and currently, the legacy Diploma in Nursing Management is only offered at Stellenbosch University in the Western Cape through the mode of distance education (Regulation 1501 of 1983). The qualification will meet the needs of the community. According to the National list of Occupations in High Demand (2018) (Government Gazette 637, 22 June 2018), nurse managers (reference 222116) in South Africa are high in command. This qualification will address the scarce skill in health services, also for the benefit of training basic nursing learners.

Health services management and leadership in South Africa is committed to meeting the Millennium Development Goals (MDGs) 3 (National Department of Health (DoH), 2010); and sustainable development goals (2030) addressing the complex burden of disease; and ensuring responsiveness to population health needs. Nurse managers have a leading role in addressing these issues, and enabling legal, policy and fiscal environment to facilitate their achievement.

Nurse managers have a long-term aim of aligning services towards the vision of a long and healthy life for all South Africans (National Department of Health). The Department of Health in South Africa has indicated its commitment towards health and nursing by focusing on important initiatives. In the National Service Delivery Agreement (2010), the South African Government has agreed on key strategic outcomes. The NSDA is a charter that reflects the commitment of key stakeholders such as nurse managers to reaching certain outcomes in the public sector. This qualification will be recognised by the South African Nursing Council (SANC), for which learners will register at SANC when they complete, for the qualification of Nursing Management/Health services management.

This qualification will be useful for:

- professional nurses who wish to progress within the field of nursing and health services management;
- persons at a commensurate level in Health and Allied Health Sciences who wish to change direction and move into nursing management,
- those wishing to progress towards a qualification such as a Master's Degree in nursing and beyond.

SANC has stipulated that it has become apparent that no single model from elsewhere in the world can adequately address the conditions and management of health services in South Africa due to the legacy of the apartheid era and political environment of the past. The population of South Africa has increased by 16% from approximately 48.687 million in 2008 (mid-year estimate) to 56.522 million in 2017 (mid-year estimate). There is a great need to have a more community-based qualification in the Western Cape Region so that engagement is facilitated. Therefore, any route such as the proposed qualification, that can be developed that recognises continuing professional development through experiences in the workplace will contribute to South African nurses' professional development.

More than 30 000 professional nurses have graduated in the past 10 years, resulting in a disparity in educational level between the experienced senior nurses in management roles, and the junior nurses whom they manage. As a result the demand for nurse managers far exceeds the number of learner nurses that should be trained per year. This is specifically more apparent with the move to a primary healthcare approach in the healthcare system of South Africa, expecting all nurses to be cognisant of the approach. A shift in leadership and management competencies are needed in a more flexible service delivery approach combatting higher levels of patient morbidity and mortality with regards to communicable and non-communicable diseases.

LEARNING ASSUMED TO BE IN PLACE AND RECOGNITION OF PRIOR LEARNING

Recognition of Prior Learning (RPL):

Recognition of Prior Learning (RPL) involves the identification, mediation, assessment and acknowledgement of knowledge and skills obtained through informal, non-formal and/ or formal learning. The RPL process is multi-dimensional and multi-contextual in nature, aimed at the individual needs of applicants and is handled in accordance with an institutional RPL

policy by a unit dedicated to this activity.

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RPL in this qualification will relate to gaining access to the qualification and/or credits/advanced standing as described in institutional guidelines. Learners may provide evidence of prior learning to comply to the requirements of the qualification. Through RPL they may receive credit towards the qualification by means of portfolios.

Learners who have worked as nurse managers in health services will be able to apply for credits. The applicants will be required to submit a portfolio and highlight how outcomes of the subjects for which credits are applied for have been met.

Learners who do not meet entry requirements but have 5 years' experience as managers in health care institutions, will complete a portfolio of evidence. The portfolio will provide evidence that the applicants have achieved the relevant outcomes through their work experience. The portfolio of evidence will then be assessed by the relevant academic staff member who will then make a recommendation to the RPL Committee in the Faculty.

If a learner wants to register as a nursing administrator the South African Nursing Council (SANC) based on previous experience, as a nurse manager. The learner will submit an application to SANC, who will evaluate the portfolio and quality of the process and register the student accordingly.

Entry Requirements:

The minimum entry requirement for this qualification is:

- Bachelor of Nursing Science, NQF Level 7.

RECOGNISE PREVIOUS LEARNING?

Y



QUALIFICATION RULES

This qualification consists of the following compulsory modules at National Qualifications Framework Level 8 totalling 120 Credits.

Compulsory Modules, Level 8, 120 Credits:

- Fundamentals in research, 8 Credits.
- Professional practice, 8 Credits.
- Research methodology, 8 Credits.
- Professional development, 16 Credits.
- Management and leadership, 16 Credits.
- Advanced management skills, 16 Credits.
- Strategic thinking and planning, 16 Credits.
- Research project in health service management, 16 Credits.
- Budgetary governance and cost containment, 16 Credits.

EXIT LEVEL OUTCOMES

1. Apply knowledge of basic research methodology in the appraisal of research articles in health services management.
2. Demonstrate accountability in managing and evaluating specialized nursing services in a variety of settings for individuals, families and communities at district, provincial and national level within the context of national health policy and international guidelines.
3. Analyse the appropriateness of a range of research methods and approaches for investigating health services management research problems to present findings comprehensive findings and ideas to members of the healthcare team and public.
4. Demonstrate a coherent and critical understanding of the principles and theories of management and leadership to identify and critique own learning strategies which effectively address his or her professional and ongoing learning needs and the professional and ongoing learning needs of others.
5. Demonstrate a comprehensive, critical and systematic knowledge and understanding of the components of the management process in the management of a health service, with a coherent and critical understanding of applying the principles and theories of management and leadership in managing health services.
6. Function effectively as a nurse manager/health service manager with other members of the health team within the healthcare system while critically reviewing and evaluating complex issues, using evidence-based solutions, strategic planning and theory driven arguments.
7. Formulate and implement an effective strategic plan for a defined healthcare service by taking cognisance of internal strengths and weaknesses and external threats and opportunities, as well as the roles and responsibilities of different components and elements to ensure a quality healthcare delivery system.
8. Demonstrate coherent and in-depth understanding of principles of budgetary governance in a health service.

ASSOCIATED ASSESSMENT CRITERIA

Associated Assessment Criteria for Exit Level Outcome 1:

- Appraise research articles in the speciality field of nursing through the application of basic knowledge of research methodology.

- Differentiate the qualitative, quantitative and mixed methods research design.
- Apply standards of the research process in appraising research articles.

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Associated Assessment Criteria for Exit Level Outcome 2:

- Applicable legislative and regulatory requirements pertaining to healthcare management are identified and debated.
- Professional misconduct is managed according to institutional and legal protocol.
- Decision-making and moral reasoning models are used to resolve ethical dilemmas within health system management.
- Advocacy principles are used to advocate for safe and quality care.

Associated Assessment Criteria for Exit Level Outcome 3:

- Evaluate standard guidelines used in the field of nursing services management by applying knowledge of the systematic review and guideline development process.
- Demonstrate academic writing skills in academic writing in nursing services management.
- Use the appropriate format such as PICO in formulating research questions.
- Apply search strategies such as Boolean in conducting a search in different databases in nursing services management.
- Evaluate speciality based guidelines using standardized evaluation tools.

Associated Assessment Criteria for Exit Level Outcome 4:

- Demonstrate mindfulness of the principles and theories of management and leadership.
- Debate the key concepts of self-responsibility, self-ownership, self-leadership and self-development.
- Assess the degree of self-ownership in developing professionally to stay a competent practitioner.
- Develop a framework to demonstrate the process of self-development in the professional career.
- Draw and apply self-mentoring strategies to develop oneself.
- Create a plan to establish academic/professional relationships to sustain lifelong learning amongst nurses.
- Use performance appraisal principles to monitor own development within the context of the programme
- Distinguish between the concepts 'mentor' and 'mentee'.
- Develop a policy for the unit/formulation in a health and/ or midwifery service, regarding a mentoring contract.
- Debate the purpose of mentorship in the nursing units.
- Applying a model for implementing mentorship in a unit for novices.
- Create a positive learning climate by establishing positive relationships.



Associated Assessment Criteria for Exit Level Outcome 5:

- Demonstrate a comprehensive understanding of the components of the management processes of planning, organizing, staffing, directing and control.
- Debate the principles and theories of management.
- Outline the differences between general management and corporate governance.
- Implement supervisory models to control the quality of management and nursing care.
- The management of a healthcare system reflects the application of the management process.
- Evidence-based practice indicates that work-related behaviour is grounded in the management processes.
- Debate the principles and theories of leadership.
- Debate the difference between management and leadership.
- Compare and practice different and collaborative leadership styles.
- Demonstrate team functioning and team leadership with available best evidence on collaborative group work.
- Develop a plan to empower nursing staff as leaders.
- Differentiate between motivational strategies for staff satisfaction.

Associated Assessment Criteria for Exit Level Outcome 6:

- Resolve conflict situations between colleagues, peers and patients in a multi-disciplinary team environment.
- Critically evaluate the principles, theories and models of communication in a multi-disciplinary team.
- Apply action learning and planning strategies to monitor innovations.
- Demonstrate the change process by comparing various change models.
- Debate the differences between risks, advert events, incidences.
- Draw a strategic plan for managing risks and advert events in a healthcare service
- Develop a quality assurance programme on a risk identified in a unit, eg. poor record keeping.
- Distinguish between the different steps and processes in quality assurance.
- Analyse, resolve a labour matter according to labour relations principles in a legal environment

Associated Assessment Criteria for Exit Level Outcome 7:

- Differentiate between strategic thinking and strategic planning.
- Explore and debate how strategic thinking for a healthcare facility is informed by global trends in health, politics, available resources, population growth, etc.
- Demonstrate how a comprehensive stakeholder analysis is done and visually presented.
- Perform a situational analysis by means of a standardized strategy e.g. SWOT analysis, TOWS matrix etc.
- Develop a strategic plan for a nurse manager in a healthcare facility according to the strategic planning steps.
- Evaluate and debate the implementation and execution of the strategic plan in a healthcare facility
- Demonstrate coherent and in-depth understanding of principles of business management pertaining to the private or public health sector.
- Apply quality control criteria to improve the patient and staff satisfaction of a defined service.

Associated Assessment Criteria for Exit Level Outcome 8:

- Debate the different types of budget and allocation.
- Compare and debate of proportion of national budget allocated to healthcare with other countries.
- Outline the steps of the budget process to be followed in being successful in allocation.
- Debate budget distribution down to your healthcare facility.
- Motivate for the successful implementation of budgets in a cost-effective environment.
- Differentiate and debate the stages of the budget execution cycle.
- Apply budget compliance control measures to evaluate institutional practice in a unit.
- Demonstrate the monitoring and evaluation of budgetary governance measures.
- Explore and debate issues related to the management and monitoring of budget execution.
- Outline the process and principles of cost containment and control in healthcare service.
- Establishing accuracy through auditing and procurement processes as control measures.
- Debate the principles of sound financial practices within an ethical legal framework.

INTERNATIONAL COMPARABILITY

The international comparison revealed that nurse managers who obtained this qualification are highly employable globally. Countries regarded as leaders, such as New Zealand and Canada, do not provide the comprehensive training that South Africa has done for Professional Nurses followed by this qualification.

Country: Namibia

Institution: International University of Namibia

A recent visit to the Department of Nursing at the International University of Namibia shed light on their offering of their post basic qualification. Their qualification is not focused on primary healthcare and places learners only in private settings with a minimal exposure to different public facilities.

The content of the new qualification is also more relevant to the current scenario of health services in solving critical issues and challenges of health service management and quality assurance in healthcare.



Country: Dublin

Institution: RCSI University of Medicine and Health Sciences

Qualification Title: Bachelor of Science in Nursing Management

This is a bridging programme for nurses/midwives who would like to further their learning with an honours level qualification. This is a part-time, one-year qualification that is aimed at nurses following a management career pathway and employed in managerial positions. The specific modules in this qualification allow learners to explore key managerial issues in relation to health financial management; human resource management; patient safety, quality improvement and risk management; and leadership and management. These outcomes are in line with those of the new qualification. Similar to this, qualification, to be eligible for the RCSI BSc in Nursing Management qualification, applicants must be licensed with the relevant regulatory body and educated to certificate or diploma level, demonstrated through a defined recognition of prior learning (RPL) process.

Country: Australia

Institution: The University of Southern Queensland

Qualification Title: Graduate Diploma of Nursing Specialisation Leadership & Management in Health (GDNG)

The entry requirements are a 3-year bachelor degree in the area of nursing or equivalent and the learner must hold a professional registration with the Australian Health Practitioner Regulation Agency (AHPRA) or relevant regulatory body in the applicant's home country, which is similar to the new qualification. After completing the Graduate Diploma of Nursing, learners are eligible to take credit from these courses and continue to complete the Master of Nursing as is possible at CPUT. Learners may also apply for recognition of prior learning for previous studies or have relevant work experience.

This qualification consists of two core modules and six specialist modules while the new qualification has three core modules and six specialist modules. The Australian qualification includes outcomes related to leadership and management in nursing including human resource management and is offered part-time, as with the new qualification.

In the United States of America, similar postgraduate diplomas are offered as Master of Science in Nursing Administration qualifications. For example, Indiana Wesleyan University-Marion, offers the Master of Science in Nursing-Nursing Administration for professional nurses. Similar to this new qualification, professional nurses as learners must be licensed with the regulatory body to attend the Indiana qualification. This is a post basic-qualification similar to the level of the South African qualification. The curriculum is delivered online with practicum experiences. The curriculum ascribes to the American Association of Colleges of Nursing's and The Essentials of Master's Education in Nursing and the competencies prescribed by the respective professional specialty organizations. Learners in the Nursing Administration qualification must complete a minimum of 245 hours of onsite supervised practicum. Although the new qualification has more Work integrated learning of 500 hours, both qualifications have a similar focus with regard to the work integrated learning.

ARTICULATION OPTIONS

This qualification allows possibilities for both vertical and horizontal articulation.

Horizontal Articulation:

- Postgraduate Diploma in Primary Care Nursing, NQF level 8
- Postgraduate Diploma in Occupational Health Nursing, NQF Level 8

Vertical Articulation:

- Master of Nursing in Nursing in Research, NQF Level 9

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MODERATION OPTIONS

N/A

CRITERIA FOR THE REGISTRATION OF ASSESSORS

N/A

NOTES

N/A

LEARNING PROGRAMMES RECORDED AGAINST THIS QUALIFICATION:

NONE

PROVIDERS CURRENTLY ACCREDITED TO OFFER THIS QUALIFICATION:

This information shows the current accreditations (i.e. those not past their accreditation end dates), and is the most complete record available to SAQA as of today. Some Primary or Delegated Quality Assurance Functionaries have a lag in their recording systems for provider accreditation, in turn leading to a lag in notifying SAQA of all the providers that they have accredited to offer qualifications and unit standards, as well as any extensions to accreditation end dates. The relevant Primary or Delegated Quality Assurance Functionary should be notified if a record appears to be missing from here.



NONE

All qualifications and part qualifications registered on the National Qualifications Framework are public property. Thus the only payment that can be made for them is for service and reproduction. It is illegal to sell this material for profit. If the material is reproduced or quoted, the South African Qualifications Authority (SAQA) should be acknowledged as the source.

ANNEXURE

“PDA BILL”

Protected Disclosures Amendment Bill, 2026 (Government Gazette, 8 April 2026)

Date: 8 April 2026

Relevance: *Preamble*



Matthee v AfroCentric Health (RF) (Pty) Ltd & Medscheme Holdings (Pty) Ltd
Statement of Claim — 9 April 2026 | EthicHawks Forensic Governance Consultancy

INVITATION FOR PUBLIC COMMENTS PROTECTED DISCLOSURES BILL, 2026

1. The Department of Justice and Constitutional Development invites interested parties to submit written comments on the proposed Protected Disclosures Bill, 2026 (the Bill).

2.1 The legal environment has a major influence on an individual's decision to make a disclosure regarding any improper conduct of an employer or employee (publicly known as 'blowing the whistle'). Protection for disclosers (who are referred to as 'whistleblowers' in the public domain) is provided by the Protected Disclosures Act, 2000 (Act No. 26 of 2000) (the PDA). The PDA was primarily enacted to provide for procedures in terms of which employees in both the private and the public sectors may disclose information regarding unlawful or irregular conduct by their employers or other employees in the employ of their employers and most importantly to provide protection to employees who make a disclosure.

2.2 It was found in the Judicial Commission of Inquiry into Allegations of State Capture, Corruption and Fraud into the Public Sector Including Organs of State (the Zondo Commission), that the PDA does not provide a clear-cut procedure for a discloser to follow, and it does not sufficiently guarantee that the disclosures will be protected. Furthermore, it is not pro-active in providing physical protection, it offers no incentives to the discloser, and it does not ensure that all such information finds its way to a destination with specialised skills in receiving, investigating, and utilising such information effectively.

2.3 The National Anti-Corruption Advisory Council (NACAC) in its final report to the President in August 2025 recommends, *inter alia*, on the strategic and systemic implementation of the Zondo Commission report, including acknowledging and supporting disclosers, preventing retaliation, and exploring mechanisms for incentivised disclosures.

2.4 A draft Bill was prepared taking into consideration the Zondo Commission's recommendations and the NACAC report, guidelines from international instruments and foreign jurisdictions.

3. The comments on the Bill must be submitted to Ms A van der Walt, on or before **14 May 2026**. The contact details are:

- (a) Postal address:
The Director-General: Justice and Constitutional Development
Private Bag X 81
Pretoria
0001;
- (b) e-mail address: whistleblowingreforms@justice.gov.za

4. Further information can be obtained from Ms A van der Walt at alvanderwalt@justice.gov.za.



REPUBLIC OF SOUTH AFRICA

PROTECTED DISCLOSURES BILL

*(As introduced in the National Assembly (proposed section 75); explanatory
summary of Bill published in Government Gazette No. of 2026)
(The English text is the official text of the Bill)*

(MINISTER OF JUSTICE AND CONSTITUTIONAL DEVELOPMENT)

[B—2026]



BILL

To provide for procedures in terms of which persons may disclose information regarding improper conduct in the public or private sectors; to provide for the conducting of investigations regarding a disclosure made; to provide for the protection of persons who make disclosures which are protected in terms of this Act; to provide for the award payable to disclosers; to provide for certain remedies in connection with any occupational detriment or detrimental action suffered by a person on account of having made a protected disclosure; and to provide for matters connected therewith.

PREAMBLE

RECOGNISING THAT—

- the Bill of Rights in the Constitution of the Republic of South Africa, 1996, enshrines the rights of all people in the Republic and affirms the democratic values of human dignity, equality and freedom;
- section 8 of the Bill of Rights provides for the horizontal application of the rights in the Bill of Rights, considering the nature of the right and the nature of any duty imposed by the right;
- criminal and other irregular conduct in the public and private sectors are detrimental to good, effective, accountable and transparent governance and can endanger the economic stability of the Republic and have the potential to cause social harm;

AND BEARING IN MIND THAT—

- neither the South African common law nor statutory law makes provision for mechanisms or procedures in terms of which employees may, without fear of reprisals, disclose information relating to suspected or alleged criminal or other irregular conduct by their employers, whether in the private or the public sector;
- every employer and employee has a responsibility to disclose criminal and any other irregular conduct in the workplace;
- every employer has a responsibility to take all necessary steps to ensure that employees who disclose such information are protected from any reprisals because of such disclosure;

AND IN ORDER TO—

- create a culture which will facilitate the disclosure of information by employees relating to criminal and other irregular conduct in the workplace in a responsible manner by providing comprehensive statutory guidelines for the disclosure of such information and protection against any reprisals because of such disclosures; and
- promote the eradication of criminal and other irregular conduct in the public and private sectors,

BE IT THEREFORE ENACTED by the Parliament of the Republic of South Africa as follows:—

ARRANGEMENT OF SECTIONS

CHAPTER 1

GENERAL PROVISIONS

1. Definitions



2. Objects and application of Act
3. Central database

CHAPTER 2 PROTECTED DISCLOSURES

Part 1

General provisions

4. Protected disclosures
5. Disclosures not protected
6. Protected disclosure to employer
7. Protected disclosure to legal practitioner or legal adviser
8. Protected disclosure to member of Cabinet, Executive Council or Municipal Council
9. Protected disclosure to certain persons or bodies
10. General protected disclosure

Part 2

Making a disclosure

11. Making disclosure to authorised person other than employer of discloser
12. Making of disclosure to employer of discloser
13. Disclosure of false information

CHAPTER 3 DEALING WITH PROTECTED DISCLOSURE

Part 1

Investigation

14. Dealing with protected disclosure
15. Application for court assistance

Part 2

Finalisation of investigation

16. Finalisation of investigation
17. Enforcement of report
18. Award to discloser

CHAPTER 4 PROTECTION OF PERSON MAKING PROTECTED DISCLOSURE

19. Confidentiality
20. Exclusion of civil and criminal liability
21. Protection from occupational detriment or detrimental action
22. Witness protection for discloser
23. Legal assistance

CHAPTER 5 COMPLAINTS MECHANISM

24. Complaints mechanism
25. Dealing with complaint
26. Finalisation of complaint

CHAPTER 6 MISCELLANEOUS

27. Revocation of protection
28. Remedies
29. Education and information
30. Regulations
31. Transitional provisions and saving



- 32. Amendment and Repeal of Laws
- 33. Short title and commencement
- Schedule

LAWS REPEALED OR AMENDED BY SECTION 32

CHAPTER 1 GENERAL PROVISIONS

Definitions

1. In this Act, unless the context otherwise indicates—
- 'authorised person'** means a person to whom a protected disclosure can be made as contemplated in sections 6, 7, 8, 9 and 10;
- 'Department'** means the Department of Justice and Constitutional Development;
- 'designated officer'** means a person designated by an employer to receive and act upon a disclosure made in terms of this Act;
- 'detrimental action'** means an act or omission that—
- (a) results in unfair discrimination;
 - (b) threatens or violates the legal rights of a discloser or a related person; or
 - (c) amounts to intimidation, harassment or causes personal harm or injury, or loss of damage to property or livelihood;
- 'Director-General'** means the Director-General of the Department of Justice and Constitutional Development and includes a person delegated by them;
- 'discloser'** means an employee in the public or the private sector or any other person who is not an employee, who makes a disclosure;
- 'disclosure'** means the disclosing of information regarding improper conduct with regards to an employer or employee, in the public or private sector;
- 'employee'** means—
- (a) any person who works or worked in the public or private sector and who receives or received, or is entitled to receive, any remuneration, including, but not limited, employees or investigators appointed in terms of the—
 - (i) South African Police Service Act, 1995 (Act No. 68 of 1995);
 - (ii) Special Investigating Units and Special Tribunals Act, 1996 (Act No 74 of 1996)
 - (iii) National Prosecuting Authority Act, 1998 (Act No. 32 of 1998);
 - (iv) Private Security Industry Regulation Act, 2001 (Act No. 56 of 2001);
 - (v) Defence Act, 2002 (Act No. 42 of 2002);
 - (vi) Intelligence Services Act, 2002 (Act No. 65 of 2002);
 - (vii) Independent Police Investigative Directorate Act, 2011 (Act No. 1 of 2011); and
 - (viii) any other organ of state authorised by law to investigate corruption or fraud, whether in a full time, part time, contractual or temporary capacity; or
 - (b) any other person who in any manner assists or assisted in carrying on or conducting or conducted the business of an employer, client of the employer, person acting on behalf of or on the authority of such employer, including volunteers and trainees;
 - (c) any other person who in any manner assists or assisted in carrying on or conducting or conducted the business of an employer, client of the employer, person acting on behalf of or on the authority of such employer as an independent contractor, consultant or agent; or



- (d) any person who renders or rendered services to an employer, client of the employer, person acting on behalf of or on the authority of such employer while being employed by a temporary employment service;

'employer' means a person contemplated in section 6;

'improper conduct' means—

- (a) any unlawful activity as contemplated in the Prevention of Organised Crime Act, 1998 (Act No. 121 of 1998);
- (b) the offence of corruption and corrupt activities as contemplated in Chapter 2 of the Prevention and Combating of Corrupt Activities Act, 2004 (Act No. 12 of 2004);
- (c) a criminal offence that has been committed, is being committed or is likely to be committed;
- (d) that a person has failed, is failing or is likely to fail to comply with any legal obligation to which that person is subject;
- (e) that a miscarriage of justice has occurred, is occurring or is likely to occur;
- (f) that the health or safety of an individual has been, is being or is likely to be endangered;
- (g) that the environment has been, is being or is likely to be damaged;
- (h) unfair discrimination as contemplated in Chapter II of the Employment Equity Act, 1998 (Act No. 55 of 1998), or the Promotion of Equality and Prevention of Unfair Discrimination Act, 2000 (Act No. 4 of 2000); or
- (i) that any matter referred to in this section has been, is being or is likely to be deliberately concealed,

irrespective of whether—

- (i) the improper conduct occurs or occurred in the Republic of South Africa or elsewhere; or
- (ii) the law applying to the improper conduct is that of the Republic of South Africa or of another country;

'Minister' means the Cabinet member responsible for the administration of justice;

'Labour Relations Act' means the Labour Relations Act, 1995 (Act No. 66 of 1995);

'law enforcement agency' means the—

- (a) Directorate for Priority Crime Investigation, established by section 17C of the South African Police Service Act, 1995 (Act No. 68 of 1995);
- (b) Special Investigating Units and Special Tribunals, established by section 2 of the Special Investigating Units and Special Tribunals Act, 1996 (Act No 74 of 1996)
- (c) Investigating Directorate against Corruption established by section 7(1A) of the National Prosecuting Authority Act, 1998 (Act No. 32 of 1998);
- (d) Independent Police Investigative Directorate established by section 3 of the Independent Police Investigative Directorate Act, 2011 (Act No. 1 of 2011);
- (e) South African Police Service; or
- (f) any other organ of state authorised by law to investigate corruption or fraud;

'occupational detriment' means, but is not limited to, being—

- (a) subjected to any disciplinary action;
- (b) dismissed, suspended, demoted, harassed, intimidated or subjected to emotional and psychological trauma;
- (c) transferred against their will;
- (d) refused transfer or promotion;
- (e) subjected to a term or condition of employment or retirement which is altered or kept altered to their disadvantage;
- (f) refused a reference, or being provided with an adverse reference, from their employer;



- (g) denied appointment to any employment, profession or office;
- (h) subjected to any civil claim for the alleged breach of a duty of confidentiality or a confidentiality agreement;
- (i) threatened with any of these actions referred to in paragraphs (a) to (h) above; or
- (j) otherwise adversely affected in respect of their employment, profession or office, including employment opportunities, work security and the retention or acquisition of contracts to perform work or render services;

'prescribed' means prescribed by regulation;

'private sector' means the private sector as defined in the Prevention and Combating of Corrupt Activities Act, 2004;

'protected disclosure' means a disclosure contemplated in section 4;

'public body' means a public body as defined in the Prevention and Combating of Corrupt Activities Act, 2004;

'related person' means any member of the family or household of a discloser, or any other person in a close relationship to, or associated with the discloser, including a person who assists the discloser in making a disclosure or a colleague of the discloser;

'temporary employment service' means any person who, as a reward, procures for, or provides to a client, another person—

- (a) who render services to, or perform work for the client; and
- (b) who are remunerated by the temporary employment service; and

'this Act' includes regulations made under this Act.



Objects and application of Act

2. (1) The objects of this Act are to—

- (a) provide for procedures in terms of which a discloser can disclose information regarding improper conduct in the public or private sector by an employer or employee;
- (b) protect a discloser and related person from being subjected to any occupational detriment or detrimental action on account of having made a protected disclosure;
- (c) provide, under certain circumstances, for the payment of an award to the discloser;
- (d) provide for the management and investigation of disclosures; and
- (e) provide for a complaints mechanism and certain remedies in connection with occupational detriment or detrimental action suffered on account of having made a protected disclosure.

(2) This Act applies to a protected disclosure made after the date on which this section comes into operation, irrespective of whether the improper conduct occurred before or after the commencement of this section.

Central database

3. (1) The Director-General must—

- (a) ensure the development and maintenance of an electronic central database for disclosures, containing information contemplated in subsection (2)(b);
- (b) designate one or more employees of the Department to oversee the maintenance and operation of the database, including identifying and addressing any problems that may arise in the effective functioning of the database.

(2) The database must make provision for—

- (a) the once off registration of an authorised person;

- (b) the uploading of information regarding—
- (i) the disclosure received by the authorised person, including the receipt of a disclosure by another authorised person if the disclosure was referred to such other authorised person;
 - (ii) the date of receipt of the disclosure;
 - (iii) whether the disclosure is—
 - (aa) being investigated;
 - (bb) referred to another authorised person for investigation, and if so the date of the referral and the authorised person to whom the disclosure is referred to;
 - (cc) dismissed and reason for dismissal; and
 - (dd) finalised including the date of finalisation.
- (3) An authorised person may designate a person or persons who would be responsible for—
- (a) registering the authorised person on the database in the prescribed manner;
 - (b) upload the information contemplated in subsection (2)(b) onto the database; and
 - (c) informing the employee or employees contemplated in subsection (1)(b) of any problems experienced in the uploading of information on the database, if any.
- (4) If no designation is made in terms of subsection (3) the authorised person must register on the database in their personal capacity in the prescribed manner.
- (5) No personal information of a discloser, as contemplated in the Protection of Personal Information Act, 2013 (Act No. 4 of 2013) may be uploaded onto the database.
- (6) The database may only be accessible to—
- (a) an authorised person registered onto the database; and
 - (b) a person designated in terms of subsection (3), to the extent necessary to perform the functions contemplated in that subsection.
- (7) The Director-General must include the information contemplated in subsection (2)(b) in the Department's Annual Report.



CHAPTER 2 PROTECTED DISCLOSURES

Part 1 *General provisions*

Protected disclosures

4. A disclosure is a protected disclosure if it is made to—
- (a) an employer as contemplated in section 6;
 - (b) a legal practitioner or legal adviser as contemplated in section 7;
 - (c) a member of the Cabinet or of the Executive Council of a province or a Municipal Council as contemplated in section 8;
 - (d) a person or body as contemplated in section 9; or
 - (e) any person, body or institution to whom a disclosure can be made as contemplated in section 10.

Disclosures not protected

5. A disclosure is not a protected disclosure if—

- (a) it is not made to a person as contemplated in section 4;
- (b) subject to section 20, the making of that disclosure constitutes a criminal offence;
- (c) it is made by a legal practitioner or legal adviser to whom the information concerned was disclosed in the course of obtaining legal advice as contemplated in section 7;
- (d) it is made by the discloser knowing, or ought reasonably to have known, that the information disclosed is false;
- (e) it is made with the intention to cause harm to the affected party and where the affected party has suffered harm as a result of the disclosure;
- (f) it is made for pecuniary gain excluding any rewards or other incentives provided for in this Act;
- (g) it is made for an illegal purpose; or
- (h) it is made solely or substantially with the motive to avoid dismissal or other disciplinary action.

Protected disclosure to employer

6. A disclosure contemplated in section 4(a) is a disclosure made to a person in the public or private sector who—

- (a) employs or provides work for another person and who remunerates or expressly or tacitly undertakes to remunerate that other person; or
- (b) permits another person in any manner to assist in the carrying on or conducting of their business,

and includes any client of the employer, or any person acting on behalf of or on the authority of that employer.

Protected disclosure to legal practitioner or legal adviser

7. A disclosure contemplated in section 4(b) is a disclosure made—

- (a) to a legal practitioner as defined in the Legal Practice Act, 2014 (Act No. 28 of 2014); or
- (b) to a person whose occupation involves the giving of legal advice, with the object of and while obtaining legal advice.

Protected disclosure to member of Cabinet, Executive Council or Municipal Council

8. A disclosure contemplated in section 4(c) is a disclosure made to a member of Cabinet, the Executive Council of a province or a Municipal Council, if the discloser's employer is—

- (a) an individual appointed in terms of legislation by a member of Cabinet, of the Executive Council of a province or a Municipal Council;
- (b) a body whose members are appointed in terms of legislation by a member of Cabinet or of the Executive Council of a province or a Municipal Council; or
- (c) a public body falling within the area of responsibility of the member concerned.

Protected disclosure to certain persons or bodies

9. A disclosure contemplated in section 4(d) is a disclosure made to—

- (a) the Public Protector;



- (b) the South African Human Rights Commission;
- (c) the Commission for Gender Equality;
- (d) the Commission for the Promotion and Protection of the Rights of Cultural, Religious and Linguistic Communities;
- (e) the Public Service Commission;
- (f) the Auditor-General;
- (g) a person appointed or designated, or a body established in terms of national legislation; or
- (h) a person or body prescribed for purposes of this section in respect of which the person concerned reasonably believes that the relevant improper conduct falls within any description of matters which, in the ordinary course, are dealt with by the person or body concerned.

Disclosure made to other person, body or institution regarded as protected disclosure



10. (1) A discloser who makes a disclosure contemplated in section 4(e) must—

- (a) reasonably believe the information disclosed and any allegation contained in the disclosure, is substantially true; and
- (b) not make the disclosure for purposes of personal gain, excluding any reward payable in terms of any law.

(2) The disclosure referred to in subsection (1) must meet one or more of the following conditions:

- (a) At the time of making the disclosure, the discloser reasonably believes that they would suffer an occupational detriment if the information were disclosed the information to their employer in accordance with section 6;
- (b) that the discloser making the disclosure has reason to believe that it is likely that evidence relating to the improper conduct is likely to be concealed or destroyed;
- (c) that the discloser has previously made a disclosure of substantially the same information to an authorised person, and no action was taken within a reasonable period after the disclosure; or
- (d) that the improper conduct is of an exceptionally serious nature, including but not limited to—
 - (i) an imminent risk of a substantial and specific danger to the life, health or safety of a person;
 - (ii) an imminent danger to the public interest;
 - (iii) a risk of irreversible damage to any property; or
 - (iv) the commission of an offence.

(3) A disclosure may be made under this section only if it is reasonable to make the disclosure, having regard to the factors listed in subsection (4).

(4) In determining for the purposes of subsection (3) whether it is reasonable for the person to make the disclosure, consideration must be given to—

- (a) the identity of the person to whom the disclosure is made;
- (b) the seriousness of improper conduct;
- (c) whether the improper conduct is continuing or is likely to occur in the future;
- (d) whether the disclosure is made in breach of a duty of confidentiality of the employer towards any other person;

- (e) in a case falling within subsection (2)(c), any action which the employer or the person or body to whom the disclosure was made has been taken or might reasonably be expected to have taken, because of the previous disclosure;
- (f) whether in making the disclosure to the employer, the employee complied with any procedure which was authorised by the employer; and
- (g) the public interest.

(5) For the purposes of this section, a subsequent disclosure may be regarded as a disclosure of substantially the same information referred to in subsection (2)(c) where such subsequent disclosure extends to information concerning an action taken or not taken by any person because of the previous disclosure.

Part 2

Making a disclosure

Making disclosure to authorised person other than employer of discloser



11. (1) A disclosure to an authorised person other than the employer of the discloser may be made—

- (a) in writing;
- (b) electronically; or
- (c) orally,

in the prescribed manner and containing the prescribed information: Provided that the disclosure is regarded as a protected disclosure only if it is made to an authorised person.

(2) A disclosure contemplated in subsection (1) may be made anonymously.

(3) (a) Where a discloser makes a disclosure orally, the authorised person to whom the disclosure is made must cause the disclosure to be reduced to writing and signed by the discloser.

(b) Where the discloser—

- (i) is illiterate; or
 - (ii) has a disability preventing them from making a written disclosure,
- the written disclosure referred to in paragraph (a) must be read aloud, interpreted and explained to the discloser in a language that the discloser understands.

(c) After the requirements in paragraph (b) have been met, the discloser must indicate their agreement before affixing a mark to the written disclosure.

(d) A certificate in the prescribed form confirming compliance with paragraphs (a) and (b) must be attached to the written disclosure.

(4) A provision in a contract of employment or other agreement between an employer and an employee is void in so far as it—

- (a) purports to exclude any provision of this Act, including an agreement to refrain from instituting or continuing any proceedings under this Act or any proceedings for breach of contract; or
- (b) purports to preclude the employee or has the effect of discouraging the employee, from making a disclosure.

Making of disclosure to employer of discloser

12. (1) Every employer must, subject to section 14—

- (a) develop appropriate procedures for receiving and dealing with information about improper conduct;
- (b) take reasonable steps to bring the internal procedures to the attention of every employee.

(2) A disclosure to the employer of a discloser must be made in accordance with a procedure contemplated in subsection (1), provided that the employee has been made aware of the procedure as required in terms of subsection (1)(b).

(3) Where there is no procedure as contemplated in subsection (1)(a), a disclosure may be made—

- (a) to the employer; or
- (b) to another authorised person, if the discloser is of the opinion that their disclosure will be better dealt with by such other authorised person.

(4) A discloser who, in terms of subsection (3)(b), makes a disclosure to another authorised person is, for the purposes of this Act, deemed to have made the disclosure to the employer.

(5) The procedures contemplated in subsection (1)(a) must include the name and contact details of a designated officer to whom a disclosure must be submitted and the procedure for—

- (a) receiving and dealing with disclosures, including time frames for the finalisation of the disclosure;
- (b) referring a disclosure to an appropriate body if the disclosure would be more appropriately dealt with by that body;
- (c) the compulsory referring of a disclosure to the South African Police Service if the designated officer reasonably believes that the matter to which the disclosure relates constitutes a criminal offence, or an imminent risk of substantial or specific danger to the life, health or safety of individuals or to the environment;
- (d) reviewing and investigating disclosures in accordance with Promotion of Administrative Justice Act, 2000 (Act No. 3 of 2000);
- (e) investigating other forms of improper conduct if, during the investigation of a disclosure, the designated officer has reason to believe that another form of wrongdoing has been committed or may be committed;
- (f) measures for anonymous reporting and how the discloser's identity will be kept confidential; and
- (g) informing the discloser of the outcome of the investigation.

Disclosure of false information

13. (1) A discloser who intentionally discloses false information—

- (a) knowing that information to be false or who ought to reasonably have known that the information is false; and
- (b) with the intention to cause harm to the affected party and where the affected party has suffered harm because of such disclosure,

is guilty of an offence and is liable on conviction to a fine or to imprisonment for a period not exceeding two years or to both such fine and imprisonment.

(2) The institution of a prosecution for an offence referred to in subsection (1) must be authorised in writing by the National Director of Public Prosecutions or by a person delegated by them.



CHAPTER 3
DEALING WITH PROTECTED DISCLOSURE
Part 1
Investigation

Dealing with protected disclosure

14. (1) An authorised person must, within five days of receipt of a disclosure made in terms of section 11, in writing acknowledge the receipt thereof to the discloser.

(2) A disclosure made to an authorised person or body contemplated in section 4(b) to (e) must be dealt with and investigated in the prescribed manner.

(3) An authorised person must within five days of receipt of a disclosure conduct a preliminary investigation into the disclosure and determine whether the disclosure—

- (a) is a protected disclosure as contemplated in section 4;
- (b) is a disclosure not protected as contemplated in section 5; or
- (c) falls within the ambit of subsection (5).

(4) After the preliminary investigation, the authorised person must decide within 10 days if they will—

- (a) further investigate the disclosure themselves; or
 - (b) refer the disclosure to another authorised person; or
- and must, in writing inform the discloser accordingly.

(5) An authorised person may decide not to investigate the disclosure if—

- (a) there is no merit in the allegations contained in the disclosure;
- (b) the disclosure is trivial;
- (c) there is a more appropriate method of dealing with the disclosure which is in line with approved policies and procedures in place and which does not include the commission of an offence; or
- (d) the disclosure was previously made and has been adequately dealt with.

(6) If the authorised person decides not to investigate the matter, they must give the discloser written reasons for the decision.

(7) (a) An authorised person who is of the opinion that the disclosure would be more appropriately dealt with by another authorised person must inform the discloser accordingly, setting out the reasons, and indicate which other person or body is more suited to deal with the disclosure.

(b) An authorised person who refers the disclosure to another authorised person must update the database accordingly as contemplated in section 3(2)(b)(i)

(c) A disclosure may not be referred to an employer if the disclosure is about the improper conduct of that employer.

(8) (a) An investigation undertaken in respect of a disclosure referred to in subsection (2) must be carried out as expeditiously as possible and must be completed within 12 months from the date of receipt of the disclosure, including a disclosure received as a result of a referral as contemplated in subsection (4)(b).

(b) If the investigation cannot be completed within the 12-month period, the person or body referred to in subsection (2) must apply, in the prescribed manner to the retired judge in terms of section 24, to extend the period for a further period not exceeding six months.

(c) An extension contemplated in paragraph (b) may only be granted once.



(d) Disclosers must be informed of the progress of the investigation at least once every three months.

- (9) (a) An authorised person may not investigate a disclosure where—
- (i) the subject matter of the disclosure is being investigated by another person, body or entity authorised by law to carry out that investigation;
 - (ii) it is clear that a criminal offence was committed, in which case the matter must be referred to a law enforcement agency for investigation.

(b) An authorised person must within five days of becoming aware of circumstances contemplated in paragraph (a), inform the discloser in writing accordingly.

(10) (a) An authorised person must, subject to the provisions of section 19 of the Protection of Personal Information Act, 2013 record and keep, or cause to be kept, all disclosures and documents in respect of the disclosure submitted to them.

(b) The records, disclosures and documents in respect of disclosures as contemplated in paragraph (a) must not be retained for a period any longer than is necessary for finalising the disclosure, unless—

- (i) the retention of the records, disclosures and documents in respect of disclosures is required or authorised by law; or
- (ii) the authorised person reasonably requires the record for lawful purposes related to its functions or activities:

Provided that the authorised person must retain the records, disclosures and documents for a longer period, they must without delay record the reasons for the continued retention and must inform the discloser accordingly.

(c) The records, disclosures and documents in respect of disclosures as contemplated in paragraph (a) must be destroyed in accordance with section 14(4) and (5) of the Protection of Personal Information Act, 2013.

(11) An authorised person who, during an investigation of a disclosure, conceals or suppresses evidence, commits an offence and is liable on conviction to a fine or to imprisonment for a period not exceeding 15 years, or to both such fine and imprisonment.

Application for court assistance

- 15.** If, during an investigation, it appears to the authorised person—
- (a) that evidence or documents relevant to the investigation are likely to be destroyed, concealed, or tampered with; or
 - (b) that a person willing to provide information relevant to the investigation is being subjected to undue pressure, inducement or intimidation to withhold that information,
- the authorised person must apply to any court having jurisdiction for an order to preserve the evidence or documents or to restrain the prevention, undue pressure, inducement or intimidation of that person.

Part 2

Finalisation of investigation

Finalisation of investigation

16. (1) Upon concluding an investigation under this Act, the authorised person must prepare a report which must contain—

- (a) the findings of the investigation; and



(b) recommendations for further steps, if any, to be taken.

(2) (a) The report contemplated in subsection (1) must be made available to—

- (i) the discloser; and
- (ii) the discloser's employer, if the discloser is an employee;
- (iii) the person or entity who was the discloser's employer at the time of the improper conduct, if the discloser is a former employee; or
- (iv) the relevant person, if the disclosure is against any other person that is not an employer.

(b) The report contemplated in subsection (1) may not disclose any personal confidential information about the discloser.

(3) A person who is aggrieved by the report of an authorised person may, in the prescribed manner, apply for the report to be reviewed by a competent court.

(4) If the improper conduct disclosed constitutes a criminal offence, the authorised person must refer the matter to the relevant law enforcement agency.



Enforcement of report

17. The authorised person or any other interested party may apply to a High Court as contemplated in the Superior Courts Act, 2013 (Act No. 10 of 2013), for the enforcement of recommendations as set out in the report prepared by the authorised person in terms of this Act.

Award to discloser

18. (1) A court which convicts an employer of improper conduct in terms of this Act and imposes a monetary sanction in respect of a disclosure may order, subject to subsection (4), order that an amount not exceeding one-fourth of that monetary sanction be awarded to the discloser whose evidence led to the conviction of that employer.

- (2) An award contemplated in subsection (1) may not be paid to a person—
- (a) in the public service;
 - (b) contemplated in section 34 of the Prevention and Combating of Corrupt Activities Act, 2004;
 - (c) who provided such information as a part of a plea agreement;
 - (d) who was an accomplice to the respective offence; or
 - (e) who is an official of a law enforcement agency, including an agent or confidential informant of that law enforcement agency who uncovered relevant information as a part of their main duties.

(3) If the conviction contemplated in subsection (1) is as a result of disclosures made by several disclosers about the same improper conduct, the court may order that the amount of the award be divided equally between the disclosers or according to their respective contributions as determined by the court.

(4) In determining if a discloser is entitled to any payment of an award as contemplated in subsection (1), the court must take into consideration whether the information—

- (a) originated from the discloser;
- (b) was not known prior to the disclosure; and
- (c) proves elements of a criminal or administrative offence.

CHAPTER 4 PROTECTION OF PERSON MAKING PROTECTED DISCLOSURE

Confidentiality

19. (1) An authorised person must not disclose information regarding the identity of the discloser or information likely to lead to the identification of the discloser unless the—

- (a) discloser has given their written consent for the release of their identity and only to the extent the discloser consented to; and
- (b) information is disclosed to employees of the authorised person who are competent to deal with the disclosure.

(2) Subject to subsection (1), an authorised person—

- (a) to whom a disclosure is made or to whom a disclosure was submitted; or
- (b) who obtains information related to the disclosure during an investigation into such disclosure,

must not disclose any information related to the investigation to any other person.

(3) (a) A discloser who discloses confidential information is permitted to do so when required—

- (a) by any competent court; or
- (b) in terms of any law.

(b) An authorised person must inform a discloser before making a disclosure that any confidential information disclosed may be required by a competent court or in terms of certain laws.

(4) Where a discloser is a witness in a matter that relates to information given by that discloser, the court before which the matter is heard may, on application by the discloser or by a person acting on behalf of the discloser, order that the discloser give evidence *in camera*.

(5) A book, paper or document which is part of evidence liable to inspection in any civil, criminal or other proceedings before any court, contains any entry in which a discloser is named or described or which might lead to the identification of the discloser, the court before which the proceedings are being held must cause all such passages to be concealed from view or to be redacted to the extent necessary to protect the identity of the discloser.

(6) A person who contravenes or fails to comply with subsection (1) or (2), commits an offence and is liable on conviction to a fine or to imprisonment for a period not exceeding 10 years, or to both such fine and imprisonment.

Exclusion of civil and criminal liability

20. Notwithstanding the provisions of any other law, no discloser is civilly or criminally liable on account of having made a disclosure in terms of this Act if they reasonably believed, at the time of the disclosure, that they were disclosing information of improper conduct.

Protection from occupational detriment or detrimental action



21. (1) No employee who is a discloser may be subjected to occupational detriment on account or partly on account of having made a protected disclosure.

(2) No discloser or related person may be subjected to detrimental action on account or partly on account of—

- (a) having made a protected disclosure; or
- (b) a belief that the discloser has made or intends to make a disclosure.

(3) A person takes detrimental action if that person—

- (a) threatens to take, attempts to take or takes detrimental action for a reason contemplated in subsection (2);
- (b) incites, authorises or knowingly permits another person to take or threaten to take such detrimental action. because—
 - (i) the discloser made the disclosure; or
 - (ii) the person believes that the discloser made or intends to make a disclosure; or
- (c) the person incites or permits another person to take or threaten to take the detrimental action for any reason under paragraph (a) or (b).

(3) A discloser or related person is not considered as having been subjected to occupational detriment or detrimental action if the occupational detriment or detrimental action is not linked in any way to the protected disclosure.

(4) A discloser or related person may approach a court with jurisdiction for appropriate relief, including interim relief in respect of any occupational detriment or detrimental action suffered.

(5) Despite anything to the contrary in any law, in any proceedings under this Act including administrative, civil, or labour proceedings concerning an alleged occupational detriment or detrimental action, the person who took or is alleged to have taken such action must show that the action was not taken in retaliation for a disclosure, subject to subsection (6).

(6) Before the evidential burden shifts in terms of subsection (5), the discloser must show that—

- (a) they made a protected disclosure; and
- (b) they or a related person suffered a detriment linked to the protected disclosure.

(7) Where an employer, or a person acting under the express or implied authority or with the knowledge of the employer or a client of the employer, subjects an employee or a related person to occupational detriment or detrimental action, the employer, such person and the client are jointly and severally liable.

(8) A person who contravenes subsection (1) or (2) commits an offence and is liable on conviction to a fine or to imprisonment for a period not exceeding 15 years, or to both such fine and imprisonment.

Witness protection for discloser

22. The provisions of the Witness Protection Act, 1998 (Act No. 112 of 1998), apply *mutatis mutandis* to a discloser or related person.

Legal assistance

23. (1) Where a discloser involved in a matter before a court or a tribunal is not represented by a legal representative because such discloser cannot afford to pay for legal representation, and the court or tribunal is of the opinion that it would be in the best interests



of the discloser to have legal representation, the court or tribunal must refer the matter to Legal Aid South Africa in terms of section 22A of the Legal Aid South Africa Act, 2014 (Act No. 39 of 2014), where substantial injustice would otherwise result.

(2) Expenditure in connection with the implementation and application of subsection (1) must be defrayed from money appropriated by Parliament or other funding made available for this purpose, which funds constitute earmarked funds on the vote of Legal Aid South Africa, and may not be used for any other purpose.

(3) The Legal Aid South Africa must preserve and protect the confidential information concerning the discloser.

CHAPTER 5 COMPLAINTS MECHANISM

Complaints mechanism

24. (1) For purposes of this Chapter—

- (a) **'Chapter 9 Institution'** means an institution referred to in Chapter 9 of the Constitution of the Republic of South Africa, 1996;
- (b) **'complainant'** means a discloser, related person or person acting on behalf of the discloser or related person;
- (c) **'retired judge'** means a judge discharged from active service as referred to in the Judges' Remuneration and Conditions of Employment Act, 2001 (Act 47 of 2001).

(2) The President must, in consultation with the Chief Justice, designate a retired judge to investigate complaints referred to in subsection (3).

(3) The retired judge may in the prescribed manner receive—

- (a) an application for extension of the period in which an investigation must be finalised as contemplated in section 14(8)(a);
- (b) complaints, including complaints from a complainant—
 - (i) who honestly and reasonably believes that they have been or are likely to be subjected to detrimental action by any person in retaliation for making a disclosure;
 - (ii) whose identity or confidential information is about to be made known in contravention of section 19(1); or
 - (iii) whose protection was revoked in terms of section 27.

(4) The retired judge must submit a written report to the Minister within three months after the end of each financial year, which must include—

- (a) the number of complaints received;
- (b) the outcome of any investigation undertaken by the retired judge; and
- (c) any referral made to a law enforcement agency, or a Chapter 9 Institution, without including any confidential personal information of a complainant.

(5) The retired judge may request information from any person or employer for purposes of an investigation by that judge.

(6) A refusal to comply with a request in terms of subsection (5) is a criminal offence for which a person, is liable upon conviction to a fine or imprisonment for a period not exceeding two years, or to both a fine and imprisonment, unless the refusal is based on a lawful ground, including—

- (a) legal professional privilege;



- (b) the protection of classified, confidential or security sensitive information in terms of any law; or
- (c) any other ground recognised by law.

(7) To the extent that it is reasonably necessary for the performance of the functions and duties of the retired judge, the retired judge—

- (a) may enter any building or premises under the control of a person or employer to obtain information and documents; and
- (b) must be afforded all reasonable assistance by a person or employer.

(8) The Minister must, in consultation with the retired judge, ensure that the retired judge has sufficient personnel and resources to fulfil their functions.

(9) The Director-General, in consultation with the retired judge, must prepare an annual operational budget for the performance of the functions and duties of the retired judge.

(10) The budget contemplated in subsection (9) may only be used for the performance of the functions and duties of the retired judge.

(11) The Director-General, in consultation with the retired judge, must develop and implement a plan to promote public awareness relating to the functions of the retired judge.

(12) A person who interferes with the retired judge in performing their functions and duties commits a criminal offence and is liable on conviction to a fine or imprisonment for a period not exceeding two years or to both such fine and imprisonment.



Dealing with complaint

25. (1) The retired judge must upon receipt of a complaint—

- (a) investigate such complaint in the prescribed manner; or
- (b) refer the complaint, in the prescribed manner, to be dealt with by the relevant—
 - (i) law enforcement agency; or
 - (ii) Chapter 9 Institution.

(2) If the retired judge decides to—

- (a) investigate the complaint, they must inform the complainant and the employer or other person who threatens the complainant with occupational detriment or detrimental action, of the decision;
- (b) not investigate the complaint, they must inform the complainant accordingly and set out the reasons for the decision; or
- (c) refer the complaint, they must inform the complainant accordingly and set out the reasons for the decision.

(3) The retired judge, law enforcement agency or Chapter 9 Institution may decline to deal with a complaint if—

- (a) the subject matter of the complaint has already been dealt with by another agency, tribunal or court;
- (b) the complaint falls within the ambit of the Labour Relations Act, or any other law; or
- (c) after assessing the complaint, there are no reasonable grounds to support the complaint.

(4) (a) If a complaint is brought in terms of section 24(3)(b)(ii) the retired judge must immediately notify any court, person, body or authority that intends to disclose confidential information under section 19(1) of this Act of the complaint received.

(b) No confidential information may be disclosed until—

- (i) the retired judge has completed the investigation; and
- (ii) submitted a recommendation, which must be considered before any further action is taken.

(5) Nothing in this section prevents a complainant from instituting civil action in a court of law in relation to the complaint.

Finalisation of complaint

26. (1) Upon the finalisation of the complaint, the retired judge, law enforcement agency or Chapter 9 Institution must inform the complainant whether the complaint was—

- (a) successful and the remedies available to the complainant; or
- (b) unsuccessful and the reasons why the complaint was unsuccessful.

(2) If the retired judge has referred the matter in accordance with section 25(1)(b), the relevant agency or Institution must, upon finalisation of the investigation, provide the retired judge with a report on the—

- (a) outcome of the investigation and remedies available to the complainant; or
- (b) set out the reasons why the complaint was unsuccessful.



CHAPTER 6 MISCELLANEOUS

Revocation of protection

27. (1) An authorised person may revoke the protection awarded to a discloser or related person for making a protected disclosure under this Act if the authorised person is of the opinion, based on an investigation or during an investigation that—

- (a) the discloser has, after making a disclosure, participated or continued to participate in the improper conduct disclosed;
- (b) the discloser, in their disclosure, intentionally made a material statement which they knew or believed to be false or did not believe to be true;
- (c) the disclosure is made solely or substantially with the motive of avoiding dismissal or other disciplinary action; or
- (d) the discloser, while making the disclosure or providing further information, commits an offence under this Act.

(2) (a) If the discloser protection is revoked under subsection (1), the authorised person must—

- (i) give a written notice to that effect to the discloser; and
- (ii) notify the Director: Office for Witness Protection, appointed in terms of section 3(1) of the Witness Protection Act, 1998, if the discloser or related person was under protection or temporary protection in terms of the Witness Protection Act, 1998, to deal with the matter in accordance with their powers.

(b) The Director: Office for Witness Protection must upon receipt of the notification contemplated in paragraph (a)(ii), make a decision with regards to the protection or temporary protection of the discloser or related person in terms of the Witness Protection Act, 1998.

(3) A discloser or related person who is aggrieved by the decision of an authorised person made under subsection (1) may—

- (a) lodge a complaint as contemplated in section 24(3); or
- (b) approach a court which has jurisdiction to adjudicate the matter.

(4) Where a discloser or related person has lodged a complaint or approached a court in terms of subsection (3), the protection that has been granted to the discloser or related person continues unless the retired judge or court rules otherwise.

Remedies

28. (1) Any discloser or related person who has been subjected, is subjected or may be subjected to occupational detriment or detrimental action in breach of section 21, or anyone acting on behalf of a discloser or related person who is not able to act in their own name, may—

- (a) lodge a complaint in term of section 24;
- (b) approach any court having jurisdiction, including the Labour Court established by section 151 of the Labour Relations Act, 1995, for appropriate relief; or
- (c) pursue any other process allowed or prescribed by any law.

(2) If the court, including the Labour Court, is satisfied that a discloser or related person has been subjected to or will be subjected to occupational detriment or detrimental action on account of making a protected disclosure, the court may make an appropriate order that is just and equitable in the circumstances, including—

- (a) an order directing the employer, the client of the employer, or a person acting on behalf of or on the authority of such employer, as the case may be, to pay compensation to the discloser or related person;
- (b) an order directing the employer, the client of the employer, or a person acting on behalf of or on the authority of such employer as the case may be, to pay compensation of actual damages suffered by the discloser or related person; or
- (c) an order directing the employer, the client of the employer, or a person acting on behalf of or on the authority of the employer as the case may be, to take steps to remedy the occupational detriment or detrimental action.

(3) For the purposes of the Labour Relations Act, 1995 including the consideration of any matter emanating from this Act by the Labour Court—

- (a) any dismissal in breach of section 21 of this Act is deemed to be an automatically unfair dismissal as contemplated in section 187 of the Labour Relations Act, 1995 and the dispute about such a dismissal may follow the procedure set out in Chapter VIII of that Act or any other process to recover damages in a competent court; and
- (b) any other occupational detriment or detrimental action in breach of section 21 of this Act is deemed to be an unfair labour practice as contemplated in section 186(2) of the Labour Relations Act, 1995, and the dispute about such an unfair labour practice must follow the procedure set out in section 191 of that Act.: Provided that if the matter fails to be resolved through conciliation, it may be referred to the Labour Court for adjudication.

(4) Any discloser who has made a protected disclosure and who reasonably believes that they may be adversely affected on account of having made that disclosure, must, at their request and if reasonably possible or practicable, be transferred from the post or position occupied by them at the time of the disclosure to another post or position in the same division or another division of their employer or, where the person making the disclosure is employed by a public body, to another public body.



(5) The terms and conditions of employment of a person transferred in terms of subsection (4) may not, without their written consent, be less favourable than the terms and conditions applicable to them immediately before their transfer.

Education and information

29. (1) The public and private sectors have a duty to promote awareness of this Act.

(2) Without derogating from the general nature of the duty referred to in subsection (1), one or more Cabinet members, designated by the President, must cause programs to be developed to—

- (a) conduct education and information campaigns to inform the public about this Act; and
- (b) train public officials on the prohibition, prevention and combating of occupational detriment or detrimental actions against a discloser, or related person which training must include social context training.

(3) The South African Judicial Education Institute established in terms of section 3 of the South African Judicial Education Institute Act, 2008 (Act No. 14 of 2008), must develop and implement training courses including social context training courses, for judicial officers for purposes of presiding in court proceedings, for the purposes of this Act.

(4) Every employer and other authorised person must, if the employer or authorised person has a website, publish at least the following information on the website—

- (a) an explanation of what constitutes a protected disclosure;
- (b) the manner in which a disclosure can be made;
- (c) contact details of the authorised person to whom a disclosure can be made;
- (d) remedies and procedures for protection against occupational detriment or detrimental action; and
- (e) conditions under which persons reporting to an authorised person are protected from incurring liability for a breach of confidentiality.

Regulations

30. (1) The Minister may make regulations regarding—

- (a) any matter required or permitted to be prescribed in terms of this Act;
- (b) generally, all matters which are reasonably necessary or expedient to be prescribed to achieve the objects of this Act.

(2) Any regulation under this section which results in State expenditure must be made in consultation with the Cabinet member responsible for finance.

(3) Any regulation made under subsection (1) may provide that any person who contravenes a provision thereof or fails to comply therewith shall be guilty of an offence and on conviction be liable to a fine, or to imprisonment for a period not exceeding three years.

Transitional provisions and saving

31. (1) Any disclosures made and actions taken before the commencement of this Act must be continued and concluded as if this Act has not been passed.

(2) Anything done in terms of a law repealed by section 32 and which could have been done in terms of this Act is regarded as having been done in terms of this Act.



(3) The regulations and practical guidelines in force before the commencement of this Act remain in force until repealed or amended.

Amendment and Repeal of Laws

32. The laws specified in the Schedule are repealed or amended to the extent indicated in that Schedule.

Short title and commencement

33. (1) This Act is called the Protected Disclosures Act, 2026, and comes into operation on a date fixed by the President by proclamation in the *Gazette*.

(2) Different dates may be fixed in respect of different provisions of this Act.

Schedule LAWS REPEALED OR AMENDED BY SECTION 31



No. and Year of Law	Short title	Extent of Repeal or Amendment
Act 112 of 1998	Witness Protection Act, 1998	(a) Amends section 1 by the substitution for the definition of 'witness' of the following definition: "witness" means any person who is or may be required to give evidence, or who has given evidence in any proceedings <u>and includes a person who made a disclosure in terms of the Protected Disclosures Act, 2000 (Act No. 26 of 2025; and"</u> (b) amends section 7(1)(a) by the addition of the following subparagraph after subparagraph (v): <u>"(vi) to an authorised person in the event of a discloser; and"</u>; and (c) amends the Schedule by the insertion of the following item after item 15: <u>"15A. A protected disclosure in terms of the Protected Disclosure Act, 2000 (Act No. 26 of 2000."</u>
Act 26 of 2000	Protected Disclosures Act, 2000	Repeal of whole of Act.
Act 39 of 2014	Legal Aid South Africa Act, 2014	Amends section 22 by the insertion of the following section after section 22: <u>"Provision of legal aid by direction of courts in matters related to protected disclosures</u>

		<u>22A. (1) For purposes of this section 'discloser' and 'related person' means a discloser or related person as defined in the Protected Disclosure Act, 2000 (Act No. 26 of 2000).</u> <u>(2) A court or tribunal, in dealing with any matter resulting from a protected disclosure in terms of the Protected Disclosures Act, 2000 may direct that a discloser or related person be provided with legal representation at state expense, if the court or tribunal has considered—</u> <u>(a) the personal circumstances of the discloser or related person concerned and that it would be in the best interests of the discloser, or related person to have legal representation;</u> <u>(b) whether any other legal representation at state expense is available or has been provided; and</u> <u>(c) any other factor which in the opinion of the court should be considered.</u> <u>(3) A direction received from a court or tribunal as contemplated in subsection (2), must be treated with the same confidentiality afforded to the discloser or related person in terms of the Protected Disclosures Act, 2000</u> <u>(4) Legal Aid South Africa must, notwithstanding the provisions of section 22 of this Act, without—</u> <u>(a) delay; and</u> <u>(b) evaluation and reporting, provide legal representation to the discloser or related person."</u>
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